

PassReady Prep — NCE Question Bank · Batch 7

Generated: 2026-10-05 · Showing: In review · Total items: 200

How to review

✓ **KEY** marks the correct option. Every option shows why it is right or wrong; check both the key and each distractor.

Mark each item **Approve**, **Revise** (note what to change) or **Retire**. Approved items are published from Admin → NCE Questions.

▮ **Check** notes mark specific facts or citations the writer was less than certain of — verify those first.

Check: one clearly best answer · factual accuracy (theorists, dates, test properties, code sections) · distractors plausible but defensibly wrong · no answer given away by length or wording · references support the key.

NCE domains (NBCC 2023 content outline)

Professional Practice and Ethics 19 · Intake, Assessment, and Diagnosis 19 · Areas of Clinical Focus 47 · Treatment Planning 14 · Counseling Skills and Interventions 48 · Core Counseling Attributes 13 — of 160 scored items

In this copy

Professional Practice and Ethics **24** · Intake, Assessment, and Diagnosis **24** · Areas of Clinical Focus **59** · Treatment Planning **17** · Counseling Skills and Interventions **60** · Core Counseling Attributes **16** · **7** flagged ▮

Professional Practice and Ethics

24 items · NCE weight 12% (19 of 160 scored items)

Q1 of 200 · nce-s-eth-145 · Professional Counseling Orientation and Ethical Practice · ACA Code of Ethics structure · hard · In review

A community agency adopts a policy that any client who misses two appointments is discharged with no phone call, letter or referral. A staff counselor believes the policy harms clients. She wants to find the 2014 ACA Code of Ethics standard on alerting employers to inappropriate policies and working to change them. In which section is that standard found?

A. Section A, The Counseling Relationship, under the standards on client welfare

Section A addresses client welfare and abandonment, but the standard on responding to employer policies sits elsewhere.

B. Section E, Evaluation, Assessment, and Interpretation, under test standards

Section E covers testing, assessment and diagnosis, not a counselor's response to employer policies.

C. Section D, Relationships With Other Professionals, under employer relations ✓ **KEY**

D.1.h Negative Conditions directs counselors to alert employers to inappropriate policies and seek change constructively.

D. Section C, Professional Responsibility, under the standards on public duties

Section C covers competence, advertising and public responsibility, not responses to an employer's policies.

Rationale: Section D of the 2014 ACA Code covers relationships with colleagues, employers and employees. Standard D.1.h, Negative Conditions, asks counselors to alert employers to inappropriate policies and practices and to try to change them through constructive action within the organization. If such policies harm clients and change cannot be achieved, counselors take further appropriate action, which may include referral to credentialing or licensure bodies or leaving the job.

References: ACA Code of Ethics — D.1.h Negative Conditions

☐ Approve ☐ Revise ☐ Retire Notes: _____

A laptop holding unencrypted intake records for 40 clients is stolen from a counselor's car. The practice is a HIPAA-covered entity, and the breach is discovered the same day. Under the HIPAA Breach Notification Rule, how must the practice notify the affected clients?

A. Without unreasonable delay and no later than 60 calendar days after discovering the breach ✓ **KEY**

The Breach Notification Rule requires individual notice without unreasonable delay and within 60 days of discovery.

B. Only if a client later reports harm, since loss of a device is not a breach under HIPAA

Loss or theft of unsecured PHI is presumed a breach unless a risk assessment shows a low probability of compromise.

C. Within one year, in a single annual letter sent to every client the practice has served

Annual logging applies to reporting small breaches to HHS, not to notifying the affected individuals.

D. Only after police finish their investigation, so the notice does not alert the thief

Delay is allowed only when law enforcement formally requests it; the practice does not wait on its own.

Rationale: Under the HIPAA Breach Notification Rule, a covered entity must notify each person whose unsecured protected health information was breached without unreasonable delay and no later than 60 calendar days after discovery. A breach affecting fewer than 500 people is also logged and reported to HHS within 60 days after the end of the calendar year. Encrypting devices is the main way to avoid a reportable breach when equipment is lost or stolen.

References: HIPAA — Breach Notification Rule, 45 CFR 164.404 (individual notice within 60 days of discovery)

☐ Approve ☐ Revise ☐ Retire Notes:

A 33-year-old client at a HIPAA-covered practice decides to pay in full, out of pocket, for all of her counseling. She asks that no information about these sessions be sent to her employer-sponsored health plan. Which statement about her request is accurate under the HIPAA Privacy Rule?

A. The practice may decline, since HIPAA lets providers refuse any restriction that a client requests

Providers may usually decline restriction requests, but not this one when services are paid in full out of pocket.

B. The practice must agree only if the plan first gives written approval for the restriction

The health plan's approval is not required; the right belongs to the client who paid in full.

C. The request is moot, as counseling notes are psychotherapy notes that plans cannot see

Psychotherapy notes are a narrow category; claims data, diagnoses and dates would still go to the plan.

D. The practice must agree, since she paid in full for the services she wants kept from the plan ✓ **KEY**

HIPAA requires providers to honor a restriction on disclosure to a health plan for services paid in full out of pocket.

Rationale: Clients may ask a covered entity to restrict uses and disclosures of their information, and providers are usually free to decline. The HITECH Act added one required exception: a provider must agree not to disclose information to a health plan for payment or health care operations when the client has paid for that item or service in full out of pocket. Counselors in practices that bill insurance should know this option when clients worry about employer plans.

References: HIPAA — Privacy Rule, 45 CFR 164.522(a) Right to request restriction; required restriction for services paid in full out of pocket

☐ Approve ☐ Revise ☐ Retire Notes:

A high school plans to print students' names, grade levels and sports teams in a booster club program. Under FERPA, what must the school do before releasing this kind of directory information without written consent?

A. Get a signed release from every parent for each separate use of a student's name

FERPA allows release of properly designated directory information without individual signed consent.

B. Give annual notice of what it calls directory information and allow families to opt out ✓ **KEY**

Schools may disclose directory information if they give public notice and a chance to opt out.

C. Nothing at all, since FERPA covers only grades and not any other student information

FERPA covers all education records; directory information is a defined exception with notice rules.

D. Ask the school counselor to approve the list, since counselors hold FERPA authority

FERPA places duties on the school, not on the counselor as an approving authority.

Rationale: FERPA lets schools disclose directory information, such as name, grade level and participation in official sports, without written consent. The school must first give public notice of the types of information it treats as directory information and allow parents or eligible students a reasonable time to opt out. School counselors should know which records are protected and which may be released under this exception.

References: FERPA — 34 CFR 99.37 Directory information: annual notice and opt-out

☐ Approve ☐ Revise ☐ Retire Notes:

A 47-year-old client who was demoted at a plant says, "My shift supervisor ruined my life, and Monday I'm bringing my rifle to work to settle it." He does not give the supervisor's name, but he has worked under only one supervisor for six years. How should the counselor view the duty to protect?

A. It does not apply, because the client did not state the supervisor's full name in the session

The duty turns on whether a victim is reasonably identifiable, not on hearing a full name.

B. It likely applies, since the target is reasonably identifiable and the threat is serious ✓ **KEY**

A specific, serious threat against a person who can be identified triggers protective action under most laws.

C. It applies only after the client repeats the threat in a second session with more detail

Waiting for a repeated threat could leave the supervisor in danger; one credible threat is enough to act.

D. It is satisfied by telling the client that such statements are a crime under state law

Warning the client about the law does not protect the supervisor or meet the duty to protect.

Rationale: Duty-to-protect laws generally apply when a client communicates a serious threat of violence against a reasonably identifiable victim. A role such as "my shift supervisor" can identify a person as clearly as a name. The counselor assesses risk, consults, follows state law on protective steps, and documents each action.

References: ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements · State law — Duty-to-protect statutes: serious threat against a reasonably identifiable victim

☐ Approve ☐ Revise ☐ Retire Notes:

After a client makes a threat against a neighbor, a counselor assesses risk, consults a colleague, and notifies the neighbor and police as state law allows. What should the counselor's record of this event include?

- A. A short note that a threat was made, leaving out the reasons so the file stays brief
Leaving out the reasoning removes the evidence that the counselor acted with sound judgment.
- B. Nothing in the client file, with the details kept in a private journal to protect the client
Keeping key risk actions out of the record leaves the file incomplete and weakens the counselor's defense.
- C. Only the police report number, since police records will hold the details of the threat
A police number does not show the counselor's assessment, consultation or reasoning.
- D. The threat, the risk assessment, the consultation, the reasons and the steps taken ✓ KEY
A full record of assessment, consultation, reasoning and actions shows that the counselor met the standard of care.

Rationale: When counselors act on a duty to protect, the record should show what the client said, how risk was assessed, whom the counselor consulted, why the chosen action was taken and what was done. Clear, timely notes document good-faith professional judgment. They are also the main evidence if the decision is later reviewed by a board or court.

References: ACA Code of Ethics — B.2.a Serious and Foreseeable Harm; A.1.b Records and Documentation · Wiger (Documentation) — Documenting risk assessment, consultation and protective actions

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor tells a client honestly that progress has stalled and explains why, even though the client hoped to hear she was nearly finished. Which ethical principle named in the 2014 ACA Code of Ethics does this MOST directly reflect?

- A. Veracity, dealing truthfully with people ✓ KEY
The ACA Code defines veracity as dealing truthfully with individuals with whom counselors come into contact.
- B. Justice, treating people fairly and equally
Justice concerns fairness and equality, not honesty with a single client about progress.
- C. Fidelity, keeping promises and commitments
Fidelity concerns honoring commitments and trust, not giving truthful feedback.
- D. Autonomy, supporting a client's own choices
Autonomy concerns self-determination; the key act here is telling the truth.

Rationale: The Preamble of the 2014 ACA Code lists six principles that guide ethical decisions: autonomy, nonmaleficence, beneficence, justice, fidelity and veracity. Veracity, dealing truthfully with people, goes beyond the five principles Kitchener originally described. Honest feedback about progress, even when unwelcome, reflects this principle.

References: ACA Code of Ethics — Preamble: fundamental principles of professional ethical behavior (veracity)

☐ Approve ☐ Revise ☐ Retire Notes:

While testing a chosen course of action, a counselor asks herself, "Would I recommend this same action to any other counselor facing this exact situation?" In the tests Stadler proposed, which are included in Forester-Miller and Davis's model, which test is she applying?

A. The justice test, asking whether others would be treated the same way

The justice test asks whether the counselor would treat others in this situation the same way.

B. The publicity test, asking about the action being reported in the press

The publicity test asks whether the counselor would be comfortable seeing the action in the news.

C. The universality test, asking if she'd advise it to any peer in her place ✓ **KEY**

The universality test asks whether the counselor would recommend the same action to anyone in the same situation.

D. The fidelity test, asking whether the action keeps promises to the client

Fidelity is a moral principle, not one of Stadler's three tests for evaluating an action.

Rationale: Forester-Miller and Davis suggest that, after choosing an action, counselors check it with Stadler's three tests: justice, publicity and universality. The universality test asks whether the counselor would recommend the same action to another counselor in the same situation. If the action fails any test, the counselor returns to earlier steps and reconsiders.

References: Forester-Miller & Davis (Practitioner's Guide to Ethical Decision Making) — Step 6: Evaluate the selected course of action (Stadler's justice, publicity and universality tests)

☐ Approve ☐ Revise ☐ Retire Notes:

A physical therapy clinic offers a counselor \$50 for each client she refers there for treatment of chronic pain. The counselor already believes the clinic does good work and would refer clients anyway. Under the ACA Code of Ethics, how should she respond to the offer?

A. Accept it, since she would make the same referrals whether or not she was paid for them

Her intent does not matter; A.10.b bars payment for referrals because it can bias referral choices.

B. Accept it if she tells each referred client about the payment before making the referral

Disclosure does not make a referral payment acceptable; the standard bars the payment itself.

C. Accept it as long as the payments go toward a reduced-fee fund for clients of her practice

Where the money goes does not change that she would be receiving remuneration for referrals.

D. Decline it, because counselors do not give or receive payment in return for client referrals ✓ **KEY**

A.10.b bars giving or receiving commissions, rebates or other payment for referring clients.

Rationale: ACA Standard A.10.b, Unacceptable Business Practices, states that counselors do not take part in fee splitting and do not give or receive commissions, rebates or any other form of payment when referring clients for professional services. Referral payments create a conflict of interest that can lead counselors to refer for gain rather than for client welfare. Counselors base referrals only on what clients need.

References: ACA Code of Ethics — A.10.b Unacceptable Business Practices

☐ Approve ☐ Revise ☐ Retire Notes:

A 16-year-old client tells her counselor that she is in a sexual relationship with a 23-year-old man she met at work and says it is fully consensual. She begs the counselor not to tell anyone. What should guide the counselor's decision about reporting?

A. Her state's age-of-consent and reporting laws, which decide whether a report is required ✓ KEY

Whether this must be reported turns on state statutes on age of consent, age gaps and reporter duties.

B. The client's view that it is consensual, which removes any duty to report under the law

A minor's consent may have no legal effect; the client's view does not settle the counselor's duty.

C. Federal law, which sets a single national age of consent that applies in all fifty states

Age of consent is set by each state and varies, so no single national rule settles the question.

D. Her parents' wishes, since only a parent may decide whether a report is to be made

Reporting duties belong to the mandated reporter by law; parents cannot waive them.

Rationale: Ages of consent and the rules on reporting sexual activity involving minors differ widely across states, and many statutes turn on the age gap between partners. The counselor must know her state's law, consult a supervisor or attorney when unsure, and then act on what the law requires. She should explain the limits of confidentiality to the client with care and document her reasoning.

References: State law — Age of consent and statutory rape reporting provisions vary by state · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in solo practice will take a three-month parental leave starting next month. Several clients are in active treatment, and two have a history of suicidal crises. Under the ACA Code of Ethics, what should the counselor do?

A. Pause all treatment and tell clients to call 911 if anything urgent comes up during the leave

Leaving clients with only emergency numbers for three months risks neglect of their ongoing care.

B. End counseling with every client now, since leave of this length requires full termination

A temporary leave does not require ending treatment; it requires a plan for continuity of care.

C. Arrange for continued care during the leave, such as a covering counselor or referrals ✓ KEY

A.12 directs counselors to arrange for continuation of treatment during interruptions such as leave.

D. Keep seeing only the two high-risk clients from home and tell the rest to wait until she returns

Working during leave from home is not a plan, and the other clients still need continued care.

Rationale: ACA Standard A.12 bars counselors from abandoning or neglecting clients and requires them to arrange for continuation of treatment when necessary during interruptions such as vacations, illness and leave. Planning ahead, telling clients early, and setting up coverage or referral, with attention to clients at higher risk, protects client welfare.

References: ACA Code of Ethics — A.12 Abandonment and Client Neglect

☐ Approve ☐ Revise ☐ Retire Notes:

An agency moves a counselor to a new program and reassigns her caseload of 30 clients to other staff. Her supervisor suggests simply sending each client a letter with the new counselor's name. Under the ACA Code of Ethics, what should the counselor ensure during the transfer?

A. That the letters go out at least two weeks before the move, after which her role is over

Early letters alone do not meet the duty to complete transfer processes and keep communication open.

B. That clinical and administrative steps are done and communication stays open with all ✓ **KEY**

A.11.d requires completing clinical and administrative processes and open communication with clients and practitioners.

C. That clients sign a waiver releasing her from any duty once their new counselor is named

A waiver does not replace the duty to complete an appropriate transfer of services.

D. That the files go to the new counselors, as clients are agency clients and need no contact

Clients belong to no one; they still need to be informed and prepared for the change in counselor.

Rationale: ACA Standard A.11.d, Appropriate Transfer of Services, requires counselors who transfer or refer clients to ensure that appropriate clinical and administrative processes are completed and that open communication is maintained with both clients and practitioners. A good transfer includes telling clients directly, discussing their feelings about the change, and sharing needed information with the new counselor with consent.

References: ACA Code of Ethics — A.11.d Appropriate Transfer of Services

☐ Approve ☐ Revise ☐ Retire Notes:

Two counselors at a busy hospital clinic start discussing a client's recent disclosure while riding a crowded public elevator. They do not use the client's name. What is the ethical concern?

A. Client details belong only in settings where privacy can be reasonably ensured ✓ **KEY**

B.3.c directs counselors to discuss confidential information only where privacy can be reasonably ensured.

B. None, because leaving out the client's name fully protects her confidentiality

Details can identify a client even without a name, especially in a clinic's own building.

C. None, because hospital staff share confidential information as a team by law

Team sharing does not cover talking in public spaces where others can overhear.

D. Only that they should have asked the client's permission before the elevator ride

Even with consent, a public elevator is not a fitting place for clinical discussion.

Rationale: ACA Standard B.3.c, Confidential Settings, directs counselors to discuss confidential information only in settings where they can reasonably ensure client privacy. Elevators, hallways and cafeterias are public spaces where others may overhear and recognize a client from details alone. Case discussion belongs in private rooms.

References: ACA Code of Ethics — B.3.c Confidential Settings

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor at a HIPAA-covered practice notes that the Privacy Rule permits sharing information with insurers for payment without a separate authorization. A new client has not yet agreed to insurance billing. Under the ACA Code of Ethics, how should the counselor proceed?

A. Bill the insurer without asking, since HIPAA permission settles the ethical question

When the Code sets a higher standard than the law, counselors follow the stricter standard.

B. Bill the insurer but leave out the diagnosis, which makes client consent unnecessary

Omitting a diagnosis still discloses information to the payer and does not replace authorization.

C. Disclose to the insurer only after the client has authorized that disclosure ✓ **KEY**

B.3.d says counselors disclose information to third-party payers only when clients have authorized it.

D. Refuse insurance entirely, since the Code bars sharing any client data with payers

The Code permits disclosure to payers with client authorization; it does not ban insurance billing.

Rationale: HIPAA permits covered entities to use and disclose information for treatment, payment and operations without a separate authorization. ACA Standard B.3.d is stricter: counselors disclose information to third-party payers only when clients have authorized it. When law permits but the Code requires more, counselors meet the higher ethical standard, usually by covering insurance billing during informed consent.

References: ACA Code of Ethics — B.3.d Third-Party Payers · HIPAA — Privacy Rule, 45 CFR 164.506 Uses and disclosures for treatment, payment and health care operations

☐ Approve ☐ Revise ☐ Retire Notes:

Q15 of 200 · nce-s-eth-159 · Professional Counseling Orientation and Ethical Practice · multiple relationships and boundary crossings · medium · In review

A counselor works full time at a community mental health agency and also has a small evening private practice. She plans to hand out her private practice cards to agency clients on the waitlist, telling them she can see them sooner privately. Under the ACA Code of Ethics, what is the problem with this plan?

A. None, since the clients would get care sooner than they would on the agency waitlist

Faster access does not justify using her agency role to build her own practice.

B. None, as long as she charges these clients the same fee that the agency would charge

Matching the agency fee does not cure using her employment to recruit private clients.

C. It is a problem only if she sees the clients privately during her agency work hours

The standard concerns recruiting through employment, not only the hours when services occur.

D. She may not use her agency position to recruit clients for her own private practice ✓ **KEY**

C.3.d bars using a place of employment or institutional affiliation to recruit private clients.

Rationale: ACA Standard C.3.d, Recruiting Through Employment, states that counselors employed by an organization do not use their place of employment or institutional affiliation to recruit clients, supervisees or consultees for their private practices. Such recruiting exploits clients' access to the agency and creates a conflict of interest. A.10.a, Self-Referral, adds that counselors in such settings do not refer clients to their own private practice unless agency policy explicitly allows it, and then clients must be told of other options.

References: ACA Code of Ethics — C.3.d Recruiting Through Employment · ACA Code of Ethics — A.10.a Self-Referral

☐ Approve ☐ Revise ☐ Retire Notes:

A man whom a counselor dated for a year in college, about 12 years ago, calls her practice asking to start counseling for work stress. They have had no contact since. Under the ACA Code of Ethics, how should the counselor respond?

A. Accept him, since more than five years have passed since the romance ended

The five-year rule applies to former clients; this standard has no time limit for former partners.

B. Decline to counsel him and offer referrals to other qualified counselors ✓ KEY

A.5.b bars counseling relationships with persons with whom the counselor had a romantic relationship.

C. Accept him only if her supervisor reviews each session for signs of bias

Supervision does not make a prohibited counseling relationship acceptable.

D. Accept him if he signs a consent form noting their prior relationship

Client consent does not lift the prohibition on counseling a former partner.

Rationale: ACA Standard A.5.b prohibits counselors from entering counseling relationships with people with whom they have had a previous sexual or romantic relationship. Unlike the five-year standard for romantic contact with former clients, this rule has no time limit. The counselor declines and offers appropriate referrals.

References: ACA Code of Ethics — A.5.b Previous Sexual and/or Romantic Relationships

☐ Approve ☐ Revise ☐ Retire Notes:

An experienced counselor reviews her outcome data and finds that clients in her caseload who are over 65 drop out far more often than younger clients and show little symptom change. Under the ACA Code of Ethics, what is her MOST fitting response?

A. Review her work with older clients and seek training, supervision or consultation ✓ KEY

C.2.d asks counselors to monitor their effectiveness and take steps to improve when needed.

B. Assume older clients are less motivated and keep using her current approach with them

Blaming age ignores her duty to examine her own effectiveness and may reflect ageist bias.

C. Stop collecting outcome data, since it may later be used to question her competence

Stopping measurement hides the problem rather than improving the care clients receive.

D. Refer every client over 65 elsewhere, since her license does not cover older adults

A license covers adults of all ages; a blanket referral avoids rather than fixes the gap.

Rationale: ACA Standard C.2.d directs counselors to continually monitor their effectiveness as professionals and take steps to improve when necessary, including seeking peer supervision. Outcome data that show poorer results for one group are a signal to examine one's approach, cultural and developmental knowledge, and possible bias. Training and consultation address the gap without denying services.

References: ACA Code of Ethics — C.2.d Monitor Effectiveness · Lambert (Outcome Monitoring) — Using outcome data to identify clients not on track

☐ Approve ☐ Revise ☐ Retire Notes:

A university training clinic has a one-way mirror, and practicum students often watch sessions from the observation room. A new client was told about video recording but not about the mirror. Under the ACA Code of Ethics, what must happen before students observe her sessions?

A. Nothing more, because consent to recording also covers live observation by students

B.6.d treats observation separately; consent to recording does not cover people watching live.

B. The observing students must sign a pledge to keep anything they see confidential

Student pledges are useful but do not replace the client's own permission to be observed.

C. The client must be told about the observation and give her permission in advance ✓ **KEY**

B.6.d requires client permission before any observation of sessions by supervisors, faculty or peers.

D. The clinic director must approve, because the clinic is part of a training program

Administrative approval does not stand in for the client's informed permission.

Rationale: ACA Standard B.6.d, Permission to Observe, requires counselors to obtain clients' permission before allowing any observation of sessions, review of transcripts, or viewing of recordings by supervisors, faculty, peers or others in a training setting. Permission to record (B.6.c) and permission to observe are separate. Clients should know who may watch and why before they agree.

References: ACA Code of Ethics — B.6.d Permission to Observe · ACA Code of Ethics — B.6.c Permission to Record

☐ Approve ☐ Revise ☐ Retire Notes:

Q19 of 200 · nce-s-eth-163 · Professional Counseling Orientation and Ethical Practice · supervision and consultation responsibilities · medium · In review

A post-master's supervisee tells her clinical supervisor that her panic attacks have returned since a family crisis. Because she trusts the supervisor, she asks him to provide her personal counseling in addition to supervision. Under the ACA Code of Ethics, how should the supervisor respond?

A. Agree, as long as the counseling sessions are held on a different day from supervision

Separate days do not change the prohibition on supervisors counseling their supervisees.

B. Agree, since the trust she already feels is likely to speed her recovery from panic

Existing trust does not remove the role conflict between evaluator and counselor.

C. Agree, but only once she signs a consent form describing both roles and their risks

Consent does not lift the standard that supervisors do not counsel supervisees.

D. Help her find appropriate services and address the issue only as it affects her work ✓ **KEY**

F.6.c: supervisors help locate services and do not provide counseling to supervisees.

Rationale: ACA Standard F.6.c states that when supervisees request counseling, supervisors help them identify appropriate services, and supervisors do not provide counseling to supervisees. Supervisors address personal concerns only as they affect clients, the supervisory relationship and professional functioning. Counseling a person one also evaluates creates a harmful dual role.

References: ACA Code of Ethics — F.6.c Counseling for Supervisees

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor licensed for two years is asked by her agency to begin providing clinical supervision to three new graduates seeking licensure. She has strong clinical skills but has had no coursework or training in supervision. Under the ACA Code of Ethics, what should happen first?

A. She may start now, since her strong clinical skills will carry over into supervision

Clinical skill alone does not equal competence in supervision methods and techniques.

B. She should get training in supervision methods and techniques before she supervises ✓ KEY

F.2.a requires training in supervision methods and techniques before offering supervision.

C. She may start now if the supervisees agree in writing to accept an untrained supervisor

Supervisee agreement does not meet the preparation standard or protect their clients.

D. She may start now, as the agency has the final say over who supervises its own staff

Agency assignment does not override the counselor's duty to be competent in supervision.

Rationale: ACA Standard F.2.a, Supervisor Preparation, states that before offering supervision, counselors are trained in supervision methods and techniques, and that supervisors regularly pursue continuing education in both counseling and supervision. Many state boards add their own training and experience rules for approved supervisors.

References: ACA Code of Ethics — F.2.a Supervisor Preparation

☐ Approve ☐ Revise ☐ Retire Notes:

A licensed professional counselor moves to a new state that has not joined the Counseling Compact. She wants to keep seeing clients who live in the new state. What must she do before she sees clients there?

A. Obtain a license from the new state, often by an endorsement path for licensed counselors ✓ KEY

Licensure is state-based; without the Compact, she needs the new state's license before practicing.

B. Nothing, since a counseling license from any one state is valid across the whole country

There is no national counseling license; each state regulates practice within its borders.

C. Only notify her old state's board, which will forward her license to the new state's board

Boards do not transfer licenses; the counselor applies to the new state herself.

D. Hold the NCC credential, which allows practice in any state while a license is pending

The NCC is a voluntary certification and grants no legal authority to practice.

Rationale: Counselor licenses are granted and regulated by individual states. A counselor who moves to a state outside the Counseling Compact must apply for that state's license, often through an endorsement or reciprocity path for counselors already licensed elsewhere, and must meet that state's requirements before practicing. Compact member states instead offer a privilege to practice.

References: State law — Licensure by endorsement for counselors licensed in another state · Counseling Compact — Privilege to practice limited to member states

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor who is a Black, queer woman begins working with a wealthy, white, heterosexual male executive who questions whether she is qualified. In the Multicultural and Social Justice Counseling Competencies, which quadrant describes this counseling relationship?

A. Privileged counselor with a marginalized client

This reverses the identities; here the counselor holds the marginalized statuses.

B. Privileged counselor with a privileged client

The counselor holds several marginalized identities, so this quadrant does not fit.

C. Marginalized counselor with a marginalized client

The client holds privileged identities in race, gender, class and sexual orientation.

D. Marginalized counselor with a privileged client ✓ KEY

The counselor holds marginalized identities and the client holds privileged ones.

Rationale: The MSJCC model places counselor and client in four quadrants based on privileged and marginalized identities: privileged counselor with marginalized client, privileged counselor with privileged client, marginalized counselor with privileged client, and marginalized counselor with marginalized client. Recognizing the quadrant helps the counselor anticipate how power and oppression may shape trust, communication and the client's challenge to her competence.

References: MSJCC (Ratts et al., 2016) — Quadrants: marginalized counselor and privileged client

☐ Approve ☐ Revise ☐ Retire Notes:

Q23 of 200 · nce-s-eth-167 · Professional Counseling Orientation and Ethical Practice · counselor advocacy and social justice · easy · In review

A counselor in private practice sets aside two sliding-scale slots each week for clients who cannot afford her fee, and she gives free talks on stress at a local library. Which ACA Code of Ethics standard does this MOST directly reflect?

A. C.5 Nondiscrimination

C.5 bars discrimination against clients and others; it does not ask counselors to give low-cost services.

B. C.6.e Contributing to the Public Good ✓ KEY

C.6.e asks counselors to offer some services for little or no financial return, such as talks or reduced fees.

C. C.2.f Continuing Education

C.2.f concerns keeping knowledge current, not offering services at little or no cost.

D. A.7.a Advocacy

A.7.a concerns advocating to remove barriers for clients, not providing low-cost services.

Rationale: ACA Standard C.6.e, Contributing to the Public Good (Pro Bono Publico), states that counselors make a reasonable effort to provide services to the public for which there is little or no financial return, such as speaking to groups, sharing professional information or offering reduced fees. It reflects the profession's responsibility to society as well as to individual clients.

References: ACA Code of Ethics — C.6.e Contributing to the Public Good (Pro Bono Publico)

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor starting a private practice wants more online visibility. She plans to email current clients asking them to post five-star reviews of her services on a popular review website. Under the ACA Code of Ethics, what is the problem with this plan?

A. None, as long as the clients may choose to post their reviews under a screen name

Screen names do not fix the problem of soliciting endorsements from current clients.

B. None, because online reviews are not advertising and fall outside the Code's scope

Requested reviews used to attract clients function as testimonials and advertising.

C. Counselors may not seek testimonials from current clients, who may feel pressured ✓ KEY

C.3.b bars soliciting testimonials from current or former clients or others open to undue influence.

D. It is fine only if she offers clients a reduced session fee in exchange for reviews

Paying for reviews adds undue influence and makes the solicitation more problematic.

Rationale: ACA Standard C.3.b states that counselors who use testimonials do not solicit them from current clients, former clients or any other persons who may be vulnerable to undue influence. Clients may feel unable to refuse a request from their counselor, and posting a review can expose the fact that they are in counseling. Counselors also avoid revealing client information when responding to online reviews.

References: ACA Code of Ethics — C.3.b Testimonials · ACA Code of Ethics — H.6.d Use of Public Social Media

☐ Approve

☐ Revise

☐ Retire

Notes:

Intake, Assessment, and Diagnosis

24 items · NCE weight 12% (19 of 160 scored items)

Q25 of 200 · nce-s-ass-145 · Counseling and Helping Relationships · intake interview structure · medium · In review

A 52-year-old Korean American client tells the counselor at intake that she has slept poorly and "worried constantly" for nearly ten years. She has never sought help before. Which question would BEST identify what prompted her to seek counseling now?

A. What led you to decide to come in for help at this point? ✓ KEY

Asking "why now" draws out the precipitating event or tipping point that turned a long-standing problem into a reason to seek help.

B. How would you rate your worry on a scale from 1 to 10?

A scaling question gauges current severity but does not reveal what changed to bring her in.

C. Did anyone in your family also have trouble with worry?

Family history belongs in the intake, but it addresses vulnerability, not the recent precipitant.

D. What have you already tried to manage the worry yourself?

Past coping attempts are useful to know but do not explain why she is seeking help at this moment.

Rationale: When a problem is long-standing, the intake should explore what has changed recently, such as a new stressor, a loss, a worsening of symptoms, or pressure from others. The precipitant often reveals the client's most urgent concern, her expectations for counseling, and possible risk. It also helps the counselor and client agree on initial goals.

References: Hays (Assessment) — Clinical interviewing: presenting problem, onset and precipitating events

☐ Approve ☐ Revise ☐ Retire Notes:

Q26 of 200 · nce-s-ass-146 · Assessment and Testing · mental status examination · easy · In review

During an interview, a 33-year-old client shifts within a minute from laughing loudly to sobbing and then to shouting angrily, with no change in topic that would explain the shifts. Which term BEST describes this client's affect in the mental status examination?

A. Blunted

Blunted affect is a marked reduction in the intensity of emotional expression, the opposite of this presentation.

B. Labile ✓ KEY

Labile affect is rapid, abrupt shifting between emotional states that are out of proportion to what is happening.

C. Constricted

Constricted affect is a narrowed range of emotional expression, not rapid swings across a wide range.

D. Incongruent

Incongruent affect is a mismatch between expressed emotion and thought content; the hallmark here is rapid shifting.

Rationale: The MSE describes affect by its range, intensity, stability, and appropriateness to content. Lability concerns stability: emotions change quickly and dramatically without an adequate trigger. It can appear in mania, intoxication, some neurological conditions, and personality disorders, so it is described and then considered alongside the rest of the assessment.

References: Hays (Assessment) — Mental status examination: affect (range, intensity, stability, appropriateness)

☐ Approve ☐ Revise ☐ Retire Notes:

While describing his week, a 24-year-old client stops abruptly in the middle of a sentence and stays silent for several seconds. When the counselor asks what happened, he says his mind "went completely empty" and he cannot recall what he was saying. How should the counselor record this in the mental status examination?

A. Circumstantiality in thought process

Circumstantiality is excessive detail that eventually reaches the point, not a sudden halt in thought.

B. Poverty of speech, reduced output

Poverty of speech is a general reduction in the amount of speech, not an abrupt break mid-sentence.

C. Thought blocking in thought process ✓ KEY

Thought blocking is a sudden interruption in the train of thought, often with the person unable to recall what was being said.

D. Psychomotor slowing in behavior

Psychomotor slowing is general slowing of movement and speech, not a sudden stop in the flow of thought.

Rationale: Thought process describes how ideas are connected and how they flow. Thought blocking is a sudden stop in that flow, after which the person may report that the thought vanished. It is often associated with psychotic disorders but can also occur with severe anxiety, so the counselor records the observation and considers it with other findings rather than diagnosing from it alone.

References: Hays (Assessment) — Mental status examination: thought process (blocking, circumstantiality, tangentiality)

☐ Approve ☐ Revise ☐ Retire Notes:

A 31-year-old client is being discharged after a week on an inpatient psychiatric unit following a suicide attempt. He says he feels "much better" and is eager to return to work. The community counselor is scheduling follow-up. Which statement about his suicide risk is MOST accurate?

A. His risk has returned to his baseline level, since he now reports better mood

Reported improvement does not cancel the recent attempt, which remains one of the strongest risk factors.

B. His risk is low now, because inpatient care has resolved the underlying crisis

A short hospital stay stabilizes acute danger but rarely resolves the stressors he returns to.

C. His risk can be judged mainly by how eager he now seems to return to his job

Eagerness is one observation, not a basis for a risk judgment; a full reassessment is needed.

D. His risk stays high in the weeks after discharge, so prompt follow-up is needed ✓ KEY

The period right after psychiatric discharge carries markedly elevated suicide risk, so early contact and a safety plan are essential.

Rationale: Suicide risk is especially high in the days and weeks after discharge from inpatient care, when people return to the stressors that preceded the crisis with less support. A recent attempt is a major risk factor even when mood appears improved. Best practice is a follow-up appointment soon after discharge, an updated safety plan, and attention to lethal means.

References: James & Gilliland (Crisis Intervention Strategies) — Suicide: risk assessment, prior attempts and periods of heightened risk · Stanley-Brown SPI — Safety planning and follow-up contact for people leaving emergency or inpatient care

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is assessing violence risk in a 29-year-old client and wants the plan to target factors that treatment could change. Which of the following is a DYNAMIC risk factor for violence?

A. Current heavy alcohol use ✓ KEY

Active substance use is a changeable (dynamic) factor that treatment and monitoring can target.

B. A history of violent arrests

Past violence is a strong predictor but a static factor; history cannot be changed.

C. Childhood exposure to abuse

Childhood abuse is a historical, static factor that remains fixed regardless of treatment.

D. Early age at first offense

Age at first offense is fixed in the past, so it is a static factor.

Rationale: Risk factors are commonly divided into static factors, which are historical and unchangeable, and dynamic factors, which can change over time and with intervention. Static factors such as prior violence help estimate baseline risk, while dynamic factors such as substance use, acute symptoms, access to weapons, and current anger guide treatment planning and monitoring.

References: James & Gilliland (Crisis Intervention Strategies) — Assessing violence potential: historical versus current, changeable risk factors

☐ Approve ☐ Revise ☐ Retire Notes:

A 46-year-old client seeks counseling because of frequent arguments with his husband about money and household roles. After a thorough assessment, the counselor finds that he does not meet criteria for any mental disorder. How can the counselor document the focus of counseling using DSM-5-TR?

A. Diagnose adjustment disorder, since some distress is present

Adjustment disorder requires clinically significant distress or impairment beyond what the stressor would explain; distress alone is not enough.

B. Record relationship distress from the Z-code conditions ✓ KEY

DSM-5-TR lists Relationship Distress With Spouse or Intimate Partner among conditions that may be a focus of clinical attention.

C. Use unspecified anxiety disorder so the service can be billed

Assigning a diagnosis that the assessment does not support misrepresents the client's condition.

D. Leave the record blank until a disorder appears in a later session

The reason for services should be documented; DSM-5-TR provides codes for non-disorder concerns.

Rationale: DSM-5-TR includes a chapter on Other Conditions That May Be a Focus of Clinical Attention, most coded with ICD-10-CM Z codes. These are not mental disorders but describe problems, such as relationship distress, that bring a person to counseling or affect care. Using them lets the counselor document the focus accurately without assigning an unsupported diagnosis.

References: DSM-5-TR — Other Conditions That May Be a Focus of Clinical Attention: Relationship Distress With Spouse or Intimate Partner (Z63.0)

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old woman who met criteria for severe alcohol use disorder has spent the past five months in a residential program where alcohol is not available. Since admission she has met no criteria for the disorder except continued craving. Which DSM-5-TR course specifiers should accompany the diagnosis?

A. In sustained remission, in a controlled environment

Sustained remission requires 12 months or longer without meeting criteria other than craving; she is at five months.

B. In early remission, with no further specifier needed

An added specifier is used when the person is in a setting where access to the substance is restricted.

C. In sustained remission, with craving noted separately

The time frame is wrong, and craving is already the one criterion allowed during remission.

D. In early remission, in a controlled environment ✓ **KEY**

Three to under 12 months with no criteria met except craving is early remission, and residential restriction adds the controlled-environment specifier.

Rationale: For substance use disorders, DSM-5-TR defines early remission as at least 3 but less than 12 months without meeting any criterion except craving, and sustained remission as 12 months or longer. The specifier "in a controlled environment" is added when the person is in a setting where access to the substance is restricted. It signals that remission has not yet been tested in an unrestricted setting.

References: DSM-5-TR — Alcohol Use Disorder: specifiers in early remission, in sustained remission, in a controlled environment

☐ Approve ☐ Revise ☐ Retire Notes:

During an intake, a 27-year-old client says his neighbors keep "trinkling the walls with glorphic waves." Asked what he means, he explains that "glorphic" is a kind of energy only he can sense and "trinkling" is how they send it. In the mental status examination, how should the counselor describe these invented words?

A. Clang associations

Clang associations link words by sound or rhyme rather than meaning; these are newly coined words.

B. Echolalia

Echolalia is repeating another person's words or phrases, not inventing new ones.

C. Neologisms ✓ **KEY**

Neologisms are new words a person creates, often with private meanings known only to the speaker.

D. Circumstantiality

Circumstantiality is excessive, roundabout detail that eventually reaches the point, not invented vocabulary.

Rationale: The MSE records the form of speech and thought separately from its content. Neologisms are invented words, or ordinary words given idiosyncratic meanings, and they are a sign of disorganized thinking most often seen in psychotic disorders. The counselor quotes examples in the record, since specific wording lets other clinicians judge the finding, and considers it alongside thought content such as the belief that neighbors are sending energy.

References: Hays (Assessment) — Mental status examination: speech and thought process (neologisms, clang associations, echolalia)

☐ Approve ☐ Revise ☐ Retire Notes:

Six weeks after giving birth, a 30-year-old client reports sudden, vivid images of dropping her baby down the stairs. She finds the images horrifying, has no wish to harm the infant, avoids carrying him near stairs, and checks many times a night that he is breathing. Her mood is otherwise stable. Which diagnosis do these symptoms MOST suggest?

A. Postpartum psychosis

She recognizes the images as unwanted and senseless; there are no delusions, hallucinations, or disorganization.

B. Generalized anxiety disorder

GAD worry is about realistic everyday matters, not intrusive, unwanted images followed by checking and avoidance.

C. Depression with peripartum onset

Her mood is stable, and the presentation centers on intrusive images and compulsive checking.

D. Obsessive-compulsive disorder ✓ KEY

Intrusive, unwanted harm images that cause distress, plus checking and avoidance meant to prevent harm, are obsessions and compulsions.

Rationale: Obsessions are intrusive, unwanted thoughts, urges, or images that cause marked distress, and the person tries to ignore, suppress, or neutralize them, often with compulsions. Unwanted thoughts of accidentally or deliberately harming an infant are common in perinatal OCD and are experienced as repugnant, which distinguishes them from psychotic thinking. Recognizing this pattern prevents both missed diagnosis and unwarranted alarm about intent to harm.

References: DSM-5-TR — Obsessive-Compulsive Disorder: obsessions, compulsions and differential diagnosis with anxiety and psychotic disorders

☐ Approve ☐ Revise ☐ Retire Notes:

A clerical speed test has 200 very easy items and a strict 5-minute time limit; scores depend almost entirely on how many items a person reaches. The developer reports an odd-even split-half reliability of .97. What is the main concern with this estimate?

A. Split-half methods inflate reliability for speeded tests ✓ KEY

On a pure speed test, odd and even half scores are nearly identical for each person, so the split-half coefficient is spuriously high.

B. Split-half methods underestimate reliability for easy items

The problem is overestimation, and it comes from the time limit, not item ease alone.

C. Odd-even splits are only valid for tests with fewer items

Test length is not the issue; a long power test can be split this way appropriately.

D. A value of .97 shows the test has strong construct validity

A reliability coefficient, even a correct one, is not evidence of construct validity.

Rationale: Internal consistency methods assume that performance reflects item content rather than how far a person gets. On a speeded test, almost every attempted item is correct, so any odd-even split produces near-identical halves and an inflated coefficient. Test-retest or alternate-form reliability, or split halves timed separately, give more appropriate estimates for speeded tests.

References: Hays (Assessment) — Reliability: internal consistency estimates and speeded versus power tests

☐ Approve ☐ Revise ☐ Retire Notes:

A graduate program studies the reliability of a reasoning test using only its admitted students, all of whom scored in the top 10% on that test. The reliability coefficient comes out much lower than the one reported in the test manual. What is the MOST likely explanation?

A. Admitted students answered the items more carelessly

Nothing suggests careless responding; the sample's score range explains the drop.

B. The narrow range of scores lowered the correlation ✓ **KEY**

Restricting the sample to top scorers shrinks score variance, and correlation-based coefficients fall when variance is restricted.

C. The test manual must have reported its values in error

The manual's value came from a broader norm sample, so the two estimates are not directly comparable.

D. Higher-scoring examinees are less consistent over time

High scorers are not inherently less consistent; the lower value reflects the limited range.

Rationale: Reliability coefficients are correlations, and correlations depend on the variability of scores in the sample. When a sample includes only a narrow slice of the score range, true-score differences shrink relative to error, and the coefficient drops. Counselors should compare their clients with the sample used to estimate reliability before relying on a manual's figure.

References: Hays (Assessment) — Factors affecting reliability: heterogeneity of the sample and restriction of range · Erford (Research and Evaluation in Counseling) — Correlation: restriction of range

☐ Approve ☐ Revise ☐ Retire Notes:

A clinic already predicts treatment dropout from a brief intake interview. Before adding a 45-minute questionnaire to every intake, the director wants to know whether the questionnaire improves prediction beyond what the interview already provides. Which type of validity evidence is the director asking about?

A. Concurrent validity

Concurrent validity compares scores with a criterion measured at the same time, not added predictive value.

B. Content validity

Content validity concerns how well items sample a domain, not prediction of dropout.

C. Incremental validity ✓ **KEY**

Incremental validity is the improvement in prediction a measure adds over information already available.

D. Convergent validity

Convergent validity is a strong correlation with other measures of the same construct.

Rationale: Incremental validity asks whether a new measure explains additional variance in a criterion after existing predictors are accounted for, often tested with hierarchical regression. A measure can be valid on its own yet add little if it overlaps with what is already known. It is a practical question whenever extra testing costs client and staff time.

References: Hays (Assessment) — Validity: criterion-related evidence and incremental validity

☐ Approve ☐ Revise ☐ Retire Notes:

To set the passing score on a licensure exam, a panel of experts estimates, for each item, the probability that a minimally competent practitioner would answer it correctly, and the estimates are summed into a cut score. What does this approach mean for how candidates' results are interpreted?

A. A fixed share of candidates will fail each time the exam is given

A fixed failure rate is a norm-referenced approach; here anyone who meets the standard passes.

B. Each candidate's score is reported as a percentile within the cohort

Percentile ranks compare candidates with each other, which this cut-score method does not do.

C. Passing depends mostly on how well others in the same cohort score

The cut score is fixed in advance from item judgments, not from the cohort's performance.

D. Passing depends on meeting a set standard, not on others' scores ✓ KEY

An expert-judged cut score makes the decision criterion-referenced: each candidate is compared with a standard of competence.

Rationale: This is the Angoff method of standard setting, widely used for licensure and certification exams. Because the cut score reflects a judged level of minimum competence, results are interpreted against that standard, so in principle every candidate could pass or fail. Norm-referenced interpretation, by contrast, locates a person relative to a comparison group.

References: Hays (Assessment) — Criterion-referenced interpretation: setting cut scores (Angoff method)

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor graphs scores from a brief screening scale and sees that, compared with a normal curve, most scores bunch tightly around the mean, forming a tall, narrow peak with heavier tails. Which term describes the shape of this distribution?

A. Leptokurtic ✓ KEY

A leptokurtic distribution is more sharply peaked than the normal curve, with scores clustered near the mean and heavier tails.

B. Platykurtic

A platykurtic distribution is flatter than normal, with scores spread more evenly across the range.

C. Positively skewed

Skew describes asymmetry, with a long tail on one side, not how peaked the curve is.

D. Bimodal

A bimodal distribution has two distinct peaks, not one tall, narrow peak.

Rationale: Kurtosis describes how peaked or flat a distribution is relative to the normal (mesokurtic) curve. Leptokurtic distributions have a tall, narrow center and heavier tails; platykurtic distributions are flat and broad. Kurtosis and skewness are separate properties: one concerns peakedness, the other symmetry.

References: Erford (Research and Evaluation in Counseling) — Descriptive statistics: shape of distributions, kurtosis (leptokurtic, mesokurtic, platykurtic)

☐ Approve ☐ Revise ☐ Retire Notes:

A parent asks a school counselor what the "standard error of measurement" printed on her son's test report means. Which explanation is accurate?

A. How far the class average falls from the national average

That describes a group comparison, not the precision of one student's score.

B. How much his score would vary if he took the test many times ✓ **KEY**

The SEM estimates the spread of a person's obtained scores around the true score across hypothetical repeated testings.

C. How much of his score reflects effort rather than ability

The SEM does not separate effort from ability; it reflects random measurement error.

D. How many items on his test were scored wrongly by mistake

Scoring mistakes are not what the SEM measures; it is a statistical estimate from reliability.

Rationale: The standard error of measurement is the standard deviation of the scores a person would obtain if tested many times, assuming no practice or fatigue. It is calculated from the test's standard deviation and reliability and is used to build confidence bands around an obtained score. A smaller SEM means a more precise score.

References: Hays (Assessment) — Standard error of measurement: definition and use in confidence intervals

☐ Approve ☐ Revise ☐ Retire Notes:

On an intelligence test, a 68-year-old retired teacher scores well above average on vocabulary and general knowledge subtests but only average on timed tasks that require solving novel visual puzzles. In Cattell's theory, the vocabulary and knowledge tasks mainly measure which ability?

A. Fluid intelligence

Fluid intelligence is reasoning with novel problems, as in the timed visual puzzles.

B. Working memory

Working memory is holding and manipulating information in mind briefly, not stored knowledge.

C. Crystallized intelligence ✓ **KEY**

Crystallized intelligence is accumulated knowledge and vocabulary gained through education and experience.

D. Processing speed

Processing speed is how quickly simple tasks are done; vocabulary and knowledge are not speed measures.

Rationale: Cattell, later with Horn, distinguished fluid intelligence (Gf), the capacity to reason and solve new problems, from crystallized intelligence (Gc), knowledge built through culture and schooling. Crystallized abilities tend to hold steady or grow into later adulthood, while fluid abilities and speed decline earlier. This pattern helps explain the profile of this older client.

References: Berk (Development Through the Lifespan) — Middle and late adulthood: crystallized and fluid intelligence · Hays (Assessment) — Theories of intelligence: Cattell-Horn fluid and crystallized abilities

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor at an outpatient clinic wants a self-report inventory designed for adults already in mental health treatment. It should focus on personality patterns and clinical syndromes and report base rate scores rather than T-scores. Which instrument fits?

A. NEO Personality Inventory-3

The NEO-PI-3 measures the five-factor traits in the general population and reports T-scores.

B. California Psychological Inventory

The CPI assesses everyday interpersonal traits in nonclinical populations.

C. The 16PF Questionnaire, 5th edition

The 16PF measures normal-range personality factors and uses sten scores.

D. Millon Clinical Multiaxial Inventory-IV ✓ KEY

The MCMI-IV is normed on adults in treatment, targets personality patterns and clinical syndromes, and uses base rate scores.

Rationale: The MCMI-IV is built on Millon's theory of personality and normed on clinical samples, so it is meant for people already presenting for mental health care, not for screening the general public. Its base rate scores adjust for how common each pattern is in clinical populations. Instruments such as the NEO-PI-3, CPI, and 16PF describe normal-range personality instead.

References: Hays (Assessment) — Personality assessment: Millon Clinical Multiaxial Inventory and base rate scores

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor wants a storytelling projective technique for a 5-year-old, using pictures designed for young children in which the characters are animals in human-like situations. Which instrument fits this description?

A. Children's Apperception Test ✓ KEY

The CAT, developed by Bellak and Bellak for young children, presents animal figures in human situations for storytelling.

B. Rotter Incomplete Sentences Blank

This is a sentence completion technique, not picture-based storytelling.

C. Rorschach Inkblot Test

The Rorschach uses inkblots that the person describes, not pictures of characters.

D. Kinetic Family Drawing

The KFD asks the child to draw the family doing something, not to tell stories about pictures.

Rationale: Apperception tests ask people to tell stories about ambiguous pictures, on the assumption that they project their own needs and conflicts into the stories. The Children's Apperception Test adapted the TAT approach for young children by using animal figures, which were thought to be easier for children to identify with; a later version uses human figures. Like other projectives, it is interpreted cautiously and alongside other data.

References: Hays (Assessment) — Projective techniques: apperception tests (TAT and Children's Apperception Test)

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor uses the CAGE questionnaire with a 58-year-old client. He answers "yes" to having felt he should cut down on his drinking and to having had a drink first thing in the morning to steady his nerves. How should this result be interpreted?

A. It confirms that he has an alcohol use disorder diagnosis

A screen identifies possible problems; diagnosis requires a full assessment against DSM-5-TR criteria.

B. It is a positive screen that calls for fuller assessment ✓ **KEY**

Two or more "yes" answers on the CAGE is generally considered clinically significant and warrants further assessment.

C. It is negative, because a positive screen needs all four

The usual cutoff is two of four items, not all four.

D. It shows low risk, since only half the items were endorsed

Two endorsements meet the usual cutoff, and a morning eye-opener is especially concerning.

Rationale: The CAGE asks four yes-or-no questions about Cutting down, Annoyance at criticism, Guilt, and an Eye-opener drink. A score of 2 or more is the usual cutoff for a positive screen. Like other screening tools, it flags people who need a fuller assessment rather than establishing a diagnosis.

References: Hays (Assessment) — Substance use screening instruments: CAGE scoring and cutoff

☐ Approve ☐ Revise ☐ Retire Notes:

Q44 of 200 · nce-s-ass-164 · Research and Program Evaluation · screening instruments (PHQ-9, GAD-7, AUDIT, CAGE) · medium · In review

A college counseling center chooses a depression screener with very high sensitivity and only moderate specificity. Which outcome should staff expect when they use it?

A. Few students it flags will turn out not to be depressed

Few false positives would reflect high specificity, which this screener lacks.

B. Many students with depression will be missed by the screen

Missing many true cases describes low sensitivity, the opposite of this tool.

C. Few depressed students are missed, but some flags are false ✓ **KEY**

High sensitivity means few false negatives; moderate specificity means a fair number of false positives.

D. Most negative results will need follow-up to be confirmed

With high sensitivity, negative results are fairly trustworthy; positives are what need follow-up.

Rationale: Sensitivity is the proportion of people with a condition whom the test correctly flags, and specificity is the proportion without it whom the test correctly clears. Screening tools are usually tuned for high sensitivity so that few true cases are missed, accepting more false positives. That is why a positive screen is followed by a fuller diagnostic assessment.

References: Hays (Assessment) — Screening: sensitivity, specificity and false positives

☐ Approve ☐ Revise ☐ Retire Notes:

Before a timed math achievement test, a proctor tells a group of high school students that the test "tends to show differences between boys and girls." Girls in this group score lower than equally prepared girls who took the test without the remark. Which phenomenon BEST explains the difference?

A. Test-retest error

Test-retest error concerns score changes across repeated testing, not a remark made before one session.

B. The Flynn effect

The Flynn effect is a rise in population test scores over decades.

C. Regression to the mean

Regression to the mean affects extreme scorers on retesting, not equally prepared groups tested once.

D. Stereotype threat ✓ **KEY**

Making a negative group stereotype salient can raise anxiety and depress performance among members of that group.

Rationale: Stereotype threat, described by Steele and Aronson, occurs when people risk confirming a negative stereotype about their group, and the resulting concern lowers their performance. It shows that test scores can be influenced by the testing context, not only by ability. Culturally fair assessment includes administering tests in conditions that avoid cueing group stereotypes.

References: Sue & Sue (Counseling the Culturally Diverse) — Stereotype threat and its effect on test performance · Hays (Assessment) — Assessment of diverse populations: test-taker and situational factors

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor gathers teacher ratings for a 12-year-old referred for possible inattention. One teacher, who rates most students across the full range, describes this boy as polite and a pleasure to have in class and rates him excellent on every scale, including organization and task completion, where his work samples show clear problems. Which rating error is MOST likely?

A. Halo effect ✓ **KEY**

A favorable overall impression from one trait, politeness, is spilling over into ratings of unrelated behaviors.

B. Central tendency error

Central tendency error is clustering ratings in the middle of the scale, not rating everything excellent.

C. Leniency error

Leniency is rating all ratees favorably; this teacher uses the full range with other students.

D. Contrast error

Contrast error is rating someone in relation to a person rated just before, which is not described here.

Rationale: Rating scales completed by teachers, parents, or other informants are vulnerable to rater errors. The halo effect occurs when a general impression, positive or negative, colors ratings of specific, unrelated traits. Counselors reduce its impact by gathering ratings from several informants, checking them against work samples and observations, and using scales with behaviorally specific items.

References: Hays (Assessment) — Rating scales and behavioral observation: rater errors (halo, leniency, central tendency)

☐ Approve ☐ Revise ☐ Retire Notes:

A 44-year-old client diagnosed with major depressive disorder, single episode, has improved over three months of treatment. She no longer meets full criteria for a major depressive episode, but she still has low energy and some trouble concentrating most days. Which DSM-5-TR specifier describes her current status?

A. In full remission

Full remission requires no significant signs or symptoms of the disturbance for the past 2 months.

B. In partial remission ✓ KEY

Some symptoms of the episode remain, but full criteria are no longer met, which defines partial remission.

C. Recurrent episode, mild

She has not had a new episode; "recurrent" requires a second episode after an interval of remission.

D. With melancholic features

Melancholic features describe the symptom pattern of an episode, not its remission status.

Rationale: DSM-5-TR uses remission specifiers when criteria for a major depressive episode are no longer met. Partial remission means symptoms of the immediately previous episode are present but full criteria are not met, or there has been a period of less than 2 months without significant symptoms. Full remission requires 2 months with no significant signs or symptoms. Residual symptoms matter clinically because they raise the risk of relapse.

References: DSM-5-TR — Major Depressive Disorder: remission specifiers (in partial remission, in full remission)

☐ Approve ☐ Revise ☐ Retire Notes:

During a substance use assessment, a client says she drinks "two big glasses of wine" each evening. To record her intake accurately, the counselor converts it into standard drinks. In the United States, one standard drink of table wine is about how much?

A. 12 ounces

Twelve ounces is a standard drink of regular beer (about 5% alcohol), not wine.

B. 8 ounces

Eight ounces is roughly a standard drink of malt liquor, not table wine.

C. 5 ounces ✓ KEY

About 5 ounces of wine at roughly 12% alcohol contains one standard drink (about 14 grams of pure alcohol).

D. 1.5 ounces

One and a half ounces is a standard drink of 80-proof distilled spirits.

Rationale: A U.S. standard drink contains about 14 grams of pure alcohol: 12 ounces of regular beer, 5 ounces of table wine, or 1.5 ounces of 80-proof spirits. Clients often pour larger servings than they realize, so a "big glass" of wine may hold two or more standard drinks. Converting reported use into standard drinks makes quantity estimates comparable across assessment sessions and tools.

References: Sobell & Sobell (Timeline Followback) — Recording daily use in standard drink units · Hays (Assessment) — Substance use assessment: quantifying consumption

☐ Approve ☐ Revise ☐ Retire Notes:

Areas of Clinical Focus

59 items · NCE weight 29% (47 of 160 scored items)

Q49 of 200 · nce-s-clf-355 · Assessment and Testing · depressive disorders · medium · In review

A 58-year-old in a severe major depressive episode tells the counselor that she has committed unforgivable sins, has ruined her family and deserves to be punished. Her family says none of this is true, and no evidence changes her mind. Which DSM-5-TR specifier BEST fits this episode?

A. With melancholic features, given her loss of pleasure and her guilt

Melancholic features describe anhedonia and vegetative signs; they do not account for fixed false beliefs.

B. With mood-congruent psychotic features, as the delusions fit her mood ✓ **KEY**

Fixed beliefs of sin, ruin and deserved punishment are delusions whose content matches depressive themes.

C. With mood-incongruent psychotic features, due to her false beliefs

Mood-incongruent delusions have content unrelated to depressive themes, such as thought insertion.

D. With anxious distress, since she fears being punished for her sins

Anxious distress covers tension, restlessness and dread, not fixed beliefs held despite clear evidence.

Rationale: DSM-5-TR adds "with psychotic features" when delusions or hallucinations occur during a major depressive episode. The features are mood-congruent when their content reflects typical depressive themes such as guilt, deserved punishment, disease, nihilism or personal inadequacy. Psychotic depression calls for prompt psychiatric evaluation, because it carries higher risk and usually needs combined medication treatment.

References: DSM-5-TR — Depressive disorders specifiers: with mood-congruent vs mood-incongruent psychotic features

☐ Approve ☐ Revise ☐ Retire Notes:

Q50 of 200 · nce-s-clf-356 · Assessment and Testing · depressive disorders · hard · In review

Five weeks after his wife died in a car crash, a 49-year-old reports depressed mood nearly all day, loss of interest, insomnia, poor concentration and constant feelings of worthlessness, and he cannot work. A colleague says major depressive disorder cannot be diagnosed during bereavement. How should the counselor respond?

A. Agree, since DSM-5-TR bars the diagnosis within 2 months of a death

This was the DSM-IV bereavement exclusion, which was removed in DSM-5 and is not in DSM-5-TR.

B. Agree, since grief must last 12 months before any disorder can apply

The 12-month rule belongs to prolonged grief disorder in adults, not to major depressive disorder.

C. Diagnose adjustment disorder instead, because a stressor came first

Adjustment disorder is not used when the criteria for another specific disorder, such as MDD, are met.

D. Disagree, as MDD may be diagnosed when full criteria are met after a loss ✓ **KEY**

DSM-5-TR lets clinicians diagnose MDD after a death when full criteria are met, using clinical judgment.

Rationale: DSM-5 removed the bereavement exclusion, and DSM-5-TR keeps that change: a major depressive episode can be diagnosed after a loss when full criteria are met. Clinicians use judgment to tell grief from depression. In grief, painful feelings come in waves tied to reminders and self-esteem is usually preserved, while pervasive low mood and feelings of worthlessness, as in this client, point toward a major depressive episode.

References: DSM-5-TR — Major depressive disorder: note on distinguishing grief from a major depressive episode

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old graduate student with bipolar I disorder has been stable for a year. With his counselor, he is writing a relapse-prevention plan that lists early warning signs of mania. Which early sign is MOST commonly reported before a manic relapse and belongs at the top of his list?

A. Sleeping much less while feeling rested and full of energy ✓ KEY

A drop in sleep without fatigue is the most frequently reported early sign of an oncoming manic episode.

B. Losing interest in hobbies and pulling away from his friends

Withdrawal and loss of interest are early signs of a depressive episode rather than mania.

C. Waking very early and feeling at his worst in the morning

Early waking with morning worsening is a melancholic depressive pattern, not a manic prodrome.

D. Feeling numb and detached from his body and surroundings

Depersonalization and derealization are dissociative experiences, not typical signs of mania.

Rationale: Decreased need for sleep is both a core manic symptom and the most common early warning sign of a manic relapse. Disrupted sleep can also trigger episodes, which is why interpersonal and social rhythm therapy works to keep sleep and daily routines steady. Relapse plans list a person's own early signs and the steps to take, such as contacting the prescriber, when they appear.

References: IPSRT — Sleep and social rhythm disruption as a trigger and early warning sign of mania · DSM-5-TR — Manic episode Criterion B: decreased need for sleep

☐ Approve ☐ Revise ☐ Retire Notes:

For 5 days, a 26-year-old has had a persistently irritable, but not elevated or expansive, mood with increased energy. She also shows decreased need for sleep, pressured speech and distractibility. Under DSM-5-TR, how many Criterion B symptoms does a hypomanic episode require in her case?

A. Two, since her energy is also increased

Increased energy is part of Criterion A and does not lower the Criterion B count.

B. Three, as for any hypomanic episode

Three symptoms suffice only when mood is elevated or expansive, not irritable alone.

C. Four, since her mood is only irritable ✓ KEY

When mood is only irritable, DSM-5-TR requires four Criterion B symptoms instead of three.

D. Five, the same as a depressive episode

Five of nine symptoms is the threshold for a major depressive episode, not hypomania.

Rationale: For both manic and hypomanic episodes, DSM-5-TR requires three or more Criterion B symptoms, or four if the mood is only irritable. The higher bar for irritable mood guards against labeling irritability from other causes as hypomania. With three symptoms and irritable mood, this client does not yet meet the threshold, so the counselor keeps assessing rather than diagnosing.

References: DSM-5-TR — Hypomanic episode Criterion B: three symptoms, or four if the mood is only irritable

☐ Approve ☐ Revise ☐ Retire Notes:

For over a year, a 40-year-old has felt intense fear whenever she is on a balcony, a bridge or a scenic overlook, and she now plans trips to avoid high places. If specific phobia is diagnosed, which DSM-5-TR specifier applies?

A. Situational

Situational phobias involve settings such as airplanes, elevators and enclosed places.

B. Animal

Animal phobias involve creatures such as spiders, dogs or snakes.

C. Blood-injection-injury

This type involves needles, blood, injuries or invasive medical procedures.

D. Natural environment ✓ KEY

DSM-5-TR lists heights, storms and water as examples of the natural environment type.

Rationale: DSM-5-TR specifies the phobic stimulus for specific phobia as animal, natural environment, blood-injection-injury, situational or other. Fear of heights falls under the natural environment type, along with storms and water. Many people have more than one specific phobia, and each relevant specifier can be recorded.

References: DSM-5-TR — Specific phobia: specifiers by phobic stimulus (natural environment: heights, storms, water)

☐ Approve ☐ Revise ☐ Retire Notes:

A 27-year-old with panic disorder fears that a racing heart means a heart attack. In session, her counselor has her run in place and breathe through a thin straw to bring on these sensations on purpose, and she stays with them until her fear fades. What is this technique called?

A. Interoceptive exposure ✓ KEY

Deliberately bringing on feared bodily sensations so fear can decrease is interoceptive exposure.

B. Systematic desensitization

Systematic desensitization pairs relaxation with an imagined fear hierarchy, not induced body sensations.

C. Flooding in imagination

Imaginal flooding uses prolonged exposure to a feared imagined scene, not to physical sensations.

D. Applied relaxation

Applied relaxation teaches a rapid relaxation response; it does not provoke the feared sensations.

Rationale: In panic control treatment, people with panic disorder learn that bodily sensations such as a fast heartbeat or breathlessness are uncomfortable but not dangerous. Interoceptive exposure uses exercises such as running in place, spinning or straw breathing to produce these sensations in a controlled way. Repeated exposure weakens the learned alarm response to internal cues and corrects catastrophic misinterpretations.

References: Barlow PCT — Interoceptive exposure: symptom induction exercises (straw breathing, running in place, spinning)

☐ Approve ☐ Revise ☐ Retire Notes:

A 31-year-old who was sexually assaulted 3 years ago has done well since completing treatment. In early October, the month of the assault, she calls her counselor reporting a sudden return of nightmares, tension and intrusive memories, with no new event. What BEST explains this flare-up?

A. PTSD with delayed expression

This specifier applies when full criteria are first met at least 6 months after the event, not to a later flare.

B. An anniversary reaction ✓ KEY

The date of a trauma acts as a reminder that can bring back symptoms for a time.

C. Failure of her earlier treatment

A brief, cue-linked return of symptoms does not mean her earlier gains were lost.

D. Adjustment disorder

No new stressor has occurred, and the symptoms are trauma-specific reminders.

Rationale: Anniversaries of a traumatic event are common trauma reminders and can bring a temporary return of intrusive memories, nightmares and arousal, even after successful treatment. These reactions fit the PTSD pattern of distress at internal or external cues. Counselors normalize them, help clients anticipate difficult dates and plan coping and support ahead of time.

References: SAMHSA TIP 57 — Trauma reminders and anniversary reactions · DSM-5-TR — PTSD Criterion B4: distress at exposure to cues resembling the event

☐ Approve ☐ Revise ☐ Retire Notes:

For 6 weeks after a mass shooting in another state, a 33-year-old office worker has watched hours of news footage of it each day. She now has nightmares, avoids crowds and feels constantly on edge. She knew no one involved. How does her exposure relate to PTSD Criterion A?

A. It qualifies, since repeated viewing counts as repeated exposure

Repeated exposure counts only for aversive details met in the course of work, not news viewing.

B. It qualifies, as learning about an event counts as indirect exposure

Learning of an event counts only when it happened to a close family member or close friend.

C. It does not qualify, since media exposure counts only if work-related ✓ KEY

DSM-5-TR excludes exposure through electronic media, television or pictures unless it is part of work.

D. It does not qualify, as Criterion A requires a threat to her own life

Witnessing in person or learning of harm to a loved one can also meet Criterion A.

Rationale: PTSD Criterion A requires exposure to actual or threatened death, serious injury or sexual violence by direct experience, witnessing in person, learning it happened to a close family member or friend, or repeated or extreme exposure to aversive details. Exposure through electronic media, television, movies or pictures does not count unless it is work-related. Her distress is still real and may warrant care under another diagnosis.

References: DSM-5-TR — PTSD Criterion A4 note: exposure through electronic media excluded unless work-related

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is trying to tell whether a 30-year-old client's repetitive thoughts are obsessions or the worries of generalized anxiety disorder. Which feature MOST points to obsessions?

A. The thoughts concern real-life matters such as bills and health
Worry about everyday concerns such as money, work and health is typical of GAD.

B. The thoughts are present on more days than not for six months
More days than not for 6 months is the GAD duration rule, not a feature of obsessions.

C. The thoughts come with muscle tension, fatigue and poor sleep
Tension, fatigue and sleep problems are associated symptoms listed for GAD.

D. The thoughts are intrusive, unwanted and met with neutralizing acts ✓ **KEY**
Obsessions are intrusive and unwanted, and the person tries to neutralize them, often with compulsions.

Rationale: DSM-5-TR notes that GAD worries are about real-life concerns, while obsessions usually are not and may have odd, irrational or seemingly magical content. Obsessions are experienced as intrusive and unwanted, and the person tries to ignore, suppress or neutralize them, most often through compulsions. Compulsions are usually present in OCD and absent in GAD.

References: DSM-5-TR — Obsessive-compulsive disorder: differential diagnosis from generalized anxiety disorder

☐ Approve ☐ Revise ☐ Retire Notes:

A 59-year-old has hoarding disorder. Her home is crowded with items she struggles to discard, and she also buys several unneeded kitchen gadgets every week even though there is no space for them. Which specifier should be added to her diagnosis?

A. With excessive acquisition ✓ **KEY**
This specifier applies when difficulty discarding comes with excessive buying or collecting of unneeded items.

B. With poor insight
Insight specifiers describe beliefs about the problem, not buying behavior.

C. With absent insight/delusional beliefs
Nothing suggests she is fully convinced her hoarding is not a problem.

D. With compulsive buying features
This is not a DSM-5-TR specifier for hoarding disorder.

Rationale: DSM-5-TR hoarding disorder has two kinds of specifiers: "with excessive acquisition," for difficulty discarding paired with excessive acquiring of items that are not needed or for which there is no space, and an insight specifier. Most people with hoarding disorder show excessive acquisition, most often through buying. Treatment then targets both acquiring and discarding.

References: DSM-5-TR — Hoarding disorder: specifier with excessive acquisition

☐ Approve ☐ Revise ☐ Retire Notes:

After 7 months without alcohol, a 45-year-old has two beers at a wedding. The next day she tells her counselor, "I've proven I'm a hopeless drunk, so there's no point in trying," and she has kept drinking since. Which relapse prevention concept describes her reaction?

A. Seemingly irrelevant decisions

These are small choices that edge a person toward high-risk situations before any use.

B. Abstinence violation effect ✓ KEY

Blaming a lapse on a fixed personal failing and then giving up is the abstinence violation effect.

C. Positive outcome expectancies

These are expected benefits of drinking, such as relaxing, not a reaction to a lapse.

D. Rebound withdrawal effect

This is not a relapse prevention concept; two beers would not cause withdrawal.

Rationale: Marlatt described the abstinence violation effect as the guilt and self-blame that follow a lapse when a person attributes it to internal, stable and global causes such as being "a hopeless drunk." That thinking turns a single lapse into a full relapse. Counselors help clients reframe a lapse as a mistake to learn from and plan for the high-risk situation that led to it.

References: Marlatt & Donovan (Relapse Prevention) — Abstinence violation effect: attributions after a lapse

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is reviewing the DSM-5-TR criteria for gambling disorder. How many of the listed criteria must a person show within a 12-month period to meet the diagnosis?

A. Two of eleven

Two of eleven is the threshold for substance use disorders, not gambling disorder.

B. Three of seven

This does not match the gambling disorder criteria set.

C. Four of nine ✓ KEY

Gambling disorder requires four or more of nine criteria in a 12-month period.

D. Five of ten

Ten criteria with a five-criterion cutoff appeared in DSM-IV pathological gambling.

Rationale: DSM-5-TR places gambling disorder in the substance-related and addictive disorders chapter and requires four or more of nine criteria within 12 months, such as needing larger bets, chasing losses and lying to hide gambling. Severity is mild with 4 to 5 criteria, moderate with 6 to 7 and severe with 8 to 9. The behavior must not be better explained by a manic episode.

References: DSM-5-TR — Gambling disorder: Criterion A (four or more of nine in 12 months) and severity

☐ Approve ☐ Revise ☐ Retire Notes:

A team is evaluating a 6-year-old for autism spectrum disorder. Under DSM-5-TR, which pattern of current or past deficits is required for the diagnosis?

A. All three social communication deficits and two of four repetitive behavior types ✓ KEY

Criterion A requires all three social communication deficits and Criterion B at least two of four types.

B. Two of three social communication deficits and one of four repetitive behavior types

Partial social deficits and a single repetitive behavior type fall short of both criteria.

C. Any one social communication deficit plus all four of the repetitive behavior types

All three social communication deficits are required, and only two repetitive types are needed.

D. All three social communication deficits, with repetitive behaviors being optional

Without restricted, repetitive behaviors, social (pragmatic) communication disorder is considered.

Rationale: DSM-5-TR autism spectrum disorder requires persistent deficits in all three areas of social communication: social-emotional reciprocity, nonverbal communication and relationships. It also requires at least two of four restricted, repetitive patterns: stereotyped movements or speech, insistence on sameness, fixated interests and unusual sensory reactivity. These may be current or by history.

References: DSM-5-TR — Autism spectrum disorder: Criteria A (all three) and B (at least two of four)

☐ Approve ☐ Revise ☐ Retire Notes:

For the past year, parents say their 8-year-old cannot sit still, interrupts constantly and loses things at home. His teacher, coach and scout leader see no such problems, and his school records are strong. Which DSM-5-TR ADHD requirement is MOST clearly unmet?

A. Several symptoms were present before age 12

He is 8, so any current symptoms began before age 12.

B. Several symptoms appear in two or more settings ✓ KEY

Symptoms are reported only at home, so the two-setting requirement is not met.

C. Symptoms have persisted for at least 6 months

The parents describe a full year of these behaviors.

D. Symptoms are not tied to any known stressor event

ADHD has no such criterion; the issue here is setting.

Rationale: DSM-5-TR requires that several ADHD symptoms be present in two or more settings, such as home, school, work or activities, and that they began before age 12 and lasted at least 6 months. When problems appear in only one setting, the counselor explores what is happening there, such as family stress, parenting patterns or differing expectations, before considering ADHD.

References: DSM-5-TR — ADHD Criterion C: several symptoms present in two or more settings

☐ Approve ☐ Revise ☐ Retire Notes:

For the past 6 months, a 41-year-old has been convinced that a well-known television news anchor is secretly in love with her and sends her coded messages during broadcasts. Apart from this belief, she works and functions well. If delusional disorder is diagnosed, which type applies?

A. Grandiose type

Grandiose delusions center on great talent, insight or an important discovery.

B. Persecutory type

Persecutory delusions involve being plotted against, spied on or harmed.

C. Jealous type

Jealous delusions involve a belief that one's partner is unfaithful.

D. Erotomantic type ✓ KEY

The belief that another person, often of higher status, is in love with her is erotomantic.

Rationale: Delusional disorder requires one or more delusions lasting at least 1 month without the other active symptoms of schizophrenia, and functioning is often not markedly impaired outside the delusion. DSM-5-TR subtypes include erotomantic, grandiose, jealous, persecutory, somatic, mixed and unspecified. In the erotomantic type, the central theme is that another person, often famous or of higher status, is in love with the individual.

References: DSM-5-TR — Delusional disorder: subtypes (erotomantic type)

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old with schizophrenia has heard voices for years. He insists he is not ill, says his medication is useless and often stops taking it. His sister calls him "just stubborn." According to DSM-5-TR, how is his lack of awareness of his illness BEST understood?

A. As a coping style that shields him from the stigma of his diagnosis

DSM-5-TR describes unawareness of illness as a symptom of the disorder, not mainly a coping strategy.

B. As a sign of a co-occurring personality disorder that needs its own care

Poor insight is common in schizophrenia itself and does not imply a separate personality disorder.

C. As a common symptom of the illness that predicts stopping treatment ✓ KEY

Unawareness of illness is a frequent symptom and the most common predictor of nonadherence.

D. As a sign that his diagnosis is probably wrong and should be reviewed

Denying illness is typical in schizophrenia and does not by itself call the diagnosis into question.

Rationale: DSM-5-TR notes that many people with schizophrenia have poor insight or are unaware of their illness, and that this is typically a symptom of the disorder rather than a coping strategy, comparable to anosognosia after brain injury. It is the most common predictor of treatment nonadherence and is linked to relapse and poorer outcomes. Counselors avoid arguing about labels and instead build on the client's own goals, often with motivational approaches and family education.

References: DSM-5-TR — Schizophrenia: associated features supporting diagnosis (unawareness of illness, anosognosia)

☐ Approve ☐ Revise ☐ Retire Notes:

A 42-year-old sales executive comes to counseling at his husband's urging. He describes himself as far more gifted than his coworkers, expects special treatment, becomes enraged when he is not praised and seems unmoved when his husband describes feeling hurt. Which personality disorder do these traits MOST suggest?

A. Narcissistic personality disorder ✓ **KEY**

Grandiosity, entitlement, need for admiration and lack of empathy define narcissistic personality disorder.

B. Histrionic personality disorder

Histrionic traits center on attention seeking, shallow emotion and dramatic display, not grandiosity.

C. Antisocial personality disorder

Nothing suggests a pattern of rule breaking, deceit or violating others' rights.

D. Paranoid personality disorder

Paranoid traits involve distrust and suspicion of others, which he does not describe.

Rationale: Narcissistic personality disorder is a Cluster B pattern of grandiosity, need for admiration and lack of empathy, beginning by early adulthood and shown across contexts. Criteria include an inflated sense of self-importance, entitlement and a belief that one is special. Beneath the grandiosity, self-esteem is often fragile, which explains his rage at criticism or lack of praise.

References: DSM-5-TR — Narcissistic personality disorder: diagnostic criteria

☐ Approve ☐ Revise ☐ Retire Notes:

Since early adulthood, a 47-year-old has doubted the loyalty of friends, read hidden insults into harmless remarks, held long grudges and refused to confide in others for fear that what he says will be used against him. He holds no fixed belief that a specific plot against him exists. Which diagnosis BEST fits?

A. Delusional disorder, persecutory type

Delusional disorder needs a fixed false belief, and he holds no delusion of a specific plot.

B. Paranoid personality disorder ✓ **KEY**

A lasting, pervasive pattern of distrust and suspicion without psychosis fits paranoid personality disorder.

C. Schizotypal personality disorder

Schizotypal traits add odd beliefs, unusual perceptions and eccentric behavior, which are absent.

D. Avoidant personality disorder

Avoidant traits stem from fear of rejection and feeling inadequate, not from distrust of others' motives.

Rationale: Paranoid personality disorder is a Cluster A pattern of pervasive distrust and suspiciousness, starting by early adulthood, in which others' motives are read as malevolent. Signs include doubting the loyalty of friends, reading hidden meanings into benign remarks, bearing grudges and reluctance to confide. It is not diagnosed when the suspicion occurs only during a psychotic disorder; the absence of fixed delusions helps separate it from delusional disorder.

References: DSM-5-TR — Paranoid personality disorder: criteria and differential from delusional disorder

☐ Approve ☐ Revise ☐ Retire Notes:

Under DSM-5-TR, how often must binge eating and inappropriate compensatory behaviors both occur, on average, to meet the criteria for bulimia nervosa?

A. Twice a week for 6 months

No version of the criteria has used this frequency and duration.

B. Once a week for 6 months

The weekly frequency is right, but the required duration is 3 months.

C. Once a week for 3 months ✓ **KEY**

Both behaviors must occur on average at least once a week for 3 months.

D. Twice a week for 3 months

Twice weekly was the DSM-IV threshold; DSM-5 lowered it to once weekly.

Rationale: DSM-5-TR requires that binge eating and inappropriate compensatory behaviors such as vomiting, laxative misuse, fasting or excessive exercise both occur, on average, at least once a week for 3 months. DSM-IV had required twice weekly. Binge-eating disorder uses the same once-a-week for 3 months threshold for binge episodes.

References: DSM-5-TR — Bulimia nervosa Criterion C: at least once a week for 3 months

☐ Approve ☐ Revise ☐ Retire Notes:

A 20-year-old college athlete denies any eating concerns. Her dentist has noted unusual wear of her tooth enamel, her face looks swollen near the jaw, and she has small calluses on the backs of her fingers. Which behavior do these signs MOST suggest?

A. Severe restriction of food intake

Restriction causes weight loss and other signs, not enamel erosion or hand calluses.

B. Regular misuse of laxatives

Laxative misuse affects the bowel and electrolytes, not the teeth or knuckles.

C. Excessive compensatory exercise

Overexercise does not produce enamel erosion, salivary gland swelling or hand calluses.

D. Repeated self-induced vomiting ✓ **KEY**

Stomach acid erodes enamel, vomiting enlarges salivary glands, and teeth scar the hand used to gag.

Rationale: DSM-5-TR notes that recurrent vomiting can cause significant, permanent loss of dental enamel and enlarged salivary glands, especially the parotid glands. People who induce vomiting by triggering the gag reflex with a hand may develop calluses or scars on its back. A counselor who notices such signs raises concerns gently and arranges medical evaluation, since purging can cause dangerous electrolyte imbalances.

References: DSM-5-TR — Bulimia nervosa: associated features (dental enamel loss, parotid enlargement, calluses on the hand)

☐ Approve ☐ Revise ☐ Retire Notes:

Over 8 months, a 72-year-old has developed a slow, shuffling, wide-based walk, urinary incontinence and growing slowness of thought. His family assumes it is Alzheimer's disease. Which cause of cognitive decline does this combination of features MOST suggest the medical team should rule out?

A. Normal pressure hydrocephalus ✓ **KEY**

Gait disturbance, urinary incontinence and cognitive slowing form the classic triad of this condition.

B. Frontotemporal lobar degeneration

This begins with changes in behavior or language, not with gait problems and incontinence.

C. Creutzfeldt-Jakob prion disease

Prion disease usually declines over weeks to months with myoclonus, not this triad.

D. Alcohol-related amnesic decline

Nothing in the history points to heavy alcohol use or a mainly amnesic picture.

Rationale: Normal pressure hydrocephalus is a structural cause of neurocognitive disorder due to another medical condition. Its classic triad is a magnetic, shuffling gait, urinary incontinence and slowed thinking, often with the gait problem appearing first. It matters because it may improve with a shunt, while Alzheimer's disease usually begins with memory loss and leaves gait intact until later stages.

References: DSM-5-TR — Major or mild NCD due to another medical condition: structural lesions such as normal-pressure hydrocephalus

☐ Approve ☐ Revise ☐ Retire Notes:

The family of a newly diagnosed 77-year-old asks which change is usually noticed FIRST in neurocognitive disorder due to Alzheimer's disease. What should the counselor explain?

A. Marked change in personality and social conduct

Early personality and conduct change points to frontotemporal neurocognitive disorder.

B. Trouble learning and recalling recent information ✓ **KEY**

Alzheimer's disease most often begins with impaired learning and memory for recent events.

C. Visual hallucinations and swings in alertness

Well-formed visual hallucinations and fluctuating attention suggest Lewy body disease.

D. Sudden stepwise drops in thinking after small strokes

Stepwise decline linked to strokes suggests vascular neurocognitive disorder.

Rationale: The typical presentation of Alzheimer's disease is amnesic: gradual decline in learning and memory, especially recall of recent information, is usually the first sign. Other domains such as language, visuospatial skills and executive function decline later, with behavioral symptoms more common as the disease advances. Knowing this pattern helps families recognize which early changes call for evaluation.

References: DSM-5-TR — Major or mild NCD due to Alzheimer's disease: amnesic presentation and early memory decline

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor-in-training tells a grieving client, "You've moved past denial and anger, so now you're in the bargaining stage, and acceptance comes after that." Her supervisor raises a concern about this statement. Which concern is MOST accurate?

A. Stages should be applied in reverse order after a sudden death

No grief model reverses its stages after sudden losses.

B. Bargaining comes before anger, so the client was placed wrongly

In Kübler-Ross's order, anger comes before bargaining, so this is incorrect.

C. Mourners do not move through grief in a fixed order of stages ✓ **KEY**

Grief is fluid and individual, and research does not support a fixed sequence of stages.

D. Stage models describe children's grief rather than adult grief

Kübler-Ross's stages came from interviews with dying adults, not from children.

Rationale: Kübler-Ross based her stages on interviews with dying patients, and later research has not shown that mourners pass through stages in a set order. Worden instead describes tasks of mourning that mourners revisit and work on in their own way and time. Telling a client which stage comes next can make normal variation in grief feel like failure.

References: Worden (Grief Counseling) — Stages, phases and tasks of mourning: limits of stage models

☐ Approve ☐ Revise ☐ Retire Notes:

Two clients each lost a father to the same type of cancer in the same month. One had a warm, secure bond with her father; the other had a stormy, highly ambivalent relationship with his and now swings between rage and guilt. Which of Worden's mediators of mourning BEST explains the difference in their grief?

A. Mode of death

Both deaths came from the same illness, so how the fathers died does not explain the difference.

B. Social mediators

Social mediators concern support and social roles, which the stem does not contrast.

C. Concurrent stressors

No other life crises are described for either client.

D. Nature of the attachment ✓ **KEY**

The security and ambivalence of the bond shape how a loss is grieved.

Rationale: Worden lists mediators that shape how a person mourns: who the person was, the nature of the attachment, mode of death, historical antecedents, personality variables, social mediators and concurrent stressors. The nature of the attachment includes the strength, security and ambivalence of the relationship. Highly ambivalent relationships are often followed by intense guilt and anger, which raises the risk of complicated grief.

References: Worden (Grief Counseling) — Mediators of mourning: nature of the attachment (ambivalence and conflict)

☐ Approve ☐ Revise ☐ Retire Notes:

At an evacuation center after a wildfire, a counselor providing psychological first aid notices a quiet 10-year-old sitting beside his mother, who is busy filling out aid forms. According to the PFA Field Operations Guide, how should the counselor BEGIN?

A. Sit with the boy and ask him to describe what he saw during the fire

PFA does not press survivors, least of all children, for details of what they went through.

B. Introduce herself to his mother, explain her role and ask permission ✓ **KEY**

PFA advises first contacting a child's parent or caregiver to explain one's role and seek permission.

C. Take the boy to a children's area so he can talk away from his mother

Separating a child from a present caregiver without permission undermines safety and trust.

D. Give his mother a referral for trauma therapy for her son right away

Most children recover naturally; PFA does not assume a need for therapy before any contact.

Rationale: The first PFA core action, contact and engagement, calls for a respectful, nonintrusive approach. With a child or adolescent, the guide advises first making contact with the parent or accompanying adult to explain the responder's role and seek permission. Working through caregivers supports family connection, which is itself a key source of children's recovery after disaster.

References: Psychological First Aid Field Operations Guide (NCTSN/NCPTSD) — Core Action 1, Contact and Engagement: approaching children through parents or caregivers

☐ Approve ☐ Revise ☐ Retire Notes:

A crisis counselor quickly rates a client's reactions in three domains, affective, behavioral and cognitive, on scales of severity. The counselor then uses the combined score to decide how directive to be. Which tool is the counselor using?

A. Triage Assessment Form ✓ **KEY**

The TAF rates affective, behavioral and cognitive reactions to set the level of counselor directiveness.

B. Columbia Suicide Severity Scale

The C-SSRS assesses suicidal ideation and behavior, not three domains of crisis reaction.

C. Mental status examination

An MSE describes appearance, mood, thought and cognition, without a crisis severity score.

D. Stanley-Brown Safety Plan

This is a collaborative suicide safety plan, not a crisis rating tool.

Rationale: James and Gilliland use Myer's Triage Assessment Form to rate a person in crisis on affective, behavioral and cognitive functioning. Higher combined severity calls for a more directive stance, while lower severity allows collaborative or nondirective work. Because it is quick, the counselor can repeat it during the session to track changes as the client stabilizes.

References: James & Gilliland (Crisis Intervention Strategies) — Triage Assessment Form: affective, behavioral and cognitive severity ratings

☐ Approve ☐ Revise ☐ Retire Notes:

A 38-year-old man tells his counselor, with visible shame, that his wife slaps and scratches him during arguments, hides his car keys and tracks his phone. He laughs it off, saying, "Guys can't really be abused." What is the counselor's BEST response?

A. Explore his own anger, since in such cases men may provoke the abuse

This blames the person being abused and reflects a gender stereotype about who causes violence.

B. Suggest couples counseling so the two can improve how they talk

Conjoint work is unsafe while one partner uses violence and control, whatever their gender.

C. Take the abuse seriously, assess his safety and plan for safety ✓ **KEY**

Male survivors deserve the same risk assessment and safety planning as any other survivor.

D. Reassure him her behavior is minor and unlikely to get any worse

Hitting plus control of movement and phone is a pattern of abuse that can escalate.

Rationale: Men can be victims of intimate partner violence, and shame and stereotypes often keep them from naming it or seeking help. The counselor validates his disclosure, assesses danger and builds a safety plan just as with any survivor, and connects him with services that serve men. Treating clients equally regardless of gender is also an ethical duty.

References: James & Gilliland (Crisis Intervention Strategies) — Intimate partner violence: risk assessment and safety planning with survivors · ACA Code of Ethics — C.5 Nondiscrimination

☐ Approve ☐ Revise ☐ Retire Notes:

A 16-year-old tells her school counselor that her boyfriend demands her social media passwords, gets angry if she does not answer his texts within minutes and insists that she share her location at all times. She says this shows how much he cares. How should the counselor view these behaviors?

A. As normal jealousy that most teen couples work through over time

Treating monitoring and control as normal jealousy misses a known pattern of dating abuse.

B. As a sign that she is not yet mature enough to date anyone at all

This places the problem on her rather than on his controlling behavior.

C. As a privacy matter to be settled only between her and her parents

The counselor has a role in helping her recognize risk and plan for safety.

D. As warning signs of controlling, abusive behavior in the relationship ✓ **KEY**

Demanding passwords, constant contact and tracking are forms of digital control in dating abuse.

Rationale: Dating violence among adolescents often begins with monitoring and control, and technology makes constant checking, password demands and location tracking easy. Teens may read these behaviors as signs of love, so counselors help them name the pattern, assess for other abuse and plan for safety. The counselor also follows school policy and mandated reporting laws if there is any abuse that triggers a report.

References: James & Gilliland (Crisis Intervention Strategies) — Intimate partner violence: adolescent dating violence and controlling behaviors

☐ Approve ☐ Revise ☐ Retire Notes:

A couple married for 35 years tells a counselor that the intense physical passion of their early years has faded, but they feel deeply close, share everything and are firmly committed to each other. In Sternberg's triangular theory, which type of love does this describe?

A. Romantic love

Romantic love combines intimacy and passion without lasting commitment.

B. Companionate love ✓ KEY

Intimacy plus commitment, with little passion, is companionate love.

C. Fatuous love

Fatuous love joins passion and commitment without intimacy.

D. Empty love

Empty love is commitment alone, without intimacy or passion.

Rationale: Sternberg's triangular theory describes love through three components: intimacy, passion and commitment. Companionate love combines intimacy and commitment, and it often grows as passion declines in long relationships. Helping couples see this shift as a normal change rather than a loss of love can ease worry in later stages of a relationship.

References: Berk (Development Through the Lifespan) — Early adulthood: Sternberg's triangular theory of love (intimacy, passion, commitment)

☐ Approve ☐ Revise ☐ Retire Notes:

A first-year college student is frustrated that her professors present several theories without saying which one is correct. She believes every question has one right answer that experts should simply hand to students. According to Perry's research on cognitive change in college, which type of thinking does she show?

A. Dualistic thinking ✓ KEY

Seeing knowledge as right or wrong answers held by authorities is dualistic thinking.

B. Relativistic thinking

Relativistic thinkers accept that several views can be valid, which she rejects.

C. Commitment within relativism

This later level involves choosing a position while knowing others are valid.

D. Pragmatic thought

Pragmatic thought is Labouvie-Vief's concept of using logic to solve real-world problems.

Rationale: William Perry found that college students often start with dualistic thinking, dividing information into right and wrong answers given by authorities. With exposure to many viewpoints, they move to relativistic thinking, seeing that knowledge depends on context, and some later reach commitment within relativistic thinking. Counselors can frame frustration with ambiguity as part of normal cognitive growth during the transition to college.

References: Berk (Development Through the Lifespan) — Early adulthood: Perry's theory of epistemic cognition (dualistic to relativistic thinking)

☐ Approve ☐ Revise ☐ Retire Notes:

A 10-year-old tells her counselor that her mother's boyfriend pays her to pose without clothes while he takes photos, which he says he shares with "friends online." She says he has never touched her. Under DSM-5-TR definitions, how should the counselor understand this situation?

A. As child neglect, since there was no physical contact or injury

Neglect is a failure to meet basic needs; this is an active sexual act involving the child.

B. As child sexual abuse, which includes noncontact exploitation ✓ **KEY**

DSM-5-TR includes noncontact exploitation for others' sexual gratification within child sexual abuse.

C. As not abuse, since sexual abuse requires direct physical contact

Direct contact is not required; enticing a child into sexual acts without touch still counts.

D. As only psychological abuse, since he has not touched her at all

The sexual purpose of the acts makes this sexual abuse, not psychological abuse alone.

Rationale: DSM-5-TR defines child sexual abuse as any sexual act involving a child that is intended to provide sexual gratification to a parent, caregiver or other responsible person. It explicitly includes noncontact exploitation, such as forcing, tricking, enticing or pressuring a child to take part in acts for the sexual gratification of others without direct physical contact. Producing sexual images of a child is also reportable maltreatment, so the counselor must make a report under state law.

References: DSM-5-TR — Child maltreatment and neglect problems: child sexual abuse, including noncontact exploitation · State law — Mandated reporting of suspected child sexual abuse and exploitation

☐ Approve ☐ Revise ☐ Retire Notes:

A pediatric team suspects that a mother has been adding her own blood to her 4-year-old son's urine samples, leading to repeated hospital tests and procedures. No external reward, such as money, is evident, and the mother appears warm and attentive on the unit. If the suspicion is confirmed, how does DSM-5-TR assign the diagnosis?

A. The child receives factitious disorder imposed on another, as the victim

This diagnosis is given to the person who falsifies the illness, not to the child who is harmed.

B. The mother receives malingering, since she produced false signs of illness

Malingering requires an external incentive; the deception here occurs without obvious external reward.

C. The child receives factitious disorder imposed on self, as he shows the signs

The child is not falsifying anything; the deception is carried out by the caregiver.

D. The mother is diagnosed with factitious disorder imposed on another ✓ **KEY**

DSM-5-TR assigns this diagnosis to the perpetrator; the child may receive an abuse-related code.

Rationale: Factitious disorder imposed on another involves falsifying or inducing illness or injury in another person, such as a child, and presenting that person as ill, with deception evident even without obvious external rewards. DSM-5-TR gives the diagnosis to the perpetrator, while the victim may be given an abuse diagnosis or code. Because the behavior is a form of child maltreatment, a counselor who suspects it is also a mandated reporter.

References: DSM-5-TR — Factitious disorder imposed on another: diagnosis given to the perpetrator, not the victim · State law — Child abuse reporting statutes: medical child abuse as reportable maltreatment

☐ Approve ☐ Revise ☐ Retire Notes:

A 41-year-old software engineer was promoted to lead a 30-person division. He earns more, but he dreads budget meetings and misses solving design problems himself. He tells his counselor he would take a pay cut to return to hands-on engineering. In Schein's model of career anchors, which anchor does this BEST suggest?

A. Technical/functional competence, valuing expertise in his field ✓ KEY

He finds his identity in using specialized skill and resists a general management role.

B. General managerial competence, valuing leading and integrating

People with this anchor seek the leadership role he dislikes.

C. Security/stability, valuing a safe and predictable long-term job

He is willing to accept less pay; his concern is the kind of work, not job security.

D. Lifestyle, valuing work that fits with his family and personal life

Nothing suggests his choice is driven by balancing work with family or personal life.

Rationale: Schein described a career anchor as the part of a person's occupational self-concept, made up of talents, motives and values, that the person will not give up when forced to choose. His eight anchors are technical/functional competence, general managerial competence, autonomy/independence, security/stability, entrepreneurial creativity, service or dedication to a cause, pure challenge, and lifestyle. People anchored in technical/functional competence often feel lost when promoted out of their specialty into general management.

References: Sharf (Career Development Theory) — Adult career development: Schein's career anchors

► **Check:** Confirm the cited Sharf edition covers Schein's career anchors; the anchors themselves are standard (Schein, 1978/1990).

☐ Approve ☐ Revise ☐ Retire Notes:

A 63-year-old nurse has cut back to three shifts a week, is handing off her charge duties to younger staff, and spends evenings planning how she and her wife will live after she retires. In Super's life-span, life-space theory, which stage BEST fits her current tasks?

A. Maintenance, as she holds on to her current position and skills

Maintenance centers on holding, updating and innovating, not on slowing down and planning to leave.

B. Establishment, as she secures her place and advances at work

Establishment involves stabilizing and advancing, the opposite of handing off duties.

C. Disengagement, as she slows down and plans for her retirement ✓ KEY

Deceleration and retirement planning are the tasks of Super's disengagement stage.

D. Exploration, as she tries out new roles while weighing options

Exploration involves crystallizing and trying out choices, not winding down a career.

Rationale: Super's life stages are growth, exploration, establishment, maintenance and disengagement (earlier called decline). The disengagement stage includes the tasks of deceleration, retirement planning and retirement living, as people reduce work involvement and shift energy to other life roles. Super saw stage ages as flexible, and people can recycle through stages as circumstances change.

References: Sharf (Career Development Theory) — Super's life-span theory: the disengagement stage (deceleration, retirement planning, retirement living)

☐ Approve ☐ Revise ☐ Retire Notes:

A 17-year-old Latina student loves her robotics club and earns B grades in math, yet she avoids advanced math courses, saying, "I'm just not a math person." Her teachers say she is clearly capable. In social cognitive career theory, which factor should the counselor MOST directly target?

A. Her Holland code, by giving an inventory to confirm a Realistic type

Her interests are already clear; another inventory does not address the belief limiting her choices.

B. Her life-role salience, by exploring how much she values work roles

Role salience comes from Super's theory; nothing suggests she places low value on work.

C. Her career maturity, by teaching the stages of planning and exploring

Career maturity is a Super construct; her barrier is a belief about her ability, not planning skills.

D. Her self-efficacy beliefs, through mastery experiences and support ✓ **KEY**

SCCT holds that low self-efficacy leads people to rule out options they could succeed in.

Rationale: Social cognitive career theory (Lent, Brown and Hackett) proposes that self-efficacy beliefs and outcome expectations shape interests, goals and actions, within contextual supports and barriers. People often rule out fields because they doubt their ability rather than because they lack it. Counselors strengthen self-efficacy through Bandura's four sources: mastery experiences, vicarious learning, verbal persuasion, and managing anxious arousal.

References: Sharf (Career Development Theory) — Social cognitive career theory: self-efficacy, outcome expectations and personal goals

☐ Approve ☐ Revise ☐ Retire Notes:

During a couples session, a husband's face flushes, his heart races, and he stops responding to his wife in the middle of an argument. He says, "I can't think straight right now." Drawing on Gottman's research, what should the counselor do?

A. Call a break of at least 20 minutes so he can calm down before going on ✓ **KEY**

Flooded partners cannot listen well; a soothing break of at least 20 minutes lets arousal settle.

B. Push them to finish the argument now so the issue does not stay unsettled

Continuing while flooded tends to escalate conflict or lead to stonewalling.

C. Ask the wife to raise her concerns more forcefully so that he will respond

Greater pressure adds to his arousal and is likely to deepen his withdrawal.

D. Treat his silence as contempt and confront him about its harm to the couple

His shutdown reflects physiological flooding, not the mockery and disdain that define contempt.

Rationale: Gottman describes flooding as diffuse physiological arousal: a racing heart and surge of stress hormones that make it hard to listen, think or respond with empathy. Flooded partners often stonewall or escalate. Couples are taught to recognize flooding, take a break of at least 20 minutes doing something soothing, and then return to the conversation.

References: Gladding (Counseling: A Comprehensive Profession) — Couples counseling: Gottman's research on flooding and self-soothing breaks

► **Check:** Gottman's own texts are not on the approved source list; confirm the cited Gladding edition covers flooding.

☐ Approve ☐ Revise ☐ Retire Notes:

Within four years, a couple in their fifties has seen their three children leave home, one son move back after a job loss, and a new son-in-law and grandchild join the family. In Carter and McGoldrick's family life cycle, which key emotional task does this stage involve?

A. Committing to a new family system as a newly formed couple

Commitment to a new system is the task of the joining of families through marriage.

B. Accepting many exits from and entries into the family system ✓ **KEY**

This is the key task of the launching stage, as children leave and in-laws and grandchildren arrive.

C. Accepting new members into the system as children are born

This is the task of families with young children, not of parents launching adult children.

D. Accepting the shift of generational roles as parents grow old

This is the task of families in later life, when the couple themselves are aging.

Rationale: Carter and McGoldrick's family life cycle names a key emotional process for each stage. In launching children and moving on, families accept a multitude of exits from and entries into the family system: they renegotiate the couple as a dyad, build adult-to-adult relationships with grown children, take in in-laws and grandchildren, and often face the illness or death of their own parents. Returning adult children are a common feature of this stage.

References: Goldenberg (Family Therapy) — The family life cycle (Carter & McGoldrick): launching children and moving on

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old who married and began a sales career at 23 now questions both choices. He says, "The life I built in my twenties felt right then, but now it feels temporary. If I'm going to change things, it has to be soon." In Levinson's theory of adult development, which period BEST fits?

A. The early adult transition, as he first leaves adolescence behind

The early adult transition spans about ages 17 to 22, when the young person leaves adolescence.

B. The midlife transition, as he weighs his youth against his aging

The midlife transition occurs around ages 40 to 45, not at 29.

C. The age-30 transition, as he reworks his first adult life structure ✓ **KEY**

Around ages 28 to 33, people reappraise the first adult life structure and feel urgency to change it.

D. The culmination of early adulthood, as he settles into his choices

The settling-down period follows the age-30 transition; he is questioning, not settling.

Rationale: Levinson described adult life as alternating stable periods, in which a life structure is built, and transitions, in which it is reappraised. In the age-30 transition (about 28 to 33), people re-examine the provisional structure formed in their twenties and often feel it is now or never to change it. This is followed by settling down in the culmination of early adulthood and, later, the midlife transition around 40 to 45.

References: Berk (Development Through the Lifespan) — Levinson's seasons of life: the age-30 transition in early adulthood

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old teacher lost her lower leg in a rare boating accident. She says none of her friends have faced anything like it, and she has no one her age to compare herself with. In Baltes's lifespan perspective, this event is BEST classified as which kind of influence?

A. An age-graded influence, typical of people at her stage of life

Age-graded influences are predictable by age, such as puberty or retirement.

B. A history-graded influence, shared by most of her generation

History-graded influences, such as a war or pandemic, affect a whole cohort, not one person.

C. A normative influence, expected for most people in her culture

Normative influences are common and expected; a rare accident is neither.

D. A nonnormative influence, an irregular event few people share ✓ KEY

Nonnormative influences are irregular events that happen to one or a few people.

Rationale: Baltes's lifespan perspective identifies three kinds of influence on development: normative age-graded influences tied to age, normative history-graded influences tied to a historical period, and nonnormative influences that are irregular and happen to only one or a few people. Nonnormative events can be especially stressful because there is little preparation, few peers who share them, and limited social scripts for coping.

References: Berk (Development Through the Lifespan) — Lifespan perspective: age-graded, history-graded and nonnormative influences

☐ Approve ☐ Revise ☐ Retire Notes:

An 80-year-old widow has stopped going to large church socials and turns down invitations from casual acquaintances, but she spends happy afternoons with her sister and two lifelong friends. Her son worries that she is withdrawing. Which theory BEST explains her choices as healthy?

A. Socioemotional selectivity theory, favoring close and meaningful ties ✓ KEY

As time feels shorter, older adults focus on emotionally rewarding relationships and trim peripheral ones.

B. Disengagement theory, as society and the elder pull away from each other

She has not withdrawn from people overall; she has chosen to invest in close bonds.

C. Activity theory, as she replaces lost roles with new and busy pursuits

She is narrowing her activities, not adding new roles to replace lost ones.

D. Continuity theory, as she keeps up the full social life of her midlife

She has deliberately cut back from her earlier, wider social life.

Rationale: Carstensen's socioemotional selectivity theory proposes that as people perceive their time as limited, they shift from gathering information and new contacts toward emotionally meaningful goals. Older adults prune peripheral relationships while keeping or deepening close ones, and this pattern is linked to stable or improved emotional well-being. Counselors can help families see a smaller network as a choice rather than a sign of decline.

References: Berk (Development Through the Lifespan) — Socioemotional selectivity theory: shrinking social networks in late adulthood

☐ Approve ☐ Revise ☐ Retire Notes:

A nursing home director says, "Our residents naturally want to pull back from the world, and that is healthy for them, so we keep activities to a minimum." Which theory does this view reflect, and how has research regarded it?

A. Activity theory, which research strongly supports for later life

Activity theory holds the opposite view: elders do best when they stay active and replace lost roles.

B. Disengagement theory, which research has largely failed to support ✓ **KEY**

Mutual withdrawal is not typical or beneficial; many elders want to stay engaged.

C. Continuity theory, which research has found applies to few elders

Continuity theory stresses keeping long-held patterns, not pulling back, and is well supported.

D. Socioemotional selectivity, which research says harms elder health

Selectivity is chosen narrowing toward close ties and is linked to well-being, not harm.

Rationale: Cumming and Henry's disengagement theory held that older adults and society naturally withdraw from each other and that this is good for both. Research has not supported it: many older adults stay engaged, and withdrawal often reflects lost opportunities, poor health or ageist settings rather than preference. A counselor would advocate for chances to stay involved rather than accept reduced activity as healthy.

References: Berk (Development Through the Lifespan) — Social theories of aging: disengagement, activity, continuity and socioemotional selectivity

☐ Approve ☐ Revise ☐ Retire Notes:

The parents of a 6-year-old with spina bifida tell a counselor they believe their son's condition is a punishment for a wrong they once committed, and they feel deep shame about it. Which model of disability do their beliefs MOST closely reflect?

A. The medical model, which sees disability as a bodily defect to treat

The medical model locates disability in the body and seeks cure or treatment, not moral meaning.

B. The social model, which locates disability in barriers in society

The social model blames inaccessible settings and attitudes, not personal wrongdoing.

C. The moral model, which ties disability to sin, fault or divine will ✓ **KEY**

Seeing disability as punishment for wrongdoing, with resulting shame, is the core of the moral model.

D. The minority model, which sees disabled people as a group facing bias

The minority model frames disability as a group identity facing discrimination, not as punishment.

Rationale: The moral (religious) model is the oldest view of disability, linking it to sin, a test of faith or divine will, and it often brings shame and hiding for the person and family. The medical model treats disability as a defect in the body to be fixed, while social and minority models locate the problem in barriers and prejudice. Counselors explore such beliefs respectfully within the family's culture and faith rather than dismissing them.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling individuals with disabilities: moral, medical and social models of disability

► **Check:** Confirm the cited Sue & Sue edition names the moral model alongside the medical and social models.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor at a medical center is screening patients for posttraumatic stress disorder. Under DSM-5-TR, which medical experience MOST clearly qualifies as a traumatic event for PTSD Criterion A?

A. Learning of a cancer diagnosis that carries a guarded prognosis

A life-threatening diagnosis is not by itself considered a Criterion A traumatic event.

B. Living for years with a painful and disabling chronic illness

DSM-5-TR states that a debilitating medical condition is not necessarily traumatic.

C. Hearing from a doctor that a scan has shown a suspicious mass

Distressing medical news is not a sudden, catastrophic event as Criterion A requires.

D. Waking up during surgery, unable to move while feeling pain ✓ **KEY**

DSM-5-TR names waking during surgery as a sudden, catastrophic medical event that qualifies.

Rationale: DSM-5-TR notes that a life-threatening illness or debilitating medical condition is not necessarily a traumatic event for PTSD. Medical incidents that do qualify are sudden and catastrophic, such as waking during surgery or anaphylactic shock. Patients distressed by a diagnosis or chronic illness may instead fit an adjustment disorder or another condition, so counselors assess the nature of the event before considering PTSD.

References: DSM-5-TR — Posttraumatic stress disorder: Criterion A and qualifying medical incidents

☐ Approve ☐ Revise ☐ Retire Notes:

A 27-year-old woman says she has never felt sexual attraction to anyone, identifies as asexual, and is content with this. Her mother urged her to come in, worried that something is wrong. She reports no distress about her lack of sexual desire. Which response by the counselor is BEST?

A. Affirm her identity, noting that no sexual dysfunction applies here ✓ **KEY**

Without distress, and with an asexual identity explaining low desire, DSM-5-TR rules out the diagnosis.

B. Diagnose female sexual interest/arousal disorder, given lifelong low desire

The disorder requires clinically significant distress and is not diagnosed when asexual identity explains it.

C. Refer her for hormone tests to find the cause of her low sexual desire

This treats a valid orientation as a medical problem she has not reported.

D. Explore what past trauma may have led her to shut down sexual feelings

Assuming trauma pathologizes her identity and reflects bias rather than her own account.

Rationale: DSM-5-TR sexual dysfunctions require clinically significant distress, and the text for female sexual interest/arousal disorder (and male hypoactive sexual desire disorder) states that the diagnosis is not made when a lifelong lack of desire is better explained by self-identification as asexual. Asexuality is a sexual orientation, not a disorder. An affirming counselor validates the client's identity and can help the family understand it rather than search for a cause.

References: DSM-5-TR — Female sexual interest/arousal disorder: Criterion C distress and the asexual self-identification exclusion

☐ Approve ☐ Revise ☐ Retire Notes:

A 24-year-old Lakota client tells her counselor, "I'm Two-Spirit." After the session, a counselor in training asks the supervisor what the term means. Which explanation is MOST accurate?

A. A clinical term for a person who has dissociative identity states

Two-Spirit is a cultural identity term, not a clinical or diagnostic label.

B. An Indigenous umbrella term for gender and sexual diversity ✓ **KEY**

Two-Spirit is a pan-Indigenous term for gender and sexual diversity rooted in Indigenous cultures.

C. A term any LGBTQ+ person may adopt to describe being bisexual

The term belongs to Indigenous people and is not a synonym for bisexuality.

D. A religious role that requires a person to stay celibate for life

The term carries no celibacy requirement; its meaning varies across nations and people.

Rationale: Two-Spirit is a pan-Indigenous umbrella term adopted in 1990 at an intertribal gathering in Winnipeg to describe Indigenous people whose gender or sexual identities reflect their cultures' understandings of gender and sexual diversity, often with cultural or spiritual roles. Many nations have their own terms, and meanings vary, so the counselor asks the client what the identity means to her. The term is used by and for Indigenous people rather than as a general LGBTQ+ label.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling Native American and Indigenous clients; LGBTQ identities: Two-Spirit people

▶ **Check:** Confirm the cited Sue & Sue edition discusses Two-Spirit identity; the term's 1990 Winnipeg origin is well documented.

☐ Approve ☐ Revise ☐ Retire Notes:

A family who moved from Ethiopia eight years ago speaks only Amharic at home, socializes almost entirely within their church and ethnic community, and avoids contact with the wider U.S. culture whenever possible. In Berry's model of acculturation, which strategy does this reflect?

A. Integration

Integration keeps the heritage culture while also engaging with the host society.

B. Assimilation

Assimilation gives up the heritage culture in favor of the host culture.

C. Separation ✓ **KEY**

Separation keeps the heritage culture while avoiding contact with the host society.

D. Marginalization

Marginalization involves losing ties to both the heritage and host cultures.

Rationale: Berry's model crosses two questions: does the person maintain the heritage culture, and does the person seek contact with the host society? Integration says yes to both, assimilation favors the host culture, separation favors the heritage culture, and marginalization rejects or loses both. Integration is generally linked to the least acculturative stress, though choices are shaped by how welcoming the host society is.

References: Sue & Sue (Counseling the Culturally Diverse) — Acculturation: Berry's four strategies (integration, assimilation, separation, marginalization)

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old woman who moved from Puerto Rico last year describes how, at her brother's funeral, she began screaming and crying uncontrollably, trembled, felt heat rising from her chest to her head, and later recalled little of it. She has had no similar episodes without a clear trigger. Which explanation BEST fits?

A. Ataque de nervios, a cultural concept of distress tied to a stressor ✓ KEY

The screaming, trembling, rising heat and partial amnesia after a family loss match ataque de nervios.

B. Panic disorder, given her recurrent unexpected surges of intense fear

Her episode followed a clear trigger, and panic disorder requires recurrent unexpected attacks.

C. Brief psychotic disorder, given her loss of memory for the episode

Amnesia alone is not psychosis; she shows no delusions, hallucinations or disorganized speech.

D. Conversion disorder, given her trembling and the sense of rising heat

These are recognized features of a culturally patterned stress response, not a neurological deficit.

Rationale: DSM-5-TR describes ataque de nervios, common among Latinx people of Caribbean origin, as a syndrome of intense emotional upset that can include screaming, crying, trembling, heat rising from the chest to the head, aggression, dissociation and a sense of losing control. It often follows a stressful family event such as a death or conflict. Unlike panic attacks, ataques are usually tied to a precipitating event and often lack the hallmark of acute fear, so they should not be equated with panic disorder.

References: DSM-5-TR — Cultural concepts of distress: ataque de nervios and its relation to panic attacks

☐ Approve ☐ Revise ☐ Retire Notes:

A devout Southern Baptist client with panic disorder tells her counselor that she will not practice mindfulness meditation because it feels like "an Eastern religious practice" that conflicts with her faith. What is the counselor's BEST response?

A. Explain that mindfulness is fully secular, so her objection has no basis

Dismissing her concern overrides her right to choose and imposes the counselor's view.

B. Respect her choice and offer other effective options that fit her faith ✓ KEY

Clients may decline an intervention, and panic disorder has effective options that need no meditation.

C. Pause all coping-skills work until she agrees to try the meditation

Making care depend on one technique pressures her and delays effective treatment.

D. Refer her to her pastor, since the matter is spiritual, not clinical

Her panic disorder is a clinical concern the counselor can treat with other methods.

Rationale: Informed consent gives clients the freedom to choose or decline specific interventions, and counselors avoid imposing their own values about religious practices. When a technique conflicts with a client's faith, the counselor respects the objection and adapts, for example by using breathing retraining, cognitive restructuring and interoceptive exposure for panic, or a faith-consistent practice the client chooses. Adapting the method keeps treatment effective while honoring the client's worldview.

References: ACA Code of Ethics — A.2.a Informed Consent (freedom to choose whether to enter into or remain in a counseling relationship); A.2.b Types of Information Needed · ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

A 40-year-old client says, "I'm spiritual but not religious. I don't belong to any church, but I feel connected to something larger when I hike, and that gives my life meaning." How should the counselor understand the distinction she is drawing?

A. Spirituality requires an organized tradition, so she has no spiritual life

Organized tradition defines religion; spirituality can exist outside any institution.

B. Religion and spirituality mean the same thing, so she just has no group

The terms overlap but differ: religion is organized and communal, spirituality is broader.

C. Her stance signals avoidance that should be explored as a clinical issue

Nothing suggests avoidance; she describes a source of meaning, which is a strength.

D. Spirituality is a search for meaning that can exist apart from religion ✓ KEY

Spirituality refers to meaning, purpose and connection, which may or may not be expressed through religion.

Rationale: Religion generally refers to organized beliefs, practices and communities shared by a group, while spirituality refers to a broader, personal search for meaning, purpose and connection to something greater. The two overlap, but either can exist without the other. Counselors assess clients' spirituality on the clients' own terms, since it is often a source of resilience and coping.

References: Gladding (Counseling: A Comprehensive Profession) — Spirituality in counseling: distinguishing spirituality from religion · Sue & Sue (Counseling the Culturally Diverse) — Spirituality and religion as dimensions of cultural identity

► **Check:** Definitions are standard; confirm the cited editions use comparable wording for the religion/spirituality distinction.

☐ Approve ☐ Revise ☐ Retire Notes:

A 14-year-old says she cuts herself after heated fights with her mother. She explains, "Once Mom sees the cuts, she stops yelling and we finally talk." She denies any wish to die. Under the DSM-5-TR proposed criteria for nonsuicidal self-injury disorder, which expectation does her account BEST illustrate?

A. Relief from a negative feeling or thought state she cannot bear

She describes a change in her mother's behavior, not relief from an inner feeling state.

B. Inducing a positive feeling state, such as a rush of calm or energy

She does not describe seeking a positive feeling from the act itself.

C. Resolving an interpersonal difficulty with someone close to her ✓ KEY

The cutting ends the fight and brings her mother closer, an interpersonal expectation in Criterion B.

D. Signaling an intent to die that she is not yet ready to admit

She denies suicidal intent, and the account points to an interpersonal purpose.

Rationale: The DSM-5-TR proposed criteria for nonsuicidal self-injury disorder list three expectations, at least one of which must be present: to obtain relief from a negative feeling or cognitive state, to resolve an interpersonal difficulty, or to induce a positive feeling state. An interpersonal function should not be dismissed as "attention-seeking"; it shows the behavior has become a way to get needs met. Treatment builds safer ways to communicate and repair conflict, while suicide risk is still assessed.

References: DSM-5-TR — Conditions for further study: nonsuicidal self-injury disorder, Criterion B expectations

☐ Approve ☐ Revise ☐ Retire Notes:

A 17-year-old in dialectical behavior therapy reports a sudden, intense urge to cut while home alone after a breakup. Her emotions feel "off the charts," and she says she cannot think clearly. Which DBT skill set is designed to bring down extreme arousal quickly in a moment like this?

A. TIP skills, such as cold water on the face or brief intense exercise ✓ KEY

TIP skills change body chemistry fast when arousal is too high to use other skills.

B. DEAR MAN, to ask her former partner clearly for what she needs

DEAR MAN is an interpersonal effectiveness skill and needs a calmer state to use well.

C. Check the facts, to test whether her emotion fits the actual event

Checking the facts is an emotion regulation skill that requires clear thinking she lacks now.

D. Wise mind, to blend her emotion and her reason before she decides

Wise mind is a mindfulness skill that is hard to reach at extreme levels of arousal.

Rationale: In Linehan's DBT, TIP skills (temperature, intense exercise, paced breathing and paired muscle relaxation) are distress tolerance crisis survival skills for moments when emotional arousal is so high that a person cannot use more complex skills. Cold water on the face, for example, triggers the dive reflex and slows the heart rate. Once arousal drops, the client can turn to emotion regulation, mindfulness or interpersonal skills.

References: Linehan (DBT Skills Training Manual) — Distress tolerance: TIP skills for changing body chemistry in a crisis

☐ Approve ☐ Revise ☐ Retire Notes:

A client's two-week sleep diary shows that she spends an average of 9 hours in bed each night (10 p.m. to 7 a.m.) but sleeps only about 6 of those hours. What is her sleep efficiency, and what does it suggest?

A. About 150%, showing that she gets more sleep than she needs

This reverses the ratio; efficiency is time asleep divided by time in bed.

B. About 67%, well below the usual target of about 85% or more ✓ KEY

Six hours asleep over nine in bed is about 67%, well below the 85% often used as a target.

C. About 33%, which is the share of the night she sleeps soundly

About 33% is the share of time in bed spent awake, not her sleep efficiency.

D. About 85%, which falls in the range viewed as healthy sleep

The arithmetic gives about 67%, not 85%; her time awake in bed is excessive.

Rationale: Sleep efficiency is total sleep time divided by time in bed, multiplied by 100; here, 6 divided by 9 is about 67%. Efficiency of roughly 85% or higher is often treated as a target in CBT-I. Low efficiency points to sleep restriction, which first limits time in bed to about the client's average sleep time (with a floor of about 5 hours) and then extends it in small steps as efficiency rises.

References: AASM Clinical Practice Guideline (Insomnia) — Sleep restriction therapy: sleep diaries, sleep efficiency and time-in-bed titration

☐ Approve ☐ Revise ☐ Retire Notes:

A 45-year-old being treated for major depressive disorder has had trouble staying asleep at least four nights a week for six months. Her mood has lifted, but the insomnia has not, and it is now her main complaint. The counselor's supervisor says it "will clear up once the depression is fully treated." What is the BEST response?

A. Agree, since insomnia with depression is only a symptom, not a target
Insomnia that persists as a main complaint warrants its own attention and treatment.

B. Agree, and suggest she spend more time in bed to make up lost sleep
Extra time in bed weakens the bed-sleep link and tends to worsen insomnia.

C. Disagree, and refer her for a hypnotic as the sole sleep intervention
CBT-I is the first-line treatment; medication alone is not the preferred approach.

D. Disagree, as insomnia disorder can coexist and responds to CBT-I ✓ **KEY**
DSM-5-TR allows a separate insomnia diagnosis with depression, and CBT-I helps in such cases.

Rationale: DSM-5-TR permits a diagnosis of insomnia disorder alongside another mental disorder when coexisting conditions do not adequately explain the predominant complaint of insomnia. Insomnia often persists after mood improves and raises the risk of depressive relapse. CBT-I is effective for insomnia that co-occurs with depression and is recommended as first-line treatment rather than waiting for sleep to recover on its own.

References: DSM-5-TR — Insomnia disorder: Criterion H and coexisting mental disorders · AASM Clinical Practice Guideline (Insomnia) — Multicomponent CBT-I for chronic insomnia, including comorbid insomnia

☐ Approve ☐ Revise ☐ Retire Notes:

A 22-year-old in counseling for bulimia nervosa, with frequent purging, says a new physician has just prescribed bupropion for depression and to help her quit smoking. She has not told the physician about her eating disorder. Why should the counselor encourage her, with consent, to share this history with the prescriber?

A. Bupropion tends to cause large weight gain that may worsen binge eating
Bupropion is generally weight neutral or linked to slight weight loss.

B. Bupropion is a benzodiazepine with a high misuse risk in eating disorders
Bupropion is a norepinephrine-dopamine reuptake inhibitor, not a benzodiazepine.

C. Bupropion raises the risk of seizures in people with bulimia or anorexia ✓ **KEY**
Bupropion lowers the seizure threshold and is contraindicated in bulimia and anorexia nervosa.

D. Bupropion blocks the effects of any psychotherapy she gets for bulimia
No antidepressant blocks the effects of psychotherapy; the concern is a medical one.

Rationale: Bupropion lowers the seizure threshold, and its labeling lists current or prior bulimia or anorexia nervosa as contraindications because seizures occurred at higher rates in patients with these disorders, likely linked to electrolyte shifts from purging and restriction. Counselors do not advise on medication changes, but they can encourage clients to share their full history and, with consent, coordinate with the prescriber. This keeps care safe without stepping outside the counselor's scope.

References: Gladding (Counseling: A Comprehensive Profession) — Psychopharmacology for counselors: antidepressant classes and adverse effects · ACA Code of Ethics — C.2.a Boundaries of Competence

► **Check:** The contraindication is from bupropion's FDA labeling; confirm Gladding covers it or substitute a psychopharmacology reference.

☐ Approve ☐ Revise ☐ Retire Notes:

Three weeks after starting an SSRI for depression, a 28-year-old client arrives talking rapidly. She says she has needed only three hours of sleep a night for a week and has spent her savings on a new business idea. What should the counselor do?

A. Contact her prescriber promptly, as these may be signs of a manic episode ✓ **KEY**

Little need for sleep, pressured speech and reckless spending may signal mania emerging on an antidepressant.

B. Praise her rise in energy as a sign that the medication is working well

These are warning signs of mania, not ordinary recovery from depression.

C. Advise her to stop the SSRI at once and resume it once she sleeps again

Medication changes are the prescriber's role and fall outside the counselor's scope.

D. Wait for two more weeks to see whether her high mood settles on its own

Mania can escalate quickly and cause serious harm, so prompt action is needed.

Rationale: Antidepressants can trigger manic or hypomanic symptoms in people with an underlying bipolar disorder. DSM-5-TR states that a full manic episode that emerges during antidepressant treatment and persists at a fully syndromal level beyond the drug's physiological effect is sufficient evidence for a manic episode and a bipolar I diagnosis. The counselor's role is to assess safety and coordinate promptly with the prescriber, not to adjust medication.

References: DSM-5-TR — Manic episode: note on episodes emerging during antidepressant treatment · ACA Code of Ethics — C.2.a Boundaries of Competence

☐ Approve ☐ Revise ☐ Retire Notes:

A 10-year-old who has struggled in math says after each low quiz score, "I'm just dumb at math, so trying harder won't help." When she does well, she says, "I got lucky." She now rarely attempts hard problems. Which pattern does her school counselor MOST likely see?

A. A mastery-oriented style, crediting her success to her own ability

Mastery-oriented children credit success to ability and effort and see failure as fixable; she does the reverse.

B. Learned helplessness, blaming failure on a fixed lack of ability ✓ **KEY**

Attributing failure to stable low ability and success to luck, then giving up, defines learned helplessness.

C. Test anxiety, with high arousal that blocks her recall on quizzes

She describes no worry or arousal during tests; the problem is her beliefs about why she fails.

D. A specific learning disorder, shown by her repeated low math scores

Low scores alone do not establish a learning disorder, and her account points to an attribution pattern.

Rationale: Mastery-oriented children attribute success to ability and failure to factors they can change, such as effort, so they persist. Children with learned helplessness attribute failure to fixed low ability and success to luck, so they expect effort to make no difference and avoid challenges. Attribution retraining helps them link outcomes to effort and strategy, using tasks hard enough to need effort but within reach, and feedback that credits what they did.

References: Berk (Development Through the Lifespan) — Middle childhood: achievement-related attributions, learned helplessness and attribution retraining

☐ Approve ☐ Revise ☐ Retire Notes:

After classroom lessons on organization for all sixth graders, a school counselor reviews data and forms a six-week small group for eight students who are still missing many assignments. In a multi-tiered system of supports (MTSS), this group is BEST described as which tier?

A. Tier 1, universal support for every student

The classroom lessons were Tier 1; the group serves a smaller set of students.

B. Tier 3, intensive support for one student

Tier 3 is the most intensive, individualized level, not a small group.

C. A special education service under an IEP

MTSS tiers are general education supports, not services under an IEP.

D. Tier 2, targeted support for a small group ✓ **KEY**

Tier 2 provides targeted, often small-group help for students not helped enough by Tier 1.

Rationale: MTSS organizes supports in tiers: Tier 1 universal instruction for all students, Tier 2 targeted interventions, often in small groups, for students who need more, and Tier 3 intensive, individualized help. Data decide who moves between tiers. ASCA describes school counselors as delivering Tier 1 and Tier 2 supports and collaborating on Tier 3 within a comprehensive school counseling program.

References: ASCA National Model — Direct student services: classroom instruction and small-group counseling aligned with MTSS tiers

► **Check:** ASCA's MTSS guidance is most explicit in its position statement; confirm the National Model citation or note the position statement.

☐ Approve ☐ Revise ☐ Retire Notes:

A 33-year-old Marine veteran with PTSD tells his counselor that he has started using cannabis every night "to treat" his nightmares and asks whether it is a recommended PTSD treatment. Based on the VA/DoD clinical practice guideline for PTSD, what should the counselor tell him?

A. Cannabis is a first-line PTSD treatment that is equal to psychotherapy

The guideline does not list cannabis among first-line treatments; trauma-focused therapy is first line.

B. Cannabis is recommended for PTSD as long as he uses it only at night

Timing of use does not change the guideline's position against cannabis for PTSD.

C. The guideline advises against cannabis for PTSD and favors therapy ✓ **KEY**

The VA/DoD guideline recommends against cannabis or its derivatives for PTSD treatment.

D. The guideline has not addressed cannabis, so his use is up to him

The guideline does address cannabis and advises against it because of weak evidence and known harms.

Rationale: The VA/DoD PTSD guideline recommends against cannabis or cannabis-derived products for treating PTSD, citing a lack of evidence of benefit and known risks such as cannabis use disorder and worsened symptoms. It recommends individual trauma-focused psychotherapies, such as prolonged exposure and cognitive processing therapy, as first-line treatment. The counselor explores his use without judgment, screens for a cannabis use disorder, and offers evidence-based options for his nightmares.

References: VA/DoD CPG — Management of PTSD and acute stress disorder: recommendation against cannabis or cannabis-derived products

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is preparing a crisis resource card for a veteran client and her spouse. How can they reach the Veterans Crisis Line?

A. Dial 988, then press 1, or text 838255 ✓ **KEY**

The Veterans Crisis Line is reached by dialing 988 and pressing 1, or by texting 838255.

B. Dial 911 and ask the dispatcher for the VA

911 is for emergency services; it is not the route to the Veterans Crisis Line.

C. Dial 211 and ask for a veteran peer mentor

211 connects callers to community resources, not to the Veterans Crisis Line.

D. Call the local VA clinic during open hours

The crisis line runs 24/7; a clinic's daytime hours do not cover a crisis.

Rationale: The Veterans Crisis Line is reached through the 988 Suicide & Crisis Lifeline by dialing 988 and pressing 1, by texting 838255, or by online chat, 24 hours a day. It serves veterans, service members, and their families and friends, including those not enrolled in VA care. Giving clients and families this information in advance is part of good safety planning.

References: SAMHSA 988 Lifeline — Veterans Crisis Line: dial 988, then press 1

☐ Approve ☐ Revise ☐ Retire Notes:

Treatment Planning

17 items · NCE weight 9% (14 of 160 scored items)

Q108 of 200 · nce-s-trt-103 · Assessment and Testing · SMART goals and measurable objectives · medium · In review

A 40-year-old Filipino American client and his counselor agree on this objective: "Client will cut arguments with his wife by half within eight weeks." Neither of them knows how often the couple argues now. What should the counselor do FIRST so that progress on this objective can be measured?

A. Have the client log each argument for a week or two to set a baseline ✓ **KEY**

A reduction "by half" can only be judged against a known starting rate, so a baseline count is needed first.

B. Change the deadline to twelve weeks so the goal is easier to reach in time

A longer timeframe may help attainability, but progress still cannot be measured without a starting point.

C. Replace the objective with a broader goal of building a closer, warmer marriage

A broad goal is less measurable than the original objective and does not solve the missing baseline.

D. Ask his wife to rate their marriage at the final session to judge the outcome

A single rating at the end gives no starting value to compare against and does not track the stated behavior.

Rationale: A measurable objective that names a change, such as cutting a behavior in half, depends on a baseline: a record of how often the behavior happens before treatment. Self-monitoring logs are a simple way to gather that baseline and then keep tracking the same behavior over time. Without a baseline, neither the counselor nor the client can tell whether the target has been met.

References: Wiger (Documentation) — Treatment plan objectives: measurable behaviors with baseline and target frequencies

☐ Approve ☐ Revise ☐ Retire Notes:

Q109 of 200 · nce-s-trt-104 · Assessment and Testing · matching level of care to need · medium · In review

A community mental health center begins using the Level of Care Utilization System (LOCUS) to decide how intensive each adult client's services should be. Which of these is one of the dimensions the counselor rates when completing the LOCUS?

A. Type of insurance and benefit limits

Payer coverage is not a LOCUS dimension; the tool rates clinical need, not what a plan will pay for.

B. Intelligence and academic achievement

Cognitive and academic testing is not part of the LOCUS rating dimensions.

C. Recovery environment: stress and support ✓ **KEY**

The LOCUS rates the recovery environment, scoring both the level of stress and the level of support around the client.

D. The DSM-5-TR diagnostic category by itself

The LOCUS is deliberately not driven by diagnosis; it rates risk, functioning and other needs across dimensions.

Rationale: The LOCUS rates six dimensions: risk of harm; functional status; medical, addictive and psychiatric co-morbidity; recovery environment (stress and support); treatment and recovery history; and engagement and recovery status. The combined ratings point to a level of care from recovery maintenance up to medically managed residential services. Because placement rests on these dimensions rather than on diagnosis or payer, clients with the same diagnosis can need very different levels of care.

References: LOCUS (AACP) — Six evaluation dimensions, including Dimension IV, Recovery Environment (Level of Stress and Level of Support)

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor tells a colleague, "Evidence-based practice just means I pick whatever manual has the most clinical trials behind it." Which description of evidence-based practice is MOST accurate?

A. Following a research-tested treatment manual exactly, with no changes for any client

Rigid adherence without regard to the client ignores the clinical expertise and client factors that are part of the definition.

B. Combining the best research with clinical expertise and the client's culture and wishes ✓ **KEY**

Evidence-based practice integrates research evidence, clinical expertise and client characteristics, culture and preferences.

C. Choosing the approach the counselor knows best, as long as clients report satisfaction

Familiarity and satisfaction alone leave out the research base, which is one of the three parts of the model.

D. Using only treatments that insurers list as approved for the client's current diagnosis

Payer approval lists are an administrative matter and are not how evidence-based practice is defined.

Rationale: Evidence-based practice is commonly defined as the integration of the best available research with clinical expertise in the context of the client's characteristics, culture and preferences. Research support is one leg of the model, not the whole of it. The counselor selects and adapts treatments by weighing all three sources together.

References: Erford (Research and Evaluation in Counseling) — Evidence-based practice: integrating best research evidence, clinical expertise and client characteristics and preferences

☐ Approve ☐ Revise ☐ Retire Notes:

A 15-year-old boy has been arrested twice for assault and burglary. He skips school, argues constantly with his grandmother, who is raising him, and spends most evenings with older peers involved in crime. His probation officer asks which treatment has the strongest support for reducing reoffending while he stays at home. What should the counselor recommend?

A. Weekly individual insight-oriented counseling held in the counselor's office

Office-based individual therapy does not reach the family, peer and school factors that drive serious antisocial behavior.

B. A wilderness boot camp built on strict discipline and group confrontation

Boot camps and other confrontational, punitive programs have not been shown to reduce reoffending and may do harm.

C. A weekly group that brings together other teens on probation for similar offenses

Grouping high-risk teens together can strengthen antisocial behavior through deviant peer influence.

D. Multisystemic therapy delivered in his home, school and neighborhood ✓ **KEY**

MST is an intensive, home-based treatment that targets family, peer, school and community factors and has strong evidence for reducing juvenile offending.

Rationale: Multisystemic therapy, developed by Henggeler and colleagues, views serious antisocial behavior as shaped by the many systems around a young person. Therapists with small caseloads work in the home and community, strengthening caregivers' parenting and monitoring, cutting ties with deviant peers and improving school functioning. Controlled trials show reduced rearrest and out-of-home placement compared with usual services.

References: Goldenberg (Family Therapy) — Multisystemic therapy (Henggeler): home- and community-based treatment for serious juvenile antisocial behavior

☐ Approve ☐ Revise ☐ Retire Notes:

After working out a treatment plan together, a counselor asks a 58-year-old client to read and sign it. The client asks why his signature is needed. What is the MAIN purpose of the client's signature on a treatment plan?

A. It shows that the client took part in the plan and agrees with its goals ✓ KEY

The signature documents that the client helped shape the plan and agrees with its goals and methods.

B. It binds the client legally to attend each of the sessions in the plan

A treatment plan is not a binding contract; clients may change or end treatment.

C. It takes the place of the informed consent form signed at the intake

Informed consent is a separate process and document; the plan signature does not replace it.

D. It gives the counselor permission to share the plan with his employer

Releasing information to third parties requires a separate, specific authorization.

Rationale: Treatment plans are meant to be developed with the client, and the client's signature is the usual record that this happened and that the client agrees with the goals, objectives and methods. Payers and auditors often look for it as evidence of collaboration. The signature does not create a legal duty to attend, replace informed consent or authorize disclosures.

References: Wiger (Documentation) — Treatment plan: client participation and signature documenting agreement

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor and a 27-year-old client use goal attainment scaling (GAS) to track progress. For each goal they write five possible outcomes, scored from -2 to +2. At the end of treatment, the client's score on one goal is 0. What does this score mean?

A. There was no change from where the client started

A score of 0 is not tied to the starting point; the client's level at intake is usually rated below 0.

B. The client gave up on this goal during treatment

GAS scores describe levels of outcome; they do not code whether a goal was abandoned.

C. The client reached the expected level of outcome ✓ KEY

In GAS, 0 is the expected level; -1 and -2 are less than expected and +1 and +2 are more than expected.

D. The client did much better than anyone expected

Much better than expected is scored +2, the top of the scale.

Rationale: Goal attainment scaling, developed by Kiresuk and Sherman, builds an individual scale for each client goal. The expected outcome is defined as 0, with two levels below (somewhat and much less than expected) and two levels above (somewhat and much more than expected). Because the scales are written for each client, GAS captures individual goals while still producing scores that can be compared and combined.

References: Erford (Research and Evaluation in Counseling) — Outcome evaluation: goal attainment scaling (Kiresuk and Sherman) and its five-point -2 to +2 scale

☐ Approve ☐ Revise ☐ Retire Notes:

In a randomized trial of a 12-session depression program, 30% of the treatment group drop out early, mostly those with the most severe symptoms. The researchers analyze every randomized participant in the group to which he or she was assigned, using last available or imputed scores for dropouts. Which statement BEST describes this approach?

A. A per-protocol analysis, which tests the effect among the clients who finished

Per-protocol analysis keeps only those who followed the protocol, the opposite of including every randomized person.

B. An intention-to-treat analysis, which keeps the benefits of random assignment ✓ KEY

Analyzing everyone as randomized, regardless of dropout or adherence, is intention-to-treat and preserves comparable groups.

C. A completer analysis, which removes the bias created by the clients who dropped out

Analyzing completers only would drop the most severe clients and likely inflate the program's apparent effect.

D. A matched-pairs analysis, which pairs clients on severity before comparing groups

Matching pairs participants on a variable; it does not describe keeping all randomized participants in their groups.

Rationale: Intention-to-treat analysis includes every participant in the group to which they were randomized, whether or not they completed treatment. Because dropout is often related to outcome, as when the most severe clients leave, analyzing only completers breaks the equivalence that randomization created and tends to overstate effects. Intention-to-treat gives a more conservative estimate that reflects how the treatment works in real use.

References: Erford (Research and Evaluation in Counseling) — Experimental designs and attrition: intention-to-treat versus completer (per-protocol) analysis

☐ Approve ☐ Revise ☐ Retire Notes:

Q115 of 200 · nce-s-trt-110 · Professional Counseling Orientation and Ethical Practice · discharge and aftercare planning · easy · In review

A 38-year-old client with bipolar I disorder is stable and preparing for discharge from outpatient counseling. During past manic episodes he was hospitalized and could not make treatment decisions. While well, he wants to put in writing which hospital, medications and support people he prefers if this happens again. Which document BEST fits his goal?

A. A release of information form

A release authorizes sharing records; it does not record his treatment preferences for a future crisis.

B. A no-harm contract with the counselor

A no-harm contract is a promise about self-harm and does not state treatment preferences.

C. A financial power of attorney

A financial power of attorney covers money and property, not mental health treatment decisions.

D. A psychiatric advance directive ✓ KEY

A psychiatric advance directive records treatment preferences, and can name an agent, for times when he lacks capacity.

Rationale: A psychiatric advance directive is a legal document, made while a person has decision-making capacity, that states preferences for mental health treatment during a future crisis and may name a trusted person to make decisions. It supports recovery-oriented aftercare by giving the client a voice when he is most unwell. Rules on form, witnessing and how far providers must follow it vary by state.

References: State law — Psychiatric advance directive statutes: capacity, treatment preferences and health care agents for mental health crises

☐ Approve ☐ Revise ☐ Retire Notes:

A crisis center wants to lower suicide risk among adults in the months after they leave its crisis stabilization unit. Many of these adults do not attend their follow-up appointments. Which intervention has research support for this period?

A. Sending brief, caring messages at set times that ask nothing in return ✓ KEY

Caring contacts, short non-demanding messages of concern over many months, are associated with reduced suicidal behavior after a crisis.

B. Having each person sign a no-suicide contract before leaving the crisis unit

No-suicide contracts lack evidence of preventing suicide and can give false reassurance.

C. Closing the cases of those who miss two visits so staff can focus on those who come

People who disengage after a crisis are at high risk; closing their cases removes support when it is most needed.

D. Mailing each person a detailed pamphlet on depression symptoms on the day they leave

A single educational handout does not provide the ongoing connection that reduces risk after discharge.

Rationale: The weeks and months after discharge from crisis or inpatient care are a period of high suicide risk. Caring contacts, first studied by Motto with letters sent to people who had declined further treatment, are brief, personal messages expressing concern that ask nothing in return. Suicide prevention guidelines such as the VA/DoD guideline support offering them as a low-cost way to maintain connection.

References: VA/DoD CPG — Assessment and management of patients at risk for suicide: periodic caring communications after a suicidal crisis or hospitalization

☐ Approve ☐ Revise ☐ Retire Notes:

A 55-year-old client who has smoked since his teens says, "I've tried to quit a dozen times and nothing ever sticks. Some people just can't do it, and I'm one of them, so why bother?" Using DiClemente's description of precontemplation, which type of precontemplator does he MOST resemble?

A. Reluctant

Reluctant precontemplators lack knowledge or inertia keeps them from thinking about change; they do not describe repeated failed attempts.

B. Rebellious

Rebellious precontemplators resist being told what to do and are invested in their behavior, not hopeless about change.

C. Resigned ✓ KEY

Resigned precontemplators feel overwhelmed and hopeless after past failures and have given up on the idea of change.

D. Rationalizing

Rationalizing precontemplators have reasons why the behavior is not a problem; he accepts it is a problem but feels unable to change.

Rationale: DiClemente described four types of precontemplators, often called the four Rs: reluctant, rebellious, resigned and rationalizing. The resigned client has lost hope after many attempts and believes change is out of reach. Counseling for this client focuses on instilling hope, exploring what worked during past attempts and building self-efficacy rather than on persuading him that smoking is harmful.

References: Transtheoretical Model — Precontemplation subtypes (DiClemente): reluctant, rebellious, resigned and rationalizing

☐ Approve ☐ Revise ☐ Retire Notes:

A 62-year-old client stopped drinking seven years ago. She says she no longer feels any urge to drink, even at family parties or under stress, and is completely confident she will not go back to it. According to Prochaska and DiClemente, which stage does this describe?

A. Maintenance

In maintenance, the person still works to prevent relapse and still has some temptation in high-risk situations.

B. Termination ✓ KEY

Termination is defined by zero temptation and full self-efficacy across situations, so no effort is needed to avoid relapse.

C. Action

Action covers roughly the first six months of changed behavior, when effort and risk of relapse are highest.

D. Preparation

Preparation comes before change, when the person intends to act soon and may be taking small steps.

Rationale: In the Transtheoretical Model, termination is the stage in which the person has no temptation to return to the old behavior and complete confidence in all situations. Maintenance, which can last for years, still involves active effort to prevent relapse. Many people with addictive behaviors stay in maintenance long-term rather than reaching termination.

References: Transtheoretical Model — Termination stage: zero temptation and 100% self-efficacy, contrasted with maintenance

☐ Approve ☐ Revise ☐ Retire Notes:

To plan new services, a counseling agency interviews a small number of people well placed to know local needs: the police chief, two school principals, a pastor and a pediatrician. Which needs assessment approach is the agency using?

A. Community forum

A community forum is an open public meeting where any resident may speak, not chosen interviews.

B. Rates under treatment

This approach counts people already served by agencies, using service records rather than interviews.

C. Field survey

A field survey gathers data directly from a sample of the general population, not selected leaders.

D. Key informant ✓ KEY

The key informant approach gathers views from people whose roles give them broad knowledge of community needs.

Rationale: Needs assessment approaches include key informant, community forum, rates under treatment, social indicators and field survey methods. The key informant approach is quick and inexpensive, drawing on leaders and professionals who see many community members. Its main weakness is that informants may reflect their own roles and biases, so it is often combined with other methods.

References: Warheit et al. (Needs Assessment) — Key informant approach compared with community forum, rates-under-treatment, social indicators and field survey

☐ Approve ☐ Revise ☐ Retire Notes:

A researcher randomly assigns clients to four groups. Two groups take a pretest on self-esteem and two do not; within each pair, one group receives a new intervention and one does not. All four groups take the posttest. What is the MAIN purpose of this design?

A. To find out if taking the pretest itself changes how people do on the posttest ✓ **KEY**

The Solomon four-group design adds unpretested groups so pretest sensitization and its interaction with treatment can be checked.

B. To let each client serve as his or her own control across several conditions

That describes a within-subjects or crossover design, where each participant receives every condition.

C. To compare the effects of four different interventions within a single study

Only one intervention is tested; the four groups vary pretesting and treatment, not four treatments.

D. To follow up clients over time to learn how long the treatment gains will last

Durability of effects is studied with follow-up assessments, not by varying who takes a pretest.

Rationale: The Solomon four-group design combines a pretest-posttest control group design with a posttest-only control group design. Comparing pretested and unpretested groups shows whether the pretest itself sensitized participants or interacted with the treatment, a threat to external validity. It is a strong design, but it needs twice as many groups and participants.

References: Erford (Research and Evaluation in Counseling) — Experimental designs: Solomon four-group design and control of pretest sensitization

☐ Approve ☐ Revise ☐ Retire Notes:

In a study at one middle school, students in some homerooms receive a new conflict resolution curriculum while other homerooms serve as controls. Midway through, control-group teachers begin using the lessons after hearing about them in the staff lounge. Which threat to internal validity has occurred?

A. Statistical regression

Regression is the drift of extreme scores toward the mean on retesting, which is not described here.

B. Maturation

Maturation is natural change in participants over time, not a spread of the intervention.

C. Diffusion of treatment ✓ **KEY**

The control group received parts of the treatment, which shrinks the difference between groups.

D. Instrumentation

Instrumentation is a change in the measures or raters, not in what the control group received.

Rationale: Diffusion (or contamination) of treatment occurs when people in the control condition are exposed to the intervention, often because groups are in the same setting and share information. It makes the groups more alike and can hide a real effect. Researchers limit it by separating groups, for example by assigning whole schools rather than homerooms.

References: Erford (Research and Evaluation in Counseling) — Threats to internal validity: diffusion of treatment

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor gives an interval-scale anxiety measure to the same 25 clients at intake and again after ten sessions. She wants to know whether the average score changed significantly. Which statistical test is MOST appropriate?

A. Independent-samples t test

This test compares two separate groups of people, not the same people measured twice.

B. Paired-samples t test ✓ KEY

A paired (dependent) t test compares two means from the same participants, such as pretest and posttest.

C. Chi-square test

Chi-square tests frequencies in categories, not means of interval-scale scores.

D. Pearson correlation

A correlation shows how two variables relate, not whether the mean changed from pretest to posttest.

Rationale: When the same participants are measured twice on an interval or ratio variable, the scores are related, and a paired-samples (dependent) t test is used to compare the two means. An independent-samples t test would wrongly treat the pretest and posttest scores as coming from different people. With three or more time points, a repeated-measures ANOVA would be used instead.

References: Erford (Research and Evaluation in Counseling) — Inferential statistics: dependent (paired) samples t test for pretest-posttest comparisons

☐ Approve ☐ Revise ☐ Retire Notes:

An agency wants to know whether the share of clients who complete treatment (completed or dropped out) differs across its three outpatient programs. Each client is counted once, in one program. Which statistical test is MOST appropriate?

A. Paired-samples t test

A paired t test compares two means from the same people, and the outcome here is a category, not a score.

B. One-way ANOVA

ANOVA compares group means on an interval or ratio variable; completion is a yes or no category.

C. Pearson correlation

Pearson r requires two continuous variables, but both program and completion are categorical.

D. Chi-square test of independence ✓ KEY

Chi-square tests whether frequencies in one categorical variable differ across levels of another.

Rationale: When both variables are categorical, here program (three levels) and completion status (two levels), the data are counts in a contingency table. The chi-square test of independence compares the observed counts with those expected if the variables were unrelated. It is a nonparametric test and requires each person to appear in only one cell.

References: Erford (Research and Evaluation in Counseling) — Nonparametric statistics: chi-square test of independence for nominal data

☐ Approve ☐ Revise ☐ Retire Notes:

A 26-year-old Deaf client has been seeing a counselor fluent in American Sign Language for social anxiety. Her plan is due for review in two months. This week her father died suddenly, and she is now barely sleeping and missing work. What should the counselor do?

A. Revise the treatment plan with her now so it addresses her grief and new needs ✓ KEY

A significant change in the client's life is a reason to update the plan with her, rather than waiting for the scheduled review.

B. Keep to the current plan and take up her grief at the review in two months

Waiting leaves the plan out of step with her current needs during a time of acute distress.

C. Talk about her grief in sessions but leave the plan and the notes as they are

Treating new problems without updating the plan leaves the record inaccurate and the work unguided by goals.

D. Refer her to a grief counselor and pause the anxiety work until she feels better

An abrupt referral can disrupt a working alliance; the current counselor can address her grief within a revised plan.

Rationale: A treatment plan is a living document. It is reviewed on a set schedule and also whenever the client's situation changes in an important way, such as a death, a crisis or a new diagnosis. Revising the plan together keeps the goals relevant to what the client needs now and keeps the record accurate.

References: Wiger (Documentation) — Treatment plan updates: revising goals and objectives when the client's needs change significantly

☐ Approve ☐ Revise ☐ Retire Notes:

Counseling Skills and Interventions

60 items · NCE weight 30% (48 of 160 scored items)

Q125 of 200 · nce-s-cou-361 · Counseling and Helping Relationships · psychodynamic and psychoanalytic theory · easy · In review

A 6-year-old who has been openly jealous since his baby sister was born now hugs her so often and so hard that she cries, and he loudly tells every visitor how much he loves her. Which defense mechanism does this BEST illustrate?

A. Sublimation

Sublimation channels an unacceptable impulse into a socially valued activity, such as art or sport, not into its exaggerated opposite.

B. Projection

Projection attributes one's own impulse to someone else; he is not claiming that his sister is jealous of him.

C. Reaction formation ✓ **KEY**

He wards off unacceptable hostility by showing the opposite feeling in an exaggerated, rigid way, and the hostility leaks through.

D. Regression

Regression is a return to behavior from an earlier stage, such as bed-wetting or baby talk, which is not described here.

Rationale: In reaction formation, a person defends against a threatening impulse by actively expressing its opposite. The display is often excessive and inflexible, which is a clue that it is defensive rather than genuine. Here the overdone affection keeps jealousy out of awareness, while the rough hugs hint at the hostility underneath.

References: Corey (Theory & Practice) — Psychoanalytic therapy: ego-defense mechanisms (reaction formation)

☐ Approve ☐ Revise ☐ Retire Notes:

Q126 of 200 · nce-s-cou-362 · Counseling and Helping Relationships · psychodynamic and psychoanalytic theory · hard · In review

A 35-year-old client raised in a strict household feels intense guilt and dread after she skips a family dinner to rest, even though no one has criticized her and nothing bad has happened. According to Freud's three types of anxiety, which type is she MOST likely experiencing?

A. Moral anxiety, a fear of her own conscience ✓ **KEY**

Moral anxiety arises when the superego threatens to punish the ego for acting against internalized standards, and it is felt as guilt.

B. Reality anxiety, a fear of a real outside threat

Reality anxiety is proportionate fear of an actual external danger; no one has criticized or threatened her.

C. Neurotic anxiety, a fear of id urges breaking out

Neurotic anxiety is fear that instinctual impulses will break through and bring punishment; her distress centers on violated standards.

D. Separation anxiety, a fear of losing an attachment

Separation anxiety is not one of Freud's three types of anxiety, and her fear is not about losing a caregiver.

Rationale: Freud described three kinds of anxiety, each tied to a source of threat to the ego. Reality anxiety responds to dangers in the outside world, neurotic anxiety to the fear that id impulses will escape control, and moral anxiety to the fear of one's own conscience. Guilt after acting against internalized rules, with no outside threat present, points to moral anxiety from the superego.

References: Corey (Theory & Practice) — Psychoanalytic therapy: anxiety (reality, neurotic and moral anxiety)

☐ Approve ☐ Revise ☐ Retire Notes:

Over several sessions, an Adlerian counselor and a 42-year-old client have explored his family constellation and early memories, and he now understands his belief that he must please everyone to belong. The counselor next helps him try new behaviors and make different choices outside sessions. Which phase of Adlerian counseling is this?

A. Establishing the therapeutic relationship

Building a cooperative, egalitarian relationship is the first phase, and it is already in place here.

B. Exploring the client's psychological dynamics

Assessment of the family constellation and early memories has already been completed.

C. Encouraging self-understanding and insight

He has already gained insight into his mistaken belief; the work now moves from insight to action.

D. Reorientation and reeducation of the client ✓ KEY

The final phase puts insight into action, as the client is encouraged to take new risks and make new choices.

Rationale: Adlerian counseling is commonly described in four phases: forming the relationship, assessing psychological dynamics through the lifestyle assessment, encouraging insight, and reorientation and reeducation. In reorientation, the counselor uses encouragement and action-oriented techniques so the client can act on new understanding. Insight alone is not seen as enough; change shows up in new behavior.

References: Corey (Theory & Practice) — Adlerian therapy: the four phases of the therapeutic process (reorientation and reeducation)

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 28-year-old client is thriving in her career but says she has no close friends and avoids dating. Her Adlerian counselor wants to assess how she is meeting the three life tasks that Adler described. Which set names those tasks?

A. Trust, autonomy and initiative

These are outcomes of Erikson's first three psychosocial stages, not Adler's life tasks.

B. Work, friendship and love ✓ KEY

Adler held that everyone must meet the tasks of occupation, society or friendship, and love or intimacy.

C. Safety, esteem and belonging

These are levels in Maslow's hierarchy of needs rather than Adler's tasks.

D. Freedom, fun and power

These are among Glasser's basic needs in choice theory, not Adler's life tasks.

Rationale: Adler proposed that each person must address three universal life tasks: work, friendship (society) and love (intimacy). Later Adlerians such as Dreikurs and Mosak added tasks related to spirituality and getting along with oneself. Difficulty in one task, like this client's avoidance of friendship and intimacy, often reflects mistaken beliefs in her lifestyle.

References: Corey (Theory & Practice) — Adlerian therapy: life tasks of work, friendship and love

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor invites a 47-year-old client to move to music, then draw whatever the movement stirred, then write freely about the drawing, with no judging of skill and with steady empathy and acceptance. Developed by Natalie Rogers, this approach extends her father Carl Rogers's core conditions. What is it called?

A. Person-centered expressive arts therapy ✓ KEY

Natalie Rogers built this approach on her father's core conditions, linking art forms in sequence, which she called the creative connection.

B. Psychoanalytic art therapy (Naumburg)

Naumburg's approach treats images as symbolic speech from the unconscious for interpretation, not as nondirective, linked expression.

C. Psychodrama with role enactment (Moreno)

Psychodrama has clients act out scenes with a director and auxiliary egos; it does not chain movement, art and writing.

D. Gestalt dream work, playing each part

Gestalt dream work has the client speak as each part of a dream, which is not the sequence described.

Rationale: Natalie Rogers developed person-centered expressive arts therapy, which applies empathy, congruence and unconditional positive regard to the use of movement, art, writing, sound and other media. She described the creative connection, in which one art form stirs feelings that feed the next, deepening self-awareness. The focus is on the client's own process and meaning, not on artistic skill or counselor interpretation.

References: Corey (Theory & Practice) — Person-centered therapy: Natalie Rogers's person-centered expressive arts therapy and the creative connection

☐ Approve ☐ Revise ☐ Retire Notes:

A new agency counselor trained in the person-centered approach asks her supervisor why traditional diagnosis and extensive formal testing play only a small role in her model. Which explanation BEST reflects Rogers's view?

A. Testing is too time-consuming to fit within a short course of counseling

Rogers's objection was about the stance diagnosis creates, not about how long tests take.

B. Diagnosis is valid only when it is made by a psychiatrist or psychologist

Rogers did not argue that only certain professions may diagnose; he questioned the expert role itself.

C. They cast the counselor as outside expert and shift focus off the client's view ✓ KEY

Rogers worried that diagnosis puts the locus of evaluation in the counselor and pulls attention from the client's own experience.

D. Test results lack the reliability needed to guide any treatment decisions

Rogers was not mainly critiquing test reliability; his concern was the effect on the relationship.

Rationale: Person-centered counselors see the client, not the counselor, as the best expert on the client's life. Rogers believed that diagnosis and heavy reliance on testing place the counselor in an authoritative, evaluating role and can encourage an external locus of evaluation. Understanding comes instead from entering the client's subjective world, though many person-centered counselors still diagnose when settings require it.

References: Corey (Theory & Practice) — Person-centered therapy: role of assessment and diagnosis

☐ Approve ☐ Revise ☐ Retire Notes:

A 39-year-old client in a loving marriage, with close friends, says, "Even with all of them, there is a part of me no one can ever fully reach. I came into the world alone and I'll leave it alone." Yalom distinguished three kinds of isolation. Which kind is the client describing?

A. Interpersonal isolation from other people

Interpersonal isolation is loneliness from a lack of relationships or social skills; she has close ties.

B. Existential isolation, an unbridgeable gulf ✓ KEY

Existential isolation is the basic separateness of each person from others and the world, which no relationship can erase.

C. Intrapersonal isolation within the self

Intrapersonal isolation is being cut off from parts of oneself, such as through repressed feelings or wishes.

D. Social isolation caused by stigma or exclusion

Exclusion by others is not one of Yalom's three forms, and she reports being well connected.

Rationale: Yalom described interpersonal isolation (separation from others), intrapersonal isolation (separation from parts of oneself) and existential isolation, the deepest form. Existential isolation refers to the unbridgeable gap between oneself and any other being, since each person enters and leaves existence alone. Existential counselors help clients face this reality and relate to others out of choice rather than out of fear of being alone.

References: Corey (Theory & Practice) — Existential therapy: Yalom on isolation as an ultimate concern

☐ Approve ☐ Revise ☐ Retire Notes:

A 33-year-old man reports problems with sexual arousal. He describes constantly watching his own body for signs of failure during intimacy. His logotherapist encourages him to turn his attention away from himself and toward his partner and the shared experience. Which technique of Viktor Frankl is this?

A. Paradoxical intention

Paradoxical intention asks the client to wish for or deliberately try to produce the feared symptom, which is not what is described.

B. Socratic dialogue

Socratic dialogue uses questions to help clients discover meaning; it does not redirect attention during the problem behavior.

C. Attitude modification

Attitude modification aims to change a client's stance toward unavoidable suffering, not to shift attention away from the self.

D. Dereflection ✓ KEY

Dereflection counters hyperreflection, excessive self-observation, by redirecting attention away from the self toward other people or meanings.

Rationale: Frankl observed that some problems, such as sexual dysfunction and insomnia, are worsened by hyperreflection, an anxious focus on oneself and one's performance. Dereflection breaks this cycle by turning the client's attention outward, toward a partner, a task or a meaning beyond the self. It differs from paradoxical intention, which deliberately invites the feared symptom.

References: Corey (Theory & Practice) — Existential therapy: Frankl's logotherapy, dereflection and hyperreflection

☐ Approve ☐ Revise ☐ Retire Notes:

A client who describes himself as "easygoing" keeps complaining that his coworkers are "so judgmental" and "always criticizing everyone." His gestalt counselor asks him to repeat each sentence about his coworkers but begin it with "I," as in "I am so judgmental." Which contact disturbance is the counselor addressing?

A. Projection ✓ KEY

In projection, a person disowns aspects of the self and attributes them to others; restating them with "I" helps him reown them.

B. Introjection

Introjection is swallowing others' standards without examining them, not attributing one's own traits to others.

C. Retroflexion

Retroflexion is turning onto oneself what one would like to do to others.

D. Confluence

Confluence blurs the boundary between self and others so differences are not experienced.

Rationale: Gestalt therapists view projection as disowning parts of oneself and seeing them in the environment instead. A common intervention is to have the client restate what he sees in others as statements about himself, which helps him notice and take responsibility for the disowned trait. The goal is greater awareness and integration rather than interpretation by the counselor.

References: Corey (Theory & Practice) — Gestalt therapy: contact and resistances to contact (projection)

☐ Approve ☐ Revise ☐ Retire Notes:

In a gestalt-oriented group, a member says she is sure nobody in the room really trusts her. The leader invites her to go to each member in turn and say something to them, such as "I'm afraid you don't trust me because..." completing the sentence for each person. What is this exercise called?

A. Empty chair

The empty chair has a client speak to an imagined person or part of the self, not to each real group member.

B. Reversal

Reversal asks a client to act out the opposite of a usual behavior, such as a timid member playing a bold one.

C. Making the rounds ✓ KEY

In making the rounds, a member speaks to or does something with each other member to explore a theme or take a risk.

D. Rehearsal

Rehearsal is practicing aloud what one has been silently preparing to say, not addressing each member in turn.

Rationale: Making the rounds is a gestalt group exercise in which one member goes around the group, speaking to or doing something with each person. It is used when a member voices a feeling about the group as a whole, such as mistrust, so the feeling can be checked directly with each member. The exercise builds awareness and gives the member a chance to take a risk in the moment.

References: Corey (Theory & Practice) — Gestalt therapy: experiments and exercises (making the rounds)

☐ Approve ☐ Revise ☐ Retire Notes:

A 9-year-old watches a skilled peer model a complex juggling routine. He pays close attention, can describe each step afterward and badly wants to perform it, yet he cannot do the routine because he lacks the coordination. In Bandura's model of observational learning, which process is limiting his performance?

A. Attention

He watched closely, so attention to the model was not the problem.

B. Retention

He can describe each step, showing that he has stored the behavior in memory.

C. Motivation

He badly wants to perform the routine, so motivation is present.

D. Reproduction ✓ KEY

Reproduction requires the physical capability to turn the stored image into action, which he lacks.

Rationale: Bandura identified four processes in observational learning: attention, retention, motor reproduction and motivation. A learner may attend to a model, remember the behavior and want to imitate it, yet still fail if he lacks the skills to reproduce it. In counseling, modeling is often paired with guided practice and feedback to build the capacity to reproduce new behavior.

References: Corey (Theory & Practice) — Behavior therapy: social learning (social-cognitive) approach and modeling

☐ Approve ☐ Revise ☐ Retire Notes:

A 50-year-old client who wants to cut back on late-night snacking agrees to write down, each time she snacks, the time, what she ate, where she was and how she felt. Within a week, before any other intervention, her snacking drops. Which statement BEST explains this?

A. Writing in a log serves as a punisher that is applied each time she eats

The log is not set up as an aversive consequence; the change comes from heightened awareness.

B. Self-monitoring can be reactive, as the act of recording changes behavior ✓ KEY

Self-monitoring is a core step in self-management, and recording a behavior often changes it in the desired direction.

C. The log has extinguished snacking by removing the reward that keeps it up

Extinction requires withholding the reinforcer; the snacks themselves were still available and eaten.

D. Recording her moods has desensitized her to cues that prompt late eating

Desensitization pairs relaxation with graded exposure; writing a log does not do this.

Rationale: Self-management programs typically include choosing a goal, defining target behaviors, self-monitoring, building a plan for change and evaluating progress. Self-monitoring, the careful recording of when and where a behavior occurs, gives data for the plan. It is also reactive: simply observing and recording a behavior often shifts it toward the client's goal.

References: Corey (Theory & Practice) — Behavior therapy: self-management programs and self-monitoring

☐ Approve ☐ Revise ☐ Retire Notes:

Early in a typical cognitive behavioral session, after a brief mood check, the counselor asks a 31-year-old client, "What problems or topics would you most like to work on today?" and the two decide together how to use the time. Which part of a CBT session is this?

A. Assigning homework

Homework, or an action plan, is usually set near the end of the session, not at the start.

B. Setting the agenda ✓ KEY

Collaborative agenda setting early in the session decides which problems the session will address.

C. Eliciting feedback

Feedback on the session is asked for at the end, after work on the agenda is done.

D. Final summary

A final summary pulls together the session's main points and comes near the close.

Rationale: J. Beck describes a structured CBT session that typically includes a mood check, setting the agenda, reviewing the action plan, working on agenda items, summarizing, setting new homework and getting feedback. Setting the agenda together makes the most of limited time and models collaboration and problem solving. Structure is a defining feature of CBT, though the counselor adapts it to each client.

References: J. Beck (CBT: Basics and Beyond) — Structure of the therapy session: setting the agenda

☐ Approve ☐ Revise ☐ Retire Notes:

A 24-year-old client predicts, "If I speak up in my team meeting, people will roll their eyes and ignore me." He and his CBT counselor agree that he will make one comment at Thursday's meeting, note exactly how people respond and compare it with his prediction. What is this intervention?

A. A behavioral experiment to test the accuracy of his prediction ✓ KEY

Behavioral experiments set up real-world tests of a specific belief and compare the outcome with the prediction.

B. Flooding with full exposure to his most feared social situation

Flooding is prolonged exposure to a highly feared stimulus; one planned comment to test a belief is not this.

C. Thought stopping to interrupt the negative thought as it arises

Thought stopping suppresses thoughts; this plan gathers evidence about the thought instead.

D. Activity scheduling to raise his overall level of pleasant events

Activity scheduling targets mood through rewarding activities, not testing a particular prediction.

Rationale: In CBT, a behavioral experiment is a planned activity that tests the validity of a thought or belief in real life. The client states the prediction, carries out the experiment and then compares what happened with what he expected. Evidence gathered firsthand is often more convincing than evidence discussed in session.

References: J. Beck (CBT: Basics and Beyond) — Behavioral experiments to test automatic thoughts and beliefs

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "My boss must approve of everything I do, and if she doesn't, it shows I'm incompetent." Her REBT counselor asks, "Where is the evidence that your boss has to approve of everything you do? Is there any proof one bit of criticism means you are incompetent?" Which type of disputing is this?

A. Pragmatic disputing of how useful the belief is

Pragmatic disputing asks where a belief gets the client and how it helps or hurts her, not whether evidence supports it.

B. Logical disputing of whether the belief follows

Logical disputing asks whether a demand follows logically from a preference, not whether evidence exists for it.

C. Empirical disputing of the facts behind a belief ✓ KEY

Empirical disputing asks for evidence that the belief is consistent with reality.

D. Imaginal disputing using a vivid mental scene

Imagery techniques use vivid mental scenes, while this response asks direct questions about evidence.

Rationale: REBT counselors dispute irrational beliefs in several ways. Empirical disputing asks whether there is evidence for the belief, logical disputing asks whether it follows from the client's preferences, and pragmatic disputing asks whether it helps the client reach her goals. Questions like "Where is the evidence?" mark the empirical approach.

References: Ellis & Dryden (REBT) — Disputing irrational beliefs: empirical, logical and pragmatic arguments

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor trained in person-centered work begins learning REBT and asks how Albert Ellis viewed counselor warmth and a close, nurturing relationship. Which statement BEST reflects Ellis's position?

A. Warmth is the main ingredient of change, so it should come before disputing

This describes a relationship-centered view; Ellis placed the active disputing of beliefs at the center of change.

B. Warmth is suited only to children, and adults respond best to firm debate

Ellis did not divide clients by age this way; his caution about warmth applied broadly.

C. Warmth is useful only once the client has fully adopted rational beliefs

Ellis did not make warmth a reward for progress; he questioned its value as a change agent at any point.

D. Full acceptance is vital, but much warmth may feed a dire need for approval ✓ KEY

Ellis valued unconditional acceptance yet warned that too much warmth can reinforce clients' need for love and approval.

Rationale: Ellis held that the counselor should accept clients unconditionally as fallible humans, but he did not see a warm, close relationship as necessary or sufficient for change. He cautioned that excessive warmth may foster dependence and reinforce the irrational belief that one must be approved of. In REBT, change comes mainly from actively disputing irrational beliefs and practicing new ones.

References: Corey (Theory & Practice) — REBT: the therapist-client relationship and the role of warmth

☐ Approve ☐ Revise ☐ Retire Notes:

A client in reality therapy offers this plan for the coming week: "I'm going to get my husband to stop criticizing me at dinner." Using Wubbolding's characteristics of an effective plan (SAMIC), what is the MAIN problem with this plan?

A. It is not controlled by the planner, since it depends on her husband ✓ KEY

Effective plans are within the client's own control; she cannot control her husband's behavior.

B. It is not immediate, since it is set for some time in the distant future

The plan is for the coming week, so it is timely enough to be acted on soon.

C. It is too simple, since good plans need several detailed parts to work

Wubbolding favors simple plans; simplicity is a strength, not the flaw here.

D. It lacks evaluation, since she has not rated her wants on a 1-10 scale

Self-evaluation is part of WDEP, but rating on a scale is not a requirement for an effective plan.

Rationale: Wubbolding summarized effective plans with SAMIC: simple, attainable, measurable, immediate, controlled by the planner, committed to and consistently done. Because choice theory holds that we can control only our own behavior, a plan that depends on changing someone else is likely to fail. The counselor would help her craft a plan about what she herself will do at dinner.

References: Corey (Theory & Practice) — Reality therapy: Wubbolding's characteristics of effective plans (SAMIC)

☐ Approve ☐ Revise ☐ Retire Notes:

A middle school principal relies on detentions, threats and reward charts to make students behave. William Glasser would describe this way of trying to make others act as we want with which term?

A. Positive behavior support

This is a school-wide behavioral framework, not Glasser's term for coercive control.

B. Logical consequences

Logical consequences is an Adlerian and Dreikurs concept, not Glasser's critique.

C. Token economy program

A token economy is a behavioral system of earned rewards, not Glasser's broader term.

D. External control psychology ✓ KEY

Glasser used this term for the belief that people can be made to behave through rewards, punishment and coercion.

Rationale: Glasser contrasted choice theory with external control psychology, the common belief that we can and should control others through rewards, punishments and pressure. He argued that external control damages relationships and that people choose their own behavior to meet their needs. He applied these ideas to schools as well as counseling.

References: Corey (Theory & Practice) — Reality therapy: choice theory versus external control psychology

☐ Approve ☐ Revise ☐ Retire Notes:

A grandmother raising her 12-year-old grandson describes his defiance in detail and clearly wants things to change, but she believes only he can fix it and sees no role for herself. In de Shazer's terms, what is this relationship and what task fits it BEST?

A. Customer; ask her to try a new action at home this week

A customer sees herself as part of the solution and is ready to act; she does not yet see a role for herself.

B. Visitor; compliment her and give no tasks at this time

A visitor does not see a problem to work on; she clearly names a problem she wants solved.

C. Complainant; compliment her and give an observing task ✓ **KEY**

A complainant describes a problem but not her part in solving it, so observation tasks fit best.

D. Customer; teach her a new discipline plan to use with him

Teaching a new plan assumes customer-level readiness and puts the counselor in the expert role.

Rationale: de Shazer described three kinds of client-counselor relationships. Visitors do not see a problem to work on, complainants describe a problem but do not yet see themselves as part of the solution, and customers are ready to act. For a complainant, the counselor compliments her concern and assigns observing tasks, such as noticing when her grandson is more cooperative.

References: Corey (Theory & Practice) — Solution-focused brief therapy: customer, complainant and visitor relationships

☐ Approve ☐ Revise ☐ Retire Notes:

At the start of a first session, a solution-focused counselor asks a 45-year-old client, "Between the time you called to make this appointment and today, what have you noticed that's already a little different or better?" What is the main purpose of this question?

A. To check symptom severity before setting a diagnosis

Solution-focused counselors do not center the work on diagnosis or symptom severity.

B. To find pre-session change the client can build on ✓ **KEY**

The question looks for improvement that happened before therapy began, highlighting the client's own resources.

C. To learn how ready the client is to accept homework

The question is about change already under way, not willingness to accept tasks.

D. To uncover the root cause of the presenting problem

Solution-focused therapy does not search for causes of problems.

Rationale: Solution-focused counselors often ask about pre-session (pre-therapy) change, since many clients notice some improvement between scheduling and the first session. Highlighting that change shows the client that change is already happening and that she contributed to it. The counselor then asks what she did to make it happen and how to do more of it.

References: Corey (Theory & Practice) — Solution-focused brief therapy: pretherapy change

☐ Approve ☐ Revise ☐ Retire Notes:

A narrative counselor helps a 17-year-old move step by step from naming small, familiar events, to linking them, to drawing lessons from them and, finally, to forming plans based on those lessons. Each question stretches just beyond what the client can already know on her own. Michael White based this "scaffolding" map on which theorist's idea?

A. Vygotsky's zone of proximal development ✓ KEY

White drew on Vygotsky's idea that learning happens in the gap between what one knows and what one can reach with support.

B. Piaget's process of cognitive accommodation

Accommodation is Piaget's term for changing schemas; White's scaffolding map draws on Vygotsky.

C. Bandura's concept of perceived self-efficacy

Self-efficacy is belief in one's capability; it is not the basis of the scaffolding map.

D. Erikson's identity vs. role confusion

Erikson's adolescent stage concerns identity, not structured steps of learning with support.

Rationale: In Maps of Narrative Practice, White describes scaffolding conversations based on Vygotsky's zone of proximal development. The counselor asks questions that help the client move in manageable steps from the known and familiar toward what is possible to know. This lets the client develop new concepts about her life and plans for action that she could not reach alone.

References: White (Maps of Narrative Practice) — Scaffolding conversations and the zone of proximal development

☐ Approve ☐ Revise ☐ Retire Notes:

In a first session, a 54-year-old man describes his life only in terms of his depression: lost jobs, missed events and "letting everyone down." Other parts of his life barely appear in his account. Narrative therapists have a name for this kind of account. What is it?

A. A unique outcome

A unique outcome is an event that contradicts the problem story, the opposite of what he is describing.

B. An alternative story

An alternative story is the preferred account built later from unique outcomes.

C. A definitional ceremony

A definitional ceremony uses outsider witnesses to acknowledge a client's preferred story.

D. A problem-saturated story ✓ KEY

A problem-saturated story is a dominant account in which the problem defines the person's life and identity.

Rationale: Narrative therapists describe the accounts clients bring as often problem-saturated, meaning the problem dominates how they see their lives and themselves. Such stories leave out experiences that do not fit the problem. The counselor externalizes the problem and searches for unique outcomes that can grow into a richer alternative story.

References: Corey (Theory & Practice) — Narrative therapy: problem-saturated stories and alternative stories

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old Korean American client says she longs to feel close to her friends, yet after past betrayals she hides her sadness and needs from them to avoid being hurt again. She notices this leaves her feeling even more alone. In relational-cultural theory, what is this dynamic called?

A. Mutual empowerment between friends

Mutual empowerment is an outcome of growth-fostering relationships, the opposite of the self-protective hiding she describes.

B. Confluence with the friend group

Confluence is a gestalt term for blurring the boundary between self and others; she is holding herself apart.

C. The central relational paradox ✓ KEY

RCT describes yearning for connection while keeping parts of oneself out of relationships for safety, which deepens disconnection.

D. An internalized gender-role message

Gender-role messages are explored in gender-role analysis; her pattern stems from relational hurt, not a gendered rule.

Rationale: Relational-cultural theory, developed by Jean Baker Miller, Judith Jordan and colleagues, holds that people grow through connection. After hurtful or disconnecting experiences, people may use strategies of disconnection, hiding parts of themselves to stay safe in relationships. This central relational paradox, seeking connection while guarding against it, leads to further isolation, and counseling aims to rebuild the capacity for authentic, mutual connection.

References: Corey (Theory & Practice) — Feminist therapy: relational-cultural theory, strategies of disconnection and the central relational paradox

☐ Approve ☐ Revise ☐ Retire Notes:

A 20-year-old college student who survived a sexual assault decides, with her feminist counselor's support, to join a campus group that lobbies for better reporting procedures and prevention programs. Which feminist therapy technique does this BEST illustrate?

A. Bibliotherapy

Bibliotherapy uses books and other media to inform clients, not involvement in advocacy work.

B. Social action ✓ KEY

Feminist counselors may encourage clients to take part in activities that work to change oppressive conditions.

C. Reframing

Reframing changes how a problem is understood, but here the client is taking action in her community.

D. Self-disclosure

Self-disclosure is the counselor sharing personal experience, not the client joining an advocacy group.

Rationale: Feminist therapy holds that the personal is political and is committed to social change, not only individual adjustment. Social action, such as volunteering at a crisis center or lobbying for policy change, can empower clients and help them see their experiences in a social context. The counselor supports, but does not require, such involvement.

References: Corey (Theory & Practice) — Feminist therapy: techniques and strategies (social action)

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old man with diabetes says he is unsure about changing his diet. The new counselor quickly lists the risks of high blood sugar and urges him to start a meal plan, and the client begins arguing for why change is not realistic. In motivational interviewing terms, what has the counselor done?

A. Developed discrepancy between his values and his current diet

Developing discrepancy evokes the client's own values; the counselor here is supplying arguments.

B. Acted on the righting reflex, which drew out more sustain talk ✓ **KEY**

The urge to fix the problem and argue for change led the ambivalent client to voice the other side.

C. Rolled with resistance by allowing him to argue against change

Rolling with discord means avoiding argument; the counselor provoked it instead.

D. Moved properly into the planning process after the focus was set

Planning comes after change talk and readiness emerge, which has not happened yet.

Rationale: Miller and Rollnick describe the righting reflex as the helper's natural urge to set things right by advising, warning or persuading. With an ambivalent client, arguing for change tends to prompt the client to argue for the other side, producing sustain talk. MI asks the counselor to resist this reflex and instead evoke the client's own reasons for change.

References: Miller & Rollnick (MI) — The righting reflex and ambivalence; sustain talk elicited by persuasion

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old client says, "My drinking is fine. I don't see why everyone is making such a fuss about it." The counselor replies in a calm, sincere tone, "So there's really nothing about your drinking that concerns you at all." Which MI response is this?

A. Amplified reflection ✓ **KEY**

It restates the client's sustain talk in a slightly stronger form, which often prompts the client to voice the other side.

B. Double-sided reflection

A double-sided reflection names both sides of ambivalence; this reply names only one side.

C. Reframing the meaning

Reframing offers a new meaning for what the client said; this reply overstates his own meaning.

D. Emphasizing autonomy

Emphasizing autonomy affirms the client's right to choose; this reply does not address choice.

Rationale: An amplified reflection reflects the client's statement back in a somewhat exaggerated form. Said without sarcasm, it often leads the client to back away from the extreme and voice some concern, which is change talk. Tone matters: sarcasm turns an amplified reflection into a confrontation that increases discord.

References: Miller & Rollnick (MI) — Responding to sustain talk and discord: amplified reflection

☐ Approve ☐ Revise ☐ Retire Notes:

A client in a DBT skills group often agrees to cover coworkers' shifts, apologizes repeatedly and shades the truth so others won't be upset, then feels ashamed afterward. Her main goal is to respect herself after these talks. Which interpersonal effectiveness skill fits this goal BEST?

A. GIVE (gentle, interested, validate, easy manner)

GIVE targets relationship effectiveness, keeping the relationship strong, not self-respect.

B. DEAR MAN (describe, express, assert, reinforce)

DEAR MAN targets objectives effectiveness, getting what one asks for, not keeping self-respect.

C. PLEASE (physical illness, eating, drugs, sleep, exercise)

PLEASE is an emotion regulation skill for reducing vulnerability, not interpersonal effectiveness.

D. FAST (fair, no apologies, stick to values, truthful) ✓ **KEY**

FAST targets self-respect effectiveness: being fair, not over-apologizing, sticking to values and being truthful.

Rationale: DBT interpersonal effectiveness has three sets of skills tied to three goals. DEAR MAN serves objectives effectiveness, GIVE serves relationship effectiveness and FAST serves self-respect effectiveness. A client who over-apologizes and shades the truth and then feels ashamed would benefit most from FAST.

References: Linehan (DBT Skills Training Manual) — Interpersonal effectiveness: FAST skills for self-respect effectiveness

☐ Approve ☐ Revise ☐ Retire Notes:

In a comprehensive DBT program, individual therapists and skills trainers meet weekly as a consultation team, where they discuss their own frustration and burnout and help one another stay effective with high-risk clients. Which function of comprehensive DBT does this team MAINLY serve?

A. Enhancing clients' capabilities through skills training

Building client skills is the main function of skills training groups, not the consultation team.

B. Generalizing skills into the client's daily environment

Generalization is mainly served by between-session phone coaching and practice.

C. Enhancing therapists' capabilities and motivation ✓ **KEY**

The consultation team serves as "therapy for the therapists," supporting skill, motivation and adherence.

D. Structuring the environment through case management

Structuring the environment involves arranging the treatment and home settings, not the team's mutual support.

Rationale: Comprehensive DBT addresses five functions: enhancing client capabilities, improving client motivation, ensuring generalization, structuring the environment and enhancing therapist capabilities and motivation. The consultation team serves the last function. Treating chronically suicidal clients is demanding, so the team supports therapists so they can keep delivering the treatment well.

References: Linehan (DBT Skills Training Manual) — Functions and modes of comprehensive DBT: therapist consultation team

☐ Approve ☐ Revise ☐ Retire Notes:

During a tense holiday visit, a 38-year-old woman stays calm as her parents argue, states her own view of a family decision without attacking anyone and remains warmly connected to both of them. A Bowenian counselor would see this as evidence of what?

A. A high level of differentiation of self ✓ **KEY**

Differentiation is the ability to keep thinking and feeling distinct and hold one's own position while staying connected.

B. An emotional cutoff from her family

Cutoff manages fusion by distancing or ending contact; she stays warmly connected.

C. A rigid boundary around her subsystem

Rigid boundaries are a structural concept describing disengagement, not her calm self-definition.

D. Emotional fusion with both her parents

Fusion would show as being swept into the conflict or reacting automatically, which she avoids.

Rationale: Bowen's central concept, differentiation of self, refers to the ability to separate one's intellectual and emotional functioning and to hold an "I-position" while remaining in contact with others. Well-differentiated people can stay calm and thoughtful amid family anxiety without either fusing with others or cutting them off. Raising differentiation is the main goal of Bowenian therapy.

References: Goldenberg (Family Therapy) — Bowen family systems theory: differentiation of self

☐ Approve ☐ Revise ☐ Retire Notes:

Q154 of 200 · nce-s-cou-390 · Counseling and Helping Relationships · family systems theories (Bowen, structural, strategic) · easy · In review

A Bowenian counselor explains that when spouses who are poorly differentiated face chronic anxiety, the fusion between them tends to show up in a few predictable ways: open marital conflict, illness or dysfunction in one spouse, impairment in a child, or emotional distance. Which Bowen concept describes these patterns?

A. Emotional cutoff

Emotional cutoff is reducing or ending contact with the family of origin to manage fusion, not the set of patterns within the nuclear family.

B. Nuclear family emotional process ✓ **KEY**

This concept names the patterns through which a couple's unresolved fusion and anxiety are expressed in the nuclear family.

C. Societal emotional process

Societal emotional process extends Bowen's ideas to how anxiety and regression operate in society as a whole.

D. Sibling position

Sibling position, drawn from Toman, links birth order to characteristic roles, not to how couples handle fusion.

Rationale: Bowen described the nuclear family emotional process (originally the nuclear family emotional system) as the ways a family manages the anxiety and undifferentiation that spouses bring from their families of origin. The common patterns are marital conflict, dysfunction in one spouse, projection of the problem onto one or more children, and emotional distance. The lower the spouses' differentiation, the more intensely these patterns appear.

References: Goldenberg (Family Therapy) — Bowen family systems theory: nuclear family emotional process (system)

☐ Approve ☐ Revise ☐ Retire Notes:

A supervisor watching a trainee's session notes that the trainee keeps good eye contact and stays on the client's topic, but speaks quickly and loudly whenever the client becomes upset. Which dimension of Ivey's attending behavior does the supervisor's feedback address?

A. Verbal tracking

Verbal tracking means staying with the client's topic, which the trainee already does well.

B. Vocal qualities ✓ **KEY**

Vocal qualities cover rate, volume and tone of voice; a rushed, loud voice can unsettle an upset client.

C. Visual contact

Eye contact is the visual component, and the supervisor reports it is appropriate.

D. Selective attention

Selective attention concerns which topics the counselor responds to, not how the counselor's voice sounds.

Rationale: Ivey summarizes attending behavior as the "3 V's + B": visuals (culturally appropriate eye contact), vocal qualities, verbal tracking and attentive body language. Vocal qualities include speech rate, volume and tone, and changes in them signal comfort or discomfort. A calm, even voice helps an upset client feel heard and settled.

References: Ivey et al. (Intentional Interviewing) — Attending behavior: the 3 V's + B (visuals, vocal qualities, verbal tracking, body language)

☐ Approve ☐ Revise ☐ Retire Notes:

A 38-year-old client says, "Things with my partner have just been off lately. It's hard to explain." The counselor wants to move the conversation from this general statement to something they can actually work on. Which response BEST does this?

A. "Why do you think things feel off with your partner lately?"

A "why" question asks for an explanation the client has said is hard to give and can sound demanding.

B. "It sounds like you're feeling distant from your partner."

This guesses at a feeling the client has not described; it does not draw out specific detail.

C. "Is your relationship with your partner going well or not?"

This closed question invites a yes-or-no reply and adds nothing concrete to the picture.

D. "Could you give me a recent example of a time it felt off?" ✓ **KEY**

Asking for a specific example builds concreteness, turning a vague statement into a situation that can be explored.

Rationale: Concreteness, a core condition in Carkhuff's model and a skill Ivey teaches through questions, helps clients move from vague generalities to specific examples. Questions such as "Could you give me a specific example?" bring out the who, what, when and where of a concern. Specific material makes it easier to understand the problem and to set goals.

References: Ivey et al. (Intentional Interviewing) — Questions: eliciting concreteness with "Could you give me a specific example?"

☐ Approve ☐ Revise ☐ Retire Notes:

A client explains at length that her brother forgot her graduation. The counselor responds, "Sounds like you feel hurt. Is that right?" Ivey describes a full reflection of feeling as having several parts. Which part is MISSING from this response?

A. The context ✓ KEY

The response never links the feeling to its source, such as "because your brother missed your graduation."

B. A feeling label

"Hurt" names the feeling, so this part is present.

C. A sentence stem

"Sounds like you feel" is a sentence stem, so this part is present.

D. A check-out

"Is that right?" is a check-out that invites the client to confirm or correct.

Rationale: Ivey's reflection of feeling typically includes a sentence stem ("You feel..."), a feeling label, the context or situation the feeling is about, and a check-out to confirm accuracy; a present-tense focus can add impact. Leaving out the context can make a reflection sound generic. Adding a brief paraphrase of the situation ties the feeling to the client's own story.

References: Ivey et al. (Intentional Interviewing) — Reflection of feeling: sentence stem, feeling label, context and check-out

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "I keep meaning to call my dad, but every time I pick up the phone I put it down again." The counselor replies, "You keep meaning to call your dad, but every time you pick up the phone you put it down again." What is the MAIN weakness of this reply?

A. It adds an interpretation the client did not offer

Nothing new is added; the reply contains only the client's own words.

B. It moves the focus from the client to the counselor

The reply stays on the client's statement, so the focus has not shifted.

C. It parrots the client instead of restating the essence ✓ KEY

Word-for-word repetition shows little understanding; a paraphrase restates the core message in fresh, briefer words.

D. It asks a closed question that limits what the client says

The reply is a statement, not a question of any kind.

Rationale: A paraphrase captures the essence of what a client said, keeping a few key words but mostly using the counselor's own, briefer language. Repeating a statement verbatim, often called parroting, sounds mechanical and does not show understanding. A better reply might be, "Part of you wants to reach out to him, and something keeps stopping you."

References: Ivey et al. (Intentional Interviewing) — Paraphrasing: restating the essence of client content in fresh words

☐ Approve ☐ Revise ☐ Retire Notes:

In a first session, a court-referred 27-year-old says he "barely drinks" and later mentions two recent blackouts. The counselor notices the discrepancy. Which approach to confrontation is MOST appropriate at this point?

A. Note the discrepancy and build trust before raising it gently ✓ KEY

Confrontation works best within a trusting relationship, so the counselor notes the mixed message and raises it supportively once rapport exists.

B. Tell him at once that his account is inconsistent and untrue

A blunt challenge in a first session with a mandated client is likely to provoke defensiveness and dropout.

C. Drop the discrepancy, because confrontation harms the alliance

Supportive confrontation is a useful skill; the issue is timing and relationship, not avoiding it.

D. Ask the referring court to settle which of his two reports is true

This sidesteps the counseling relationship and casts the counselor as an investigator.

Rationale: Supportive confrontation points out discrepancies so that clients can examine them, but it depends on a solid working relationship. Ivey stresses listening first to identify mixed messages and challenging in a nonjudgmental way once the client feels understood. Early, harsh confrontation, especially with mandated clients, tends to increase defensiveness rather than change.

References: Ivey et al. (Intentional Interviewing) — Confrontation: supportive challenge built on listening and a working relationship · Gladding (Counseling: A Comprehensive Profession) — Counseling skills: confrontation and the importance of timing and rapport

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor notes that a client says her health comes first, yet she answers work email until midnight. The client pauses, then says, "You're right. Starting Monday I'll turn off alerts at 8 and tell my manager I'm offline after that." On Ivey's Client Change Scale, where does this response fall?

A. Partial examination

Partial examination involves working on only part of the discrepancy without fully accepting it.

B. Denial

In denial, the client does not acknowledge the discrepancy at all.

C. Creation of a new solution ✓ KEY

The client both accepts the discrepancy and produces a concrete new plan to resolve it.

D. Transcendence

Transcendence is a broader shift in thinking that reaches well beyond this single issue.

Rationale: Ivey's Client Change Scale rates responses to confrontation in five levels: denial, partial examination, acceptance and recognition, creation of a new solution, and transcendence. Acceptance alone would be level 3; this client moves further by generating a specific, workable change. Counselors use the scale to judge whether a challenge is helping and to plan the next intervention.

References: Ivey et al. (Intentional Interviewing) — Client Change Scale: denial, partial examination, acceptance and recognition, new solution, transcendence

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor asks an 8-year-old to close her eyes, imagine she is a rosebush, then draw it and speak as the bush: "I have thorns so no one can get close." The counselor uses clay, sensory activities and drawing to strengthen the child's sense of self and contact. Which approach is this?

A. Filial family therapy, developed by Bernard Guerney

Filial therapy trains parents to hold child-centered play sessions; it does not use this fantasy-and-art method.

B. Gestalt play therapy, developed by Violet Oaklander ✓ **KEY**

Oaklander's rosebush fantasy and her focus on the senses, self and contact are hallmarks of Gestalt play therapy.

C. Jungian sandplay therapy, developed by Dora Kalff

Sandplay uses a sand tray and miniature figures within a Jungian frame.

D. Child-centered play therapy, developed by Axline

Virginia Axline's child-centered play therapy is nondirective, so the counselor would not lead a fantasy exercise like this.

Rationale: Violet Oaklander, author of *Windows to Our Children*, applied Gestalt principles to work with children. Her methods include drawing, clay, sensory awareness and projective fantasies such as the rosebush, in which the child speaks as the image in the first person. The aim is to strengthen the child's sense of self and ability to make contact, so that blocked feelings can be expressed.

References: Corey (Theory & Practice) — Gestalt therapy: applications with children; Oaklander's projective techniques

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor equipping a child-centered playroom includes a bop bag, toy soldiers, a rubber knife and handcuffs. A teacher worries these toys encourage violence. Using Landreth's toy categories, what is their MAIN purpose?

A. To let the counselor test whether the child is at risk of violence

These toys are not assessment tools; using them this way would misread normal symbolic play.

B. To teach children rules for safe play through counselor instruction

Child-centered play therapy is nondirective; the counselor does not instruct the child in play rules.

C. To reward children for cooperative behavior earlier in the session

Toys are not used as behavioral rewards in child-centered play therapy.

D. To let children safely express anger, fear and aggression in play ✓ **KEY**

Acting-out and aggressive-release toys give children a safe outlet for feelings they cannot yet put into words.

Rationale: Landreth groups playroom toys into real-life (nurturing) toys, acting-out or aggressive-release toys, and toys for creative expression and emotional release. Aggressive-release toys let children express anger, fear and the need for control symbolically, within limits the counselor sets only when needed. Expressing these feelings in play, rather than acting them out on others, is part of what makes play therapeutic.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: play therapy and the selection of toys and materials

☐ Approve ☐ Revise ☐ Retire Notes:

Building on Lindemann's work, which psychiatrist described crisis as a temporary loss of equilibrium when usual coping fails, and introduced primary, secondary and tertiary prevention to community mental health?

A. Albert Roberts

Roberts developed the later seven-stage crisis intervention model.

B. Kendra Kanel

Kanel developed the ABC model of crisis intervention.

C. Viktor Frankl

Frankl founded logotherapy, an existential approach, not crisis theory.

D. Gerald Caplan ✓ **KEY**

Caplan extended Lindemann's work into crisis theory and preventive psychiatry with three levels of prevention.

Rationale: Gerald Caplan, working with Erich Lindemann, framed crisis as a time-limited state of disequilibrium that occurs when a person's usual problem-solving fails. In *Principles of Preventive Psychiatry* (1964), he described primary, secondary and tertiary prevention, which shaped community mental health. Crisis intervention models that followed build on these ideas.

References: James & Gilliland (*Crisis Intervention Strategies*) — Historical foundations of crisis intervention: Lindemann and Caplan

☐ Approve ☐ Revise ☐ Retire Notes:

A 58-year-old who devoted her life to her company learns she will be laid off. She tells the counselor, "None of it meant anything. I don't know who I am or why I bothered." Using James and Gilliland's types of crisis, which type does this MOST closely fit?

A. Existential crisis ✓ **KEY**

Her distress centers on purpose, meaning and identity, which defines an existential crisis.

B. Developmental crisis

Developmental crises arise from normal life transitions, such as a birth or retirement, not from a sudden loss of meaning.

C. Ecosystemic crisis

Ecosystemic crises are natural or human-caused disasters affecting a whole community.

D. Psychiatric crisis

This is not one of James and Gilliland's crisis types, and nothing suggests a psychiatric emergency.

Rationale: James and Gilliland describe normal developmental, situational, existential and ecosystemic crises. A layoff is a situational trigger, but this client's core struggle is about meaning, purpose and identity, which marks an existential crisis. Naming the type helps the counselor attend to what matters most to the client, here meaning and self-worth beyond the job itself.

References: James & Gilliland (*Crisis Intervention Strategies*) — Types of crises: developmental, situational, existential, ecosystemic

☐ Approve ☐ Revise ☐ Retire Notes:

Years after a robbery at her store, a 46-year-old owner says she "got over it" quickly and never talked about it. After a neighbor's shop is burglarized, she suddenly cannot sleep, avoids the store and feels as panicked as on the night of the robbery. In James and Gilliland's terms, what BEST describes this?

A. A new developmental crisis set off by midlife

Developmental crises arise from normal life transitions; her reaction is tied to the earlier robbery.

B. An ecosystemic crisis affecting her community

Ecosystemic crises are large-scale disasters; one nearby burglary does not fit that type.

C. A transcrisis state from the unresolved robbery ✓ KEY

The original crisis was submerged rather than resolved, and a new stressor has brought it back to the surface.

D. Normal equilibrium after a successfully resolved crisis

Her sudden, intense distress shows the earlier crisis was not resolved in an adaptive way.

Rationale: James and Gilliland describe a transcrisis state as a crisis that was not resolved but pushed out of awareness, where it can lie dormant for years. A later stressor, often one that resembles the original event, can bring the old reaction back with its original intensity. Counselors who recognize this pattern address the earlier, unresolved crisis rather than only the recent trigger.

References: James & Gilliland (Crisis Intervention Strategies) — Transcrisis states: unresolved crises that resurface with later stressors

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor and a first-generation college student map the jobs, education and work values of the student's parents, grandparents, aunts and uncles across three generations. What is this career intervention called?

A. A card sort

A card sort has the client sort occupation or value cards, not map family members.

B. A career genogram ✓ KEY

A career genogram maps the occupations and work messages in a client's family across generations.

C. A lifeline

A lifeline plots the client's own past and future life events along a timeline.

D. An ecomap

An ecomap shows the client's current links to people and community systems, not work histories by generation.

Rationale: The career genogram, adapted from family therapy by Okiishi, charts occupations, education and work-related beliefs across about three generations. It helps clients see family expectations, role models and messages that shape their own choices. It is especially useful for exploring family and cultural influences on career decisions.

References: Sharf (Career Development Theory) — Qualitative career assessment: the career genogram

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor helps a 44-year-old client weave together love, labor, learning and leisure, attend to spirituality and global needs, and see her life as a quilt of connected pieces rather than a single job choice. Which approach is the counselor using?

A. Parsons's trait-and-factor approach

Parsons matched personal traits to job requirements; he did not address life roles or global needs.

B. Roe's personality development theory

Roe linked early family climate to person- or non-person-oriented occupations.

C. Hansen's integrative life planning ✓ KEY

Hansen's model uses the quilt metaphor and integrates love, labor, learning and leisure within broader life tasks.

D. Holland's theory of vocational types

Holland matches RIASEC personality types to work environments, not whole-life planning.

Rationale: Sunny Hansen's integrative life planning views career as part of a whole life, using the image of a quilt to show how the pieces fit together. It joins the four L's of love, labor, learning and leisure and names critical life tasks such as finding work that needs doing in changing global contexts and exploring spirituality and life purpose. It is a holistic alternative to single-decision matching models.

References: Sharf (Career Development Theory) — Hansen's integrative life planning: quilt metaphor and critical life tasks

☐ Approve ☐ Revise ☐ Retire Notes:

A 22-year-old completes the Career Thoughts Inventory. His highest score shows that he struggles to weigh his own view of his interests against the strong opinions of his parents and partner about what he should do. Which CTI scale is MOST likely elevated?

A. Decision-Making Confusion

This scale reflects trouble starting or sustaining the decision process because of emotional distress or limited knowledge of how to decide.

B. External Conflict ✓ KEY

External Conflict reflects difficulty balancing one's own self-perceptions with input from significant others.

C. Commitment Anxiety

Commitment Anxiety reflects anxiety about committing to a choice once options are known.

D. Vocational Identity

Vocational Identity is a scale of Holland's My Vocational Situation, not the CTI.

Rationale: The Career Thoughts Inventory, developed by Sampson and colleagues within the cognitive information processing approach, measures negative career thoughts. Its three scales are Decision-Making Confusion, Commitment Anxiety and External Conflict. A high External Conflict score points the counselor toward helping the client sort out his own views from the expectations of important others.

References: Sharf (Career Development Theory) — Cognitive information processing: Career Thoughts Inventory scales

☐ Approve ☐ Revise ☐ Retire Notes:

A 13-year-old refuses to go to school after getting a bad haircut, convinced that "everyone will be staring and talking about it all day." Which concept from Elkind's account of adolescent egocentrism BEST explains this?

A. The imaginary audience ✓ KEY

The imaginary audience is the belief that one is the focus of everyone else's attention and judgment.

B. The personal fable

The personal fable is the belief that one's experiences are unique or that one is invulnerable.

C. Identity foreclosure

Foreclosure is Marcia's status of commitment without exploration, unrelated to feeling watched.

D. Centration

Centration is a preoperational tendency to focus on one feature of a problem.

Rationale: David Elkind described two forms of adolescent egocentrism that come with the new ability to think about thinking. The imaginary audience is the belief that others are constantly watching and evaluating the teen, which fuels self-consciousness. The personal fable, its partner, is the belief that one's feelings and experiences are unique.

References: Berk (Development Through the Lifespan) — Adolescent cognitive development: imaginary audience and personal fable

☐ Approve ☐ Revise ☐ Retire Notes:

Q170 of 200 · nce-s-cou-406 · Human Growth and Development · attachment theory · hard · In review

A 2½-year-old protests less when his mother leaves for work now. He asks when she will return, and he asks her to read one story before she goes, and she agrees. In Bowlby's phases of attachment, which phase does this reflect?

A. Preattachment

Preattachment covers roughly the first six weeks, when infants are comforted by any caregiver.

B. Clear-cut attachment

In clear-cut attachment, infants show strong separation protest without yet understanding or negotiating departures.

C. Attachment in the making

This phase, around 6 weeks to 6-8 months, involves growing preference for familiar caregivers.

D. Formation of a reciprocal relationship ✓ KEY

Growing language and representation let the toddler understand the parent's comings and goings and negotiate.

Rationale: Bowlby described four phases: preattachment, attachment in the making, clear-cut attachment and formation of a reciprocal relationship. From about 18 months to 2 years, language and mental representation let toddlers understand why a parent leaves and predict a return. Separation protest declines, and children begin to negotiate with caregivers, using requests and persuasion.

References: Berk (Development Through the Lifespan) — Bowlby's ethological theory: phases of attachment, formation of a reciprocal relationship

☐ Approve ☐ Revise ☐ Retire Notes:

During a long hospital stay without his parents, a toddler first screams and searches for his mother, then becomes listless and withdrawn. By the third week he seems cheerful with staff but treats his mother with indifference when she visits. Which phase describes his final reaction?

A. Detachment ✓ KEY

Detachment is the last phase, in which the child appears to recover but responds to the parent with indifference.

B. Despair

Despair is the middle phase of listlessness and withdrawal, which came before this reaction.

C. Protest

Protest is the first phase of crying and searching for the absent parent.

D. Avoidance

Avoidance names an insecure attachment pattern in the Strange Situation, not a phase of prolonged separation.

Rationale: James Robertson and John Bowlby observed that young children separated from parents for long periods move through protest, despair and detachment. In detachment, the child seems to adjust and engages with others but shows apparent indifference to the parent, masking the attachment loss. These observations helped change hospital policies to allow parents to stay with young children.

References: Berk (Development Through the Lifespan) — Attachment: effects of prolonged separation (protest, despair, detachment)

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor serving undocumented immigrant families testifies before a state legislative committee and joins a coalition lobbying for a bill that would expand access to mental health care regardless of immigration status. In the MSJCC socioecological model, at which level is she intervening?

A. Interpersonal

Interpersonal work targets relationships among family, friends and peers.

B. Institutional

Institutional work targets organizations such as schools, agencies and employers.

C. Intrapersonal

Intrapersonal work targets the client's own beliefs, feelings and identity.

D. Public policy ✓ KEY

Testifying and lobbying for legislation target laws and policies, the public policy level.

Rationale: The Multicultural and Social Justice Counseling Competencies use a socioecological model with intrapersonal, interpersonal, institutional, community, public policy and international or global levels. Counselors may intervene at any level, from individual counseling to systemic advocacy. Work on laws, regulations and funding falls at the public policy level.

References: MSJCC (Ratts et al., 2016) — Counseling and advocacy interventions: socioecological levels, including public policy

☐ Approve ☐ Revise ☐ Retire Notes:

A Black client is guarded with his White counselor, shares little personal detail early on, and asks about the counselor's experience working with Black men. A colleague suggests the client may be paranoid. Which interpretation is MOST consistent with Sue and Sue?

A. Resistance that should be confronted directly in the session

Treating adaptive caution as resistance pathologizes the client and is likely to harm the alliance.

B. Healthy cultural paranoia, an adaptive response to racism ✓ **KEY**

Grier and Cobbs's concept frames such mistrust as a reasonable, protective response to experiences of racism.

C. An early sign of a paranoid personality style needing assessment

Nothing beyond appropriate caution suggests a personality disorder; this would misdiagnose a cultural response.

D. Transference of feelings about his father onto the counselor

The guardedness reflects sociopolitical realities, not displaced feelings about a parent.

Rationale: Grier and Cobbs used the term healthy cultural paranoia to describe the adaptive mistrust many Black Americans develop in response to racism. Sue and Sue caution that counselors may misread this caution as pathology or resistance. Openly acknowledging racial differences and earning trust are more helpful than labeling the client.

References: Sue & Sue (Counseling the Culturally Diverse) — Sociopolitical dimensions: mistrust, healthy cultural paranoia and counselor credibility

☐ Approve ☐ Revise ☐ Retire Notes:

A White first-year counseling student says, "I don't really see color. I treat everyone the same, so race doesn't come up for me." She has had little contact with people of other races. In Helms's White racial identity model, which status does this reflect?

A. Autonomy

Autonomy reflects a mature, nonracist identity that actively values diversity.

B. Disintegration

Disintegration involves guilt and conflict after racism becomes hard to ignore.

C. Contact ✓ **KEY**

Contact is marked by a color-blind stance and little awareness of race or racism.

D. Reintegration

Reintegration involves idealizing White people and blaming people of color.

Rationale: Helms's model begins with contact, in which White people are largely unaware of race and racism and often claim to be color-blind. Later statuses are disintegration, reintegration, pseudo-independence, immersion/emersion and autonomy. Counselor education aims to move trainees past contact toward an informed, nonracist White identity.

References: Sue & Sue (Counseling the Culturally Diverse) — White racial identity development: Helms's contact status

☐ Approve ☐ Revise ☐ Retire Notes:

A Latino 20-year-old tells his counselor he avoids speaking Spanish in public, prefers only White friends and calls his own culture "backward." In Atkinson, Morten and Sue's Racial/Cultural Identity Development model, which stage does this fit?

A. Resistance and immersion

This stage involves rejecting dominant culture and embracing one's own group.

B. Conformity ✓ KEY

Conformity involves preferring dominant-culture values and depreciating one's own group.

C. Dissonance

Dissonance begins when experiences challenge the client's devaluing beliefs, causing conflict.

D. Integrative awareness

Integrative awareness reflects a secure identity that values both one's own and other cultures.

Rationale: The R/CID model has five stages: conformity, dissonance, resistance and immersion, introspection, and integrative awareness. In conformity, people from marginalized groups favor dominant-culture values and hold negative views of their own group. Counselors working with clients in this stage may be valued if they belong to the dominant group, which shapes how culture can be explored.

References: Sue & Sue (Counseling the Culturally Diverse) — Racial/Cultural Identity Development model: conformity stage

☐ Approve ☐ Revise ☐ Retire Notes:

In her second session, a 31-year-old with repeated childhood abuse wants to describe her worst memories in detail. She reports cutting herself this week, has no stable housing and has few coping skills. Which plan BEST fits a phase-oriented, trauma-informed approach?

A. Build safety, stability and coping skills before trauma processing ✓ KEY

A phase-oriented approach puts safety and stabilization first, then trauma processing once the client can tolerate it.

B. Begin detailed trauma processing now, since she is asking to do it

Processing trauma while she is self-harming and unstable risks overwhelming and destabilizing her.

C. Advise her to avoid discussing the abuse for the whole course of care

Trauma processing is not ruled out; it is sequenced after stabilization.

D. Refer her to a group for survivors so she can share her story there

This shifts disclosure to another setting without first addressing safety and stability.

Rationale: Trauma-informed, phase-oriented treatment begins with safety and stabilization: managing self-harm, meeting basic needs and building emotion-regulation skills. Trauma processing follows when the client can tolerate distressing material without becoming overwhelmed. Reconnection with relationships and daily life is the final focus.

References: SAMHSA TIP 57 — Trauma-specific treatment: establishing safety and stabilization before trauma processing

☐ Approve ☐ Revise ☐ Retire Notes:

A college student who was assaulted says, "I didn't fight or scream. I just froze. Maybe that means I let it happen." Which counselor response BEST reflects trauma-informed practice?

A. Ask her to think about what she could do differently next time
This implies she was responsible and could have prevented the assault.

B. Move on to safety planning and come back to this feeling later
Moving on misses the self-blame she has voiced, which is central to her distress.

C. Reassure her that she should not dwell on what happened that night
This dismisses her experience and gives no understanding of her reaction.

D. Explain that freezing is an automatic survival response, not consent ✓ **KEY**
Psychoeducation that normalizes freezing as an involuntary survival response reduces shame and self-blame.

Rationale: Trauma-informed care includes psychoeducation that helps clients understand common reactions to trauma. Freezing is an automatic survival response controlled by the nervous system, not a choice or a form of consent. Normalizing such reactions reduces shame and self-blame and supports recovery.

References: SAMHSA TIP 57 — Common responses to trauma; psychoeducation that normalizes survival reactions

☐ Approve ☐ Revise ☐ Retire Notes:

In a chronic pain support group, a 61-year-old member describes her problems in detail each week and asks for help. When members offer ideas, she explains why each one will not work, and her complaints seem to grow as the group tries harder. Which problem member type in Yalom's account does she MOST resemble?

A. The monopolist
A monopolist talks compulsively and dominates airtime; the defining feature here is rejecting the help offered.

B. The silent member
She talks readily each week, so she is not a silent member.

C. The help-rejecting complainer ✓ **KEY**
She asks for help, then rejects every suggestion while her complaints continue or grow.

D. The scapegoat
A scapegoat is a member the group blames; here, she is the one turning aside the group's efforts.

Rationale: Yalom describes the help-rejecting complainer as a member who repeatedly asks for help, then rejects each offer, often implying that the problem is beyond solving. Other members grow frustrated and may withdraw. Leaders avoid joining the cycle of advice and instead help the member and the group look at the pattern itself.

References: Yalom & Leszcz (Group Psychotherapy) — Problem group members: the help-rejecting complainer

☐ Approve ☐ Revise ☐ Retire Notes:

In an interpersonal group, Mei says, "There's something I've never told anyone, and I'm scared of how you'll all look at me after." Instead of asking for the secret, the leader asks what makes sharing it here feel risky and how she imagines each member might react. In Yalom's terms, the leader is encouraging:

A. vertical disclosure, about the depth of the secret's content

Vertical disclosure goes deeper into the content of the secret itself, which the leader is not asking for.

B. catharsis, the release of emotion tied to an old memory

Catharsis is a therapeutic factor about emotional release, not a type of disclosure the leader is shaping.

C. horizontal disclosure, about disclosing to this group now ✓ KEY

Horizontal disclosure is disclosure about the act of disclosing, focused on here-and-now feelings toward the group.

D. self-disclosure by the leader to model openness for members

The leader is inviting Mei's disclosure, not sharing anything personal.

Rationale: Yalom and Leszcz distinguish vertical disclosure, which goes into the depth of past or private content, from horizontal disclosure, which is disclosure about the disclosure itself. Horizontal disclosure keeps the work in the here-and-now: what makes it hard to share with these people, and what reactions the member expects. It can deepen engagement and trust even before, or without, revealing the secret.

References: Yalom & Leszcz (Group Psychotherapy) — The here-and-now: vertical versus horizontal self-disclosure

☐ Approve ☐ Revise ☐ Retire Notes:

Q180 of 200 · nce-s-cou-416 · Counseling and Helping Relationships · family systems theories (Bowen, structural, strategic) · hard · In review

Two anxious parents worry constantly about one of their three children, a 12-year-old they see as "fragile." They monitor her closely, and over time she has become the most anxious and least independent of the siblings. Which Bowen concept BEST describes this pattern?

A. Emotional cutoff

Emotional cutoff is distancing from one's family of origin to manage unresolved fusion.

B. Sibling position

Sibling position links birth order to traits; it does not explain one child absorbing parental anxiety.

C. Societal emotional process

This concept concerns anxiety at the level of society, not within one family.

D. Family projection process ✓ KEY

Parents project their own undifferentiation and anxiety onto one child, who becomes the most impaired.

Rationale: In Bowen's family projection process, parents transmit their own anxiety and lack of differentiation to a particular child. That child becomes the focus of worry, absorbs the most anxiety and tends to end up with the lowest differentiation of self. Repeated over generations, this process feeds the multigenerational transmission of symptoms.

References: Goldenberg (Family Therapy) — Bowen family systems theory: family projection process

☐ Approve ☐ Revise ☐ Retire Notes:

In a family session, the parents of a 15-year-old do not know who his friends are, rarely eat with him and learned about his school suspension weeks later. Each family member seems to live separately. In structural family therapy, this family's boundaries are BEST described as:

A. rigid, producing disengagement ✓ KEY

Rigid boundaries limit contact and support, so members are disengaged from one another.

B. diffuse, producing enmeshment

Diffuse boundaries create over-involvement, the opposite of this family's distance.

C. clear, producing healthy autonomy

Clear boundaries allow autonomy along with support and contact, which is missing here.

D. triangulated, producing coalitions

Triangulation describes drawing a third person into a dyad's conflict, not general distance.

Rationale: Minuchin described boundaries on a continuum from rigid to diffuse. Rigid boundaries lead to disengagement, in which members are isolated and slow to respond to one another's needs. Diffuse boundaries lead to enmeshment, and clear boundaries allow both closeness and autonomy.

References: Goldenberg (Family Therapy) — Structural family therapy: boundaries, disengagement and enmeshment

☐ Approve ☐ Revise ☐ Retire Notes:

In a psychodrama group, a 33-year-old man re-enacts a painful conversation with his late father. The director asks another group member to stand beside him and voice the grief and anger he seems unable to say aloud. In Moreno's psychodrama, what role is this member playing?

A. The protagonist

The protagonist is the man whose story is being enacted, not the member who supports him.

B. An auxiliary ego (double) ✓ KEY

An auxiliary ego plays roles in the protagonist's drama; as a double, it voices his unspoken thoughts and feelings.

C. The director

The director is the leader who guides the enactment and assigned this member the role.

D. The audience

The audience observes and later shares; this member has stepped into the enactment.

Rationale: Jacob Moreno's psychodrama includes the protagonist, the director, auxiliary egos, the audience and the stage. Auxiliary egos are group members who take roles in the protagonist's drama, such as significant others. In the doubling technique, an auxiliary ego stands beside the protagonist and expresses inner thoughts and feelings he has not voiced, helping him access and own them.

References: Gladding (Counseling: A Comprehensive Profession) — Group work approaches: Moreno's psychodrama (protagonist, director, auxiliary ego, doubling)

☐ Approve ☐ Revise ☐ Retire Notes:

In 1977, Tuckman and Jensen added a fifth stage to Tuckman's original model of forming, storming, norming and performing. What is this stage, and what happens in it?

A. Transforming, as the group sets new goals

Transforming is not a stage in Tuckman's model.

B. Adjourning, as the group comes to an end ✓ KEY

Adjourning covers ending the group, completing tasks and dealing with separation.

C. Reforming, as members renegotiate roles

Reforming is not a stage in Tuckman's model.

D. Mourning, as members grieve a lost member

Some writers use "mourning" informally, but the stage Tuckman and Jensen named is adjourning.

Rationale: Tuckman's 1965 model described forming, storming, norming and performing. In 1977, Tuckman and Jensen added adjourning, the stage in which the group completes its work and members deal with ending and separation. Leaders help members review what they gained and say goodbye.

References: Gladding (Counseling: A Comprehensive Profession) — Group work: stages of group development (Tuckman and Jensen)

☐ Approve ☐ Revise ☐ Retire Notes:

At the end of an eight-week panic disorder group, a 45-year-old member says what helped most was the leader's explanation of how the body's alarm response produces panic symptoms, along with practical advice for managing them. Which of Yalom's therapeutic factors does this BEST illustrate?

A. Imparting information ✓ KEY

Didactic teaching and direct advice from the leader or members make up imparting information.

B. Universality

Universality is the relief of learning that others share one's problems.

C. Catharsis

Catharsis is the release of strong emotion, which the member does not describe.

D. Instillation of hope

Instillation of hope comes from seeing others improve, not from learning facts about symptoms.

Rationale: Yalom's imparting information covers didactic instruction about mental health and illness and direct advice from the leader or other members. It is especially prominent in psychoeducational and symptom-focused groups, such as groups for panic disorder. Explaining how symptoms work can itself reduce fear by making them understandable.

References: Yalom & Leszcz (Group Psychotherapy) — Therapeutic factors: imparting information

☐ Approve ☐ Revise ☐ Retire Notes:

Core Counseling Attributes

16 items · NCE weight 8% (13 of 160 scored items)

Q185 of 200 · nce-s-cor-097 · Counseling and Helping Relationships · empathy and its levels · easy · In review

A 46-year-old hospice nurse says, "I hold it together for every family all day, and then I cry the whole drive home." The counselor replies, "You keep it together for the families all day, and then the tears come once you're alone in the car." On Carkhuff's five-level empathy scale, this response is BEST rated as

A. Level 2, subtractive

A Level 2 response drops noticeable feeling or meaning; this reply keeps both the strain and the release she described.

B. Level 3, interchangeable ✓ KEY

The reply returns essentially the same feeling and content the client expressed, neither adding nor subtracting, which defines Level 3.

C. Level 4, additive

An additive response names a deeper feeling or implied meaning the client has not voiced; this one stays with what she said.

D. Level 1, irrelevant or hurtful

Level 1 responses ignore or misread the client; this reply is accurate and respectful.

Rationale: Carkhuff's scale rates responses from Level 1 (hurtful) to Level 5 (deeply additive). Level 3 is the interchangeable, minimally facilitative level: the response captures the feeling and content the client expressed at about the same depth. Additive responses (Levels 4 and 5) go further by reflecting implied feelings or meanings the client has not stated.

References: Ivey et al. (Intentional Interviewing) — Empathic understanding: subtractive, interchangeable (basic) and additive responses, adapted from Carkhuff

☐ Approve ☐ Revise ☐ Retire Notes:

Q186 of 200 · nce-s-cor-098 · Counseling and Helping Relationships · unconditional positive regard · easy · In review

Carl Rogers used which term interchangeably with unconditional positive regard to describe a counselor's warm, nonpossessive caring for the client as a separate person?

A. mirroring

Mirroring is a self psychology concept from Kohut describing a selfobject need, not Rogers's term for acceptance.

B. holding

The holding environment is Winnicott's object relations concept, not a Rogerian term.

C. prizing ✓ KEY

Rogers described unconditional positive regard as acceptance, caring or prizing of the client as a person.

D. attunement

Affect attunement comes from Stern's work on infant development and is not Rogers's term for this condition.

Rationale: Rogers described unconditional positive regard as a warm acceptance, or prizing, of the client as a separate person with permission to have their own feelings and experiences. This caring is nonpossessive and does not depend on the client meeting conditions. It is one of the three counselor-offered core conditions, alongside congruence and accurate empathic understanding.

References: Corey (Theory & Practice) — Person-centered therapy: unconditional positive regard and acceptance (prizing) as a core condition

☐ Approve ☐ Revise ☐ Retire Notes:

After months of person-centered counseling, a 35-year-old architect who used to feel worthless after any mistake says, "I can mess up a design and still feel okay about who I am." In Rogers's theory, this change BEST reflects the client developing

A. unconditional positive self-regard ✓ KEY

Valuing himself regardless of performance shows he has internalized the counselor's unconditional regard as self-regard.

B. stronger conditions of worth

Conditions of worth make self-value depend on performance; his new stance is the opposite.

C. a greater need for positive regard

Rogers saw the need for positive regard as universal; therapy does not increase it, and his statement shows less reliance on approval.

D. a more idealized self-concept

A more idealized self would widen the gap between ideal and actual self; he is accepting his real, imperfect self.

Rationale: Rogers held that conditions of worth, learned when regard from others depends on meeting their standards, lead people to value themselves only when they perform. When a counselor consistently offers unconditional positive regard, the client can internalize it as unconditional positive self-regard. The client then values himself even when he falls short, and his self-concept becomes more congruent with his experience.

References: Corey (Theory & Practice) — Person-centered therapy: conditions of worth and the development of unconditional positive self-regard

☐ Approve ☐ Revise ☐ Retire Notes:

As a 40-year-old client describes her father's constant belittling, the counselor feels the same humiliation and helplessness the client describes. Weeks later, the counselor catches herself feeling impatient and critical toward the client, much as the father was. In Racker's terms, the counselor's FIRST reaction is BEST described as

A. complementary identification

Complementary identification is the later reaction: the counselor identifies with the client's internal figure, the critical father.

B. projective identification

Projective identification names the client's defensive process of inducing feelings in others, not the type of counselor identification.

C. reaction formation

Reaction formation is a defense that turns an impulse into its opposite; the counselor's feeling matches the client's.

D. concordant identification ✓ KEY

Feeling what the client feels, in tune with her own experience, is Racker's concordant identification.

Rationale: Heinrich Racker distinguished two kinds of countertransference identification. In concordant identification the counselor identifies with the client's own feelings and self-experience, which underlies empathy. In complementary identification the counselor identifies with someone in the client's inner world, such as a critical parent, and may feel pulled to treat the client as that person did. Noticing which kind is active helps the counselor understand the client instead of acting it out.

References: Gelso & Hayes (Countertransference) — Classical and object relations contributions: Racker's concordant and complementary identifications

☐ Approve ☐ Revise ☐ Retire Notes:

In a first family session, a 15-year-old answers in a few words, glances at her stepfather before each reply, and says nothing when asked what she would like to change at home. In Friedlander and colleagues' SOFTA model, the counselor's choice to spend part of the session with her alone MOST directly targets which alliance dimension?

A. Engagement in the therapeutic process

Engagement concerns taking part in setting goals and doing the work; her guardedness centers on whether it is safe to speak.

B. Safety within the therapeutic system ✓ KEY

Checking with a family member before speaking signals she does not feel safe to be open; individual time addresses that safety.

C. Emotional connection to the therapist

Emotional connection is the bond with the counselor; nothing suggests she dislikes or distrusts the counselor in particular.

D. Shared sense of purpose in the family

Shared purpose is family members' agreement and collaboration with each other, which a meeting apart does not directly build.

Rationale: The System for Observing Family Therapy Alliances (SOFTA) has four dimensions: engagement in the therapeutic process, emotional connection to the therapist, safety within the therapeutic system, and shared sense of purpose within the family. Safety refers to feeling comfortable taking risks and being open in front of other family members. When a member seems to monitor another before speaking, the counselor can build safety by meeting separately, setting ground rules or protecting the member from blame.

References: Friedlander et al. (Alliances in Couple & Family Therapy) — SOFTA dimensions: engagement, emotional connection, safety within the therapeutic system, shared sense of purpose

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor and a long-term client share a warm, genuine connection in which each perceives the other fairly accurately, apart from any transference and apart from their collaboration on goals and tasks. In Gelso's tripartite model of the therapy relationship, this component is called the

A. working alliance

The working alliance is the collaboration on goals and tasks with a bond, which the stem sets aside.

B. holding environment

The holding environment is Winnicott's term and is not one of the three components in Gelso's model.

C. real relationship ✓ KEY

The real relationship is the personal bond marked by genuineness and realistic perception of each other.

D. corrective experience

A corrective emotional experience is Alexander's term for a therapeutic event, not a component of Gelso's model.

Rationale: Gelso's tripartite model describes every therapy relationship as having three parts: the working alliance, the transference-countertransference configuration, and the real relationship. The real relationship is defined by genuineness (being authentic with each other) and realism (perceiving each other as they actually are). Research links a stronger real relationship to better outcomes, apart from the alliance.

References: Gelso & Hayes (Countertransference) — Tripartite model of the therapy relationship: working alliance, transference-countertransference configuration, real relationship

☐ Approve ☐ Revise ☐ Retire Notes:

A 27-year-old Indian American client says she has agreed to meet a man her parents found through family networks, with marriage as the shared goal. The counselor, who personally values choosing a partner on one's own, notices a flicker of unease. Which counselor response BEST reflects cultural humility?

A. Ask what this path means to her and what she hopes for, while holding her own values lightly ✓ **KEY**

Exploring the client's meaning and hopes puts her worldview first and keeps the counselor's preference out of the lead.

B. Gently remind her that she has the right to choose a partner without her parents' involvement

This presumes the counselor's individualistic value is the right one and implies her choice is not truly hers.

C. Explore whether she feels pressured, since family introductions tend to limit a woman's choice

Starting from an assumption of coercion stereotypes a consensual practice she has described as her own choice.

D. Share that the counselor also grew up with strong family expectations about whom to marry

This self-disclosure shifts the focus to the counselor before the client's own experience is understood.

Rationale: Cultural humility is an other-oriented stance in which the counselor recognizes the limits of her own worldview and lets the client define what a practice means to her. Family-arranged or family-assisted introductions are a consensual and valued tradition for many people and are distinct from forced marriage. The counselor notes her own reaction, explores the client's hopes, and stays open to concerns only if the client raises them.

References: Sue & Sue (Counseling the Culturally Diverse) — Collectivistic versus individualistic values; family role in decision making among Asian Americans · MSJCC (Ratts et al., 2016) — Counselor self-awareness and client worldview domains

☐ Approve ☐ Revise ☐ Retire Notes:

A 72-year-old Chinese immigrant referred by his physician for depression describes poor sleep, low energy and "a heavy feeling in my chest," but says he does not feel sad. Which counselor approach BEST reflects cultural humility?

A. Explain that chest heaviness is a common sign of depression so he can accept the diagnosis

Imposing the counselor's framework before learning his understanding can feel dismissive and weaken engagement.

B. Note that he is minimizing his mood and gently confront the denial of sadness in session

Treating his description as denial pathologizes a culturally common way of expressing distress.

C. Refer him back to his physician, since the concerns he presents are mainly physical ones

The physician already referred him; somatic expression of distress is within the counselor's work.

D. Ask what he thinks is causing the heaviness and what might help, using his words for it ✓ **KEY**

Exploring his own explanation of the problem respects his worldview and builds a shared understanding.

Rationale: Many clients, including some older Chinese immigrants, express psychological distress through bodily symptoms rather than mood words. A culturally humble counselor asks about the client's own explanatory model, meaning his view of what is wrong, why, and what would help, instead of correcting or confronting it. Using the client's language helps engagement and lets treatment fit his understanding.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling Asian Americans: somatic expression of psychological distress and culturally responsive assessment

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor realizes she tends to see clients' problems as the result of their own choices and believes anyone who works hard can change their circumstances. She wonders how this view lands with clients who face discrimination. In D. W. Sue's worldview model, her outlook is BEST described as

A. external locus of control, internal locus of responsibility

EC-IR describes people who feel little control yet blame themselves, a pattern Sue linked to marginality.

B. internal locus of control, internal locus of responsibility ✓ **KEY**

Believing people control their fate (IC) and cause their own problems (IR) is the IC-IR worldview typical of mainstream U.S. culture.

C. internal locus of control, external locus of responsibility

IC-ER holds that people can shape their lives when given a chance but that barriers are often systemic, which she does not endorse.

D. external locus of control, external locus of responsibility

EC-ER sees both control and blame as lying in the system, the opposite of her view.

Rationale: Sue's worldview model crosses locus of control (internal or external) with locus of responsibility (internal or external) to form four quadrants. The IC-IR quadrant reflects the individualistic values of mainstream U.S. culture and much of counseling theory. Counselors who hold this view without examining it may blame clients facing systemic barriers, so self-awareness of one's own worldview is a core multicultural competence.

References: Sue & Sue (Counseling the Culturally Diverse) — Worldview: locus of control and locus of responsibility, four quadrants (IC-IR, EC-IR, EC-ER, IC-ER)

☐ Approve ☐ Revise ☐ Retire Notes:

Reviewing her case notes, a counselor sees she described a 45-year-old client who identifies as bisexual and is married to a man as "formerly bisexual, now in a heterosexual marriage." The client has never described herself that way. What does this wording MOST likely reveal?

A. Accurate shorthand, since the marriage shows what the client's orientation is now

A partner's gender does not define a person's orientation; the client still identifies as bisexual.

B. A clinical clue that the client may be uncertain about her own sexual orientation

Nothing the client said suggests uncertainty; the label came from the counselor, not from her.

C. The counselor's assumption that a partner's gender defines the client's orientation ✓ **KEY**

Relabeling her identity from her marriage reflects the counselor's own bias, a form of bisexual erasure.

D. A reasonable summary, since case notes need to record only current relationship status

The note does more than record status; it overrides the identity the client herself claims.

Rationale: Sexual orientation is self-defined and is not determined by the gender of a current partner. Describing a bisexual client as "formerly bisexual" because she married a man reflects a common bias that erases bisexual identity. Counselor self-awareness includes catching how such assumptions surface in notes and language, correcting the record, and using the client's own terms.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ individuals: bisexual invisibility and counselor heterosexist assumptions

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old counselor notices that she looks forward to sessions with a charming male client, dresses with more care on those days, and has started daydreaming about him. What should she do FIRST?

A. Seek supervision to explore the attraction and keep firm professional boundaries in session ✓ KEY

Supervision helps her understand and contain the feelings while she continues to protect the client and the work.

B. Tell the client about her feelings so the attraction is out in the open and can be processed

Disclosing attraction burdens the client, serves the counselor's needs and risks a boundary violation.

C. Refer him to another counselor at once without explanation, to remove the risk completely

An abrupt, unexplained referral can harm the client; feelings alone do not require ending the work.

D. Wait to see whether the feelings fade, since attraction to clients tends to pass on its own

Waiting alone leaves the feelings unexamined and free to shape her behavior in session.

Rationale: Sexual attraction to a client is a common form of countertransference and is not unethical in itself; acting on it is. The ACA Code of Ethics prohibits sexual or romantic relationships with current clients, so the counselor's task is to recognize, understand and manage the feelings. Supervision or consultation is the first step, and a referral is considered only if the feelings cannot be managed in the client's interest.

References: ACA Code of Ethics — A.5.a Sexual and/or Romantic Relationships Prohibited · Gelso & Hayes (Countertransference) — Managing countertransference: self-insight and the use of supervision

☐ Approve ☐ Revise ☐ Retire Notes:

Three months after her own father died, a counselor realizes that whenever a grieving client starts talking about his late father, she shifts the conversation to coping strategies or scheduling. This pattern is BEST understood as

A. empathic pacing that protects the client from being flooded

Pacing follows the client's readiness; here the shift follows the counselor's discomfort, not the client's cues.

B. projective identification arising from the client's grief

Nothing suggests the client is inducing these feelings; the source is the counselor's own recent loss.

C. overinvolvement from overidentifying with the client's loss

Overinvolvement draws the counselor closer to the material; she is moving away from it.

D. avoidant countertransference rooted in her own recent loss ✓ KEY

Steering away from material that touches her unresolved grief is a classic avoidant countertransference behavior.

Rationale: Countertransference can show up as overinvolvement or as underinvolvement, such as avoiding, withdrawing from or changing the subject when material touches the counselor's unresolved issues. Here the counselor's recent loss leads her away from the very grief the client needs to explore. Recognizing the pattern and taking it to supervision lets her return to the client's material.

References: Gelso & Hayes (Countertransference) — Countertransference behavior: avoidance and underinvolvement versus overinvolvement

☐ Approve ☐ Revise ☐ Retire Notes:

A 36-year-old client says she skipped her sister's wedding after a long feud. The counselor replies, "You really should have gone. Family has to come first, even when it's hard." Among Gordon's roadblocks to listening, which type of response is this?

A. warning or threatening

Warning points to consequences if the client does not change; this reply states what she ought to have done.

B. interpreting or analyzing

Interpreting offers a hidden reason for the behavior; the counselor here passes moral judgment instead.

C. moralizing or preaching ✓ **KEY**

Telling the client what she should have done and invoking a rule about family is moralizing.

D. questioning or probing

The reply asks nothing; it delivers a verdict on her choice.

Rationale: Thomas Gordon described twelve roadblocks that interrupt listening, including ordering, warning, advising, arguing, moralizing, judging, approving, shaming, interpreting, reassuring, questioning and distracting. Moralizing tells the client what she "should" do and imposes the counselor's values, which tends to produce defensiveness and shuts down exploration. A nonjudgmental reply would instead reflect her experience and explore what led to the decision.

References: Miller & Rollnick (MI) — Gordon's twelve roadblocks to listening, including moralizing, preaching and telling clients what they should do

☐ Approve ☐ Revise ☐ Retire Notes:

A 47-year-old client recently laid off from a warehouse job lists one failure after another and calls himself "useless." To convey respect for him as a capable person, the counselor asks about times he handled hard situations well and what he values in himself. In Ivey's microskills approach, this strategy is called the

A. positive asset search ✓ **KEY**

Ivey's positive asset search draws out strengths, past successes and resources to build on.

B. reflection of meaning

Reflection of meaning explores the values and purpose behind experience, not a deliberate search for strengths.

C. logical consequences

Logical consequences is an influencing skill that explores likely outcomes of the client's choices.

D. confronting a discrepancy

Confrontation points out mixed messages or contradictions; the counselor is eliciting strengths instead.

Rationale: Ivey and colleagues build a positive asset search, also framed as a strength and wellness approach, into every interview. Asking about past successes, personal strengths and supports counters a problem-saturated story and treats the client as capable, which communicates respect. Strengths identified this way also become resources for later goal setting.

References: Ivey et al. (Intentional Interviewing) — Positive psychology and the positive asset search: strengths, wellness and resources

☐ Approve ☐ Revise ☐ Retire Notes:

At the end of an intake, a counselor tells a 58-year-old client, "You meet criteria for GAD with comorbid MDD, so we'll start a CBT protocol with an exposure hierarchy." The client nods but looks confused and says nothing. Which counselor action BEST addresses the power difference in this moment?

A. Give the client printed handouts on each diagnosis to read at home before the next session
Handouts can help later, but they leave the confusion and one-way communication in the room unaddressed.

B. Proceed with the plan, since the client nodded and can bring any questions to a later session
A nod from a confused client is not understanding or agreement; the counselor keeps all the control.

C. Repeat the diagnoses more slowly so the client can write the clinical terms down to look up
Slower jargon is still jargon; the client remains outside the professional language.

D. Restate the findings in plain language, ask what the client makes of them and plan together ✓ KEY
Plain language and an invitation to respond share power and make the plan a joint decision.

Rationale: Clinical jargon signals expertise and can silence clients, who may nod along rather than question a professional. The ACA Code of Ethics asks counselors to communicate in clear, understandable language that fits the client's development and culture. Translating findings and inviting the client's view turns a one-way pronouncement into collaborative treatment planning.

References: ACA Code of Ethics — A.2.c Developmental and Cultural Sensitivity · MSJCC (Ratts et al., 2016) — Counseling relationship domain: addressing power differences between counselor and client

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor has been asked to write a letter supporting a 27-year-old transgender woman's gender-affirming surgery. At their first meeting, the client seems guarded and gives answers that sound rehearsed. Which counselor action BEST addresses the power dynamic in this relationship?

A. Reassure her that the letter is a formality and that she need not worry about the outcome
This offers false reassurance and glosses over the real influence the counselor holds.

B. Name the counselor's role in the letter and invite her to say how it shapes what she shares ✓ KEY
Openly naming the gatekeeping role makes the power difference discussable and invites collaboration.

C. Postpone any talk of the letter until she has built enough trust to open up in sessions
Avoiding the topic leaves the power difference unspoken, which can deepen her guardedness.

D. Explain the assessment criteria in detail so that she knows which answers will qualify her
Framing it as right answers keeps the evaluative stance and invites more rehearsed replies.

Rationale: When a counselor controls access to something the client needs, such as a letter for surgery, the power difference can lead the client to present a guarded, rehearsed version of herself. Naming the role openly and inviting the client's view brings the dynamic into the relationship where it can be addressed. This reflects the MSJCC counseling relationship domain, which asks counselors to attend to how privilege and power shape the work.

References: MSJCC (Ratts et al., 2016) — Counseling relationship domain: addressing power and privilege between counselor and client

☐ Approve ☐ Revise ☐ Retire Notes: